



User Guide

This report contains a wide range of useful information about the liver transplant program at UPMC Children's Hospital of Pittsburgh. The report has three main sections:

- A. Program Summary
- B. Waiting List Information
- C. Transplant Information

The Program Summary is a one-page summary highlighting characteristics of the program, including the number of candidates on the waiting list, the number of transplants performed at the program, the number of patients being cared for by the program, and patient outcomes, including outcomes while on the waiting list (the transplant rate and the death rate while on the waiting list) and outcomes after transplant (patient and graft survival probabilities). If the program performed transplants in both adults and children, survival probabilities for adults and children (pediatrics) are provided separately. For each of the outcomes measures presented, a comparison is provided showing what would be expected at this program if it were performing as similar programs around the country perform when treating similar patients. More details regarding these outcome measures are provided in Sections B and C of the report.

The Waiting List Information section contains more detailed information on how many candidates are on the waiting list at the program, the types of candidates on the waiting list, how long candidates typically have to wait for a transplant at this program, how frequently candidates successfully receive a transplant, and how often candidates on the waiting list die before receiving a transplant.

Table B1 shows the activity on this program's waiting list during two recent 1-year periods and provides comparisons to all programs within this program's OPTN region (see <http://optn.transplant.hrsa.gov/members/regions.asp> for information on OPTN regions) and the nation as a whole. Tables B2 and B3 describe the candidates on the waiting list at this program, with comparisons to candidates waiting in the same donor service area (OPO/DSA) the OPTN region, and the nation as a whole.

Table B4 shows how many candidates were removed from the waiting list because they received a transplant. The program's transplant rate is calculated as the number of candidates who received a transplant divided by the person-years observed at the program (person-years is a combination of how many candidates were on the waiting list along with how long each candidate was followed since some candidates are not on the waiting list for the entire year). The transplant rate and comparisons to what would be expected at this program are presented in Figures B1 and B2. Figure B1 shows the transplant rate compared to what was expected at this program. The expected transplant rate is an estimate of what we would expect at this program if it were performing transplants at rates similar to other programs in the US with similar candidates on their waiting lists. The expected rate is only an estimate, and is made with a certain level of uncertainty. This uncertainty is shown in Figure B2. Figure B2 displays the ratio of the observed to the expected transplant rate. A ratio of 1 indicates that the observed transplant rate was equal to the expected transplant rate, while a ratio less than 1 indicates the observed rate was lower than expected rate and a ratio greater than 1 indicates the observed rate was higher than the expected rate. However, the level of uncertainty must be considered when interpreting these numbers. The 95% interval is also shown on Figure B2. This interval provides a range within which the true ratio of observed to expected transplant rates is likely to be. If this



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confidence interval includes (crosses) 1.0, then we cannot say that this program's observed transplant rate is different from what would be expected. The observed transplant rate at this program was 106.3 per 100 person-years. Transplant rates are also provided for adult and pediatric patients separately along with comparisons to adult and pediatric rates in the DSA, the OPTN region, and the nation. Transplant rates are also presented excluding transplants from a living donor (Table B4D and Figures B1D-B3D). Please refer to the PSR Technical Methods documentation available at <http://www.srtr.org> for more detail regarding how expected rates are calculated.

The pre-transplant mortality rate (previously called the waiting list mortality rate) for candidates on the waiting list is presented in Table B5 and Figures B4-B6. These data are presented in the same way as the transplant rate data in the previous section. The intent of this table and figures is to describe risk of death once candidates are listed rather than while they are listed, but before they are transplanted. Therefore, time at risk and deaths after removal from the waiting list for reasons other than transplant, transfer to another transplant program, or recovery (no longer needing a transplant), and before any subsequent transplant, are included. As with transplant rates, mortality rates should be interpreted carefully taking into consideration the interval displayed in Figure B5. For a complete description of how observed and expected mortality rates are calculated, please refer to the technical documentation available at <http://www.srtr.org>.

Survival from listing is presented in Table B6 and Figures B7-B9. These data are presented in the same way as the pre-transplant mortality rate data in the previous section. The intent of this table and figures is to describe risk of death once candidates are listed rather than while they are listed, including after a transplant. As with transplant rates, mortality rates should be interpreted carefully taking into consideration the interval displayed in Figure B8. For a complete description of how observed and expected mortality rates are calculated, please refer to the technical documentation available at <http://www.srtr.org>.

Table B7 presents information on what happens to candidates on the waiting list by three different time points after listing: 6 months, 12 months, and 18 months. The table displays percentages of candidates who have died, been removed from the waiting list, been transplanted, or been transferred or lost-to-follow-up. Tables B8 and B9 provide more detail regarding how many candidates have received a deceased donor transplant by certain time points during the first 3 years after being put on the transplant waiting list. Each row of Tables B8 and B9 presents the percent of candidates who received a deceased donor transplant by each time point. Table B10 presents data on the time it took for different percentages of patients to be transplanted for candidates added to the list between 01/01/2020 and 06/30/2025. The time it took for 5% (the 5th percentile) of patients to receive a transplant at this program was 0.3 months. If "Not Observed" is displayed in the table, then too few candidates received transplants before 12/31/2025 to calculate a particular percentile of transplant times.

Table B11 contains a summary of the offer acceptance practices of the program. The offer acceptance ratio indicates whether the program is more or less likely to accept offers than the average program. If the offer acceptance ratio is greater than 1.0, then the program tends to accept more offers than average; if the offer acceptance ratio is less than 1.0, then the program tends to accept fewer offers than average. Figure B10 shows the distribution of program offer acceptance rates as well as the offer acceptance rate for this program. Figures B11 - B15 similarly show offer acceptance rates for subsets



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of offers.

The Transplant Information section begins with descriptions of transplant recipients in Tables C1 and C2. Data on recipients of deceased donor transplants are presented (Tables C1D and C2D); if applicable, data on recipients of living donor transplants are presented separately (Tables C1L and C2L). Comparisons to the region and the nation as a whole are provided. A description of the deceased donors used at this program is provided in Table C3D, along with characteristics of living donors in Table C3L, if applicable. Finally, information on the transplant procedure for deceased and living donor transplants is presented in Tables C4D and C4L, respectively.

Starting with Table C5, transplant outcomes are presented along with comparisons to what would be expected at this program and what happened in the nation as a whole. Tables C5-C14 (tables C5-C10 for Pancreas) present information on graft survival (survival of the transplanted organ), with data presented separately for adult and pediatric recipients. Patients are followed from the time of transplant until either failure of the transplanted organ or death, whichever comes first. Please refer to the technical methods for more information on these calculations (<http://www.srtr.org>).

While Tables C5-C14 present data on graft survival, Tables C15-C20 (tables C11-C20 for Pancreas) present information on patient survival. For these tables, patients are followed from the time of transplant until death, regardless of whether the transplant is functioning or the patient required another transplant to survive.

Tables C21 and C22 summarize the multiorgan transplant outcomes at this program. The summary statistics in these tables are descriptive and are not risk-adjusted for different donor and candidate characteristics.

Table D1 shows the rates of follow-up for living donors.

Additional information regarding the technical methods and the risk adjustment models used to estimate expected event rates is available on the SRTR website at <http://www.srtr.org>. We welcome and encourage feedback on these reports. Please feel free to share feedback with the SRTR at the following e-mail: srtr@srtr.org.



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A. Program Summary

Figure A1. Waiting list and transplant activity

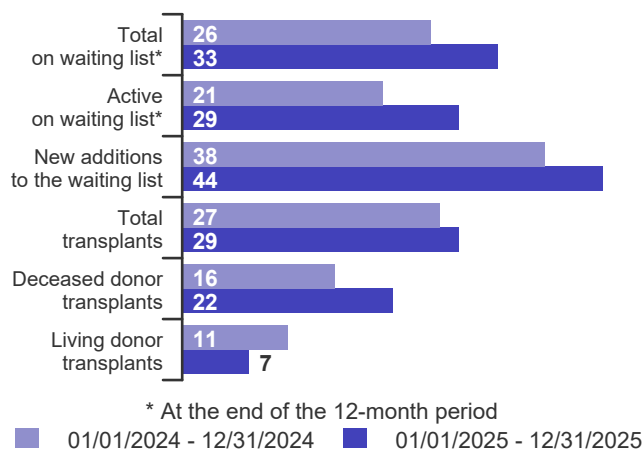


Table A1. Census of transplant recipients

Recipients	01/01/2024-12/31/2024	01/01/2025-12/31/2025
Transplanted at this center	27	29
Followed by this center*	285	239
...transplanted at this program	273	229
...transplanted elsewhere	12	10

* Recipients followed are transplant recipients for whom the center has submitted a post-transplant follow-up form for a transplant that took place before the 12-month interval for each column.

Figure A2. Transplant rates
01/01/2024 - 12/31/2025

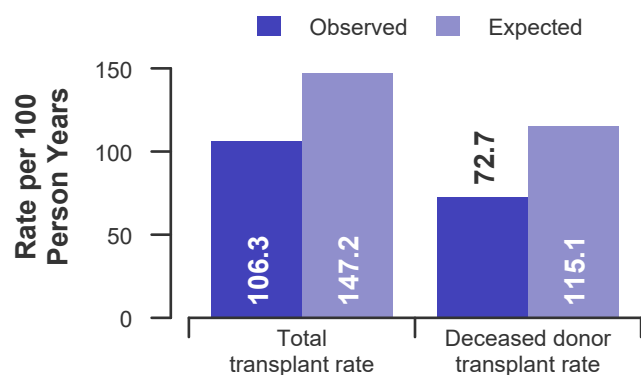


Figure A3. Pre-transplant mortality rates
01/01/2024 - 12/31/2025

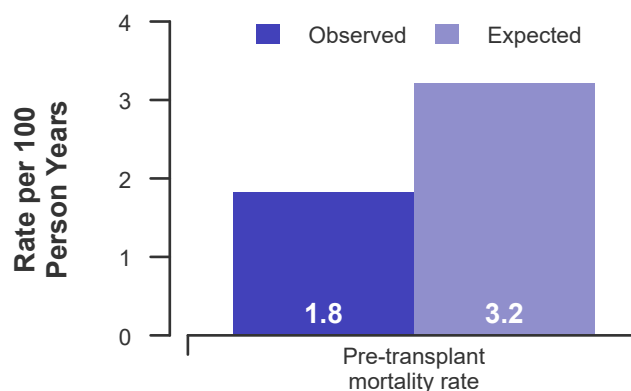


Figure A4. First-year adult graft and patient survival: 01/01/2023 - 06/30/2025

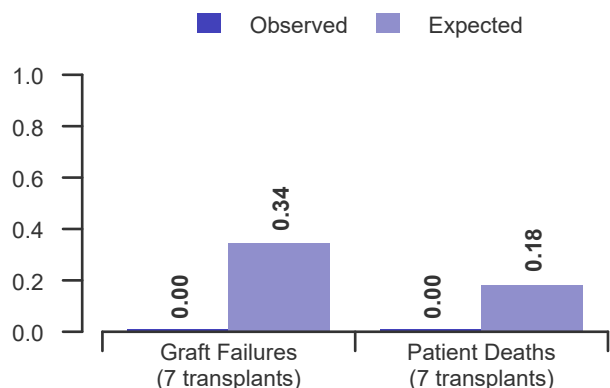
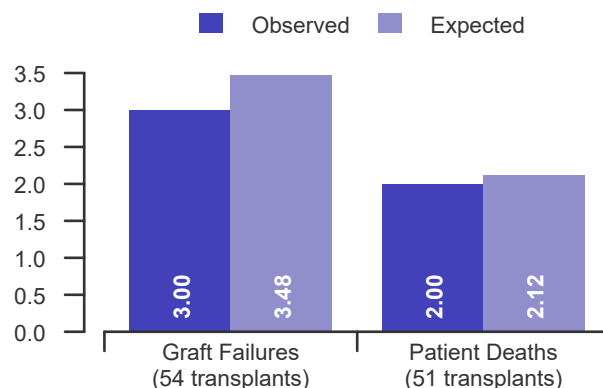


Figure A5. First-year pediatric graft and patient survival: 01/01/2023 - 06/30/2025





B. Waiting List Information

Table B1. Waiting list activity summary: 01/01/2024 - 12/31/2025

Waiting List Registrations	Counts for this center		Activity for 01/01/2025 to 12/31/2025 as percent of registrants on waiting list on 01/01/2025		
	01/01/2024- 12/31/2024	01/01/2025- 12/31/2025	This Center (%)	OPTN Region (%)	U.S. (%)
On waiting list at start	21	26	100.0	100.0	100.0
Additions					
New listings at this center	38	44	169.2	117.1	169.4
Removals					
Transferred to another center	1	2	7.7	1.9	1.3
Received living donor transplant*	11	7	26.9	13.5	7.4
Received deceased donor transplant*	17	22	84.6	73.0	121.9
Died	1	0	0.0	9.7	8.6
Transplanted at another center	3	1	3.8	3.2	2.5
Deteriorated	0	0	0.0	7.2	9.1
Recovered	0	4	15.4	9.8	11.5
Other reasons	0	1	3.8	8.7	11.2
On waiting list at end of period	26	33	126.9	90.1	96.1

* These patients were removed from waiting list with removal code indicating transplant; this may not equal the number of transplants performed at this center during the specified period.



B. Waiting List Information

Table B2. Demographic characteristics of waiting list candidates**Candidates registered on the waiting list between 01/01/2025 and 12/31/2025**

Demographic Characteristic	New Waiting List Registrations 01/01/2025 to 12/31/2025 (%)			All Waiting List Registrations on 12/31/2025 (%)		
	This Center (N=44)	OPTN Region (N=1,503)	U.S. (N=16,177)	This Center (N=33)	OPTN Region (N=1,156)	U.S. (N=9,175)
All (%)	100.0	100.0	100.0	100.0	100.0	100.0
Ethnicity/Race (%)*						
White	79.5	73.3	64.6	69.7	70.6	61.8
African-American	9.1	11.2	6.9	18.2	12.3	6.9
Hispanic/Latino	6.8	8.7	19.1	12.1	9.6	22.0
Asian	2.3	4.0	4.4	0.0	4.9	5.4
Other	0.0	0.3	2.2	0.0	0.4	2.3
Unknown	2.3	2.5	2.7	0.0	2.2	1.7
Age (%)						
<2 years	29.5	2.5	2.0	15.2	1.3	1.3
2-11 years	34.1	2.7	1.5	30.3	2.7	1.8
12-17 years	18.2	1.3	1.1	15.2	2.2	1.4
18-34 years	11.4	8.3	7.0	27.3	8.5	6.7
35-49 years	6.8	24.6	23.9	12.1	21.8	21.6
50-64 years	0.0	39.7	43.2	0.0	44.2	45.6
65-69 years	0.0	15.3	14.9	0.0	14.5	15.5
70+ years	0.0	5.7	6.5	0.0	4.8	6.0
Gender (%)						
Male	47.7	57.4	57.7	60.6	59.2	57.3
Female	52.3	42.6	42.3	39.4	40.8	42.7

* Race and ethnicity are reported together as a single data element, reflecting their data collection (either race or ethnicity is required, but not both). Patients formerly coded as white and Hispanic are coded as Hispanic. Race and ethnicity sum to 100%.



B. Waiting List Information

Table B3. Medical characteristics of waiting list candidates

Candidates registered on the waiting list between 01/01/2025 and 12/31/2025

Medical Characteristic	New Waiting List Registrations 01/01/2025 to 12/31/2025 (%)			All Waiting List Registrations on 12/31/2025 (%)		
	This Center	OPTN Region	U.S.	This Center	OPTN Region	U.S.
	(N=44)	(N=1,503)	(N=16,177)	(N=33)	(N=1,156)	(N=9,175)
All (%)	100.0	100.0	100.0	100.0	100.0	100.0
Blood Type (%)						
O	38.6	44.0	47.0	51.5	48.7	51.8
A	45.5	37.1	36.8	33.3	36.2	37.4
B	9.1	15.0	12.4	6.1	13.0	9.2
AB	6.8	3.8	3.8	9.1	2.1	1.6
Unknown	0.0	0.0	0.0	0.0	0.0	0.0
Previous Transplant (%)						
Yes	11.4	4.9	4.5	21.2	4.7	4.2
No	88.6	95.1	95.5	78.8	95.3	95.8
Unknown	0.0	0.0	0.0	0.0	0.0	0.0
Primary Disease (%)						
Acute Hepatic Necrosis	4.5	2.4	1.9	0.0	1.0	0.9
Non-Cholestatic Cirrhosis	2.3	7.9	9.9	0.0	27.2	22.6
Cholestatic Liver Disease/Cirrhosis	4.5	6.4	6.2	3.0	7.6	7.1
Biliary Atresia	11.4	1.8	1.6	12.1	2.1	2.0
Metabolic Diseases	40.9	3.0	1.8	36.4	2.7	1.4
Malignant Neoplasms	9.1	11.1	11.0	6.1	10.1	12.2
Other	27.3	67.1	67.5	42.4	49.0	53.5
Missing	0.0	0.4	0.1	0.0	0.3	0.1
Medical Urgency Status/MELD/PELD at Listing (%)*						
Status 1A	9.1	3.1	2.3	3.0	0.3	0.2
Status 1B	11.4	0.5	0.5	9.1	0.3	0.2
Status 2A	0.0	0.0	0.0	0.0	0.0	0.0
Status 2B	0.0	0.0	0.0	0.0	0.0	0.0
Status 3	0.0	0.0	0.0	0.0	0.0	0.0
MELD 6-10	9.1	12.2	11.1	27.3	26.1	20.3
MELD 11-14	2.3	11.6	12.0	9.1	20.2	21.5
MELD 15-20	11.4	19.8	22.6	9.1	28.1	31.2
MELD 21-30	6.8	20.8	23.1	9.1	15.4	14.0
MELD 31-40	2.3	17.4	13.6	0.0	1.6	1.2
PELD less than or equal to 10	34.1	2.5	1.2	21.2	2.4	1.9
PELD 11-14	2.3	0.3	0.3	6.1	0.3	0.2
PELD 15-20	2.3	0.5	0.4	3.0	0.2	0.3
PELD 21-30	6.8	0.3	0.2	0.0	0.1	0.1
PELD 31 or greater	0.0	0.0	0.1	0.0	0.0	0.0
Temporarily Inactive	2.3	6.5	8.5	3.0	4.9	8.8

* MELD/PELD score based on laboratory measures is shown for listings beginning 2/27/2002 unless patient is Status 1 or Temporarily Inactive. MELD/PELD scores based on exception rules are not used. Status 1 separated into 1A and 1B in August 2005.



B. Waiting List Information

Table B4. Transplant rates: 01/01/2024 - 12/31/2025

Waiting List Registrations	This Center	OPO/DSA	Region	U.S.
All Candidates				
Count on waiting list at start*	21	182	1,371	10,135
Person Years**	53.6	365.8	2,547.6	19,006.5
Removals for Transplant	57	512	2,198	23,801
Adult (18+) Candidates				
Count on waiting list at start*	6	167	1,315	9,731
Person Years**	17.7	329.8	2,413.2	18,177.0
Removals for transplant	4	459	2,047	22,637
Pediatric (<18) Candidates				
Count on waiting list at start*	15	15	56	404
Person Years**	36.0	36.0	134.4	829.5
Removals for transplant	53	53	151	1,164

* Counts in this table may be lower than similar counts in other waiting list tables, such as Table B1. A small percentage (~1%) of patients are found to have died or been transplanted before being removed from the waiting list, so these patients are excluded if the event occurs prior to the start of the study period. Inactive time on the waiting list is included in the calculations for this table.

** Person years are calculated as days (converted to fractional years). The number of days from January 1 or from the date of first wait listing until death, transplant, removal from the waiting list or December 31.

Figure B1. Observed and expected transplant rates: 01/01/2024 - 12/31/2025

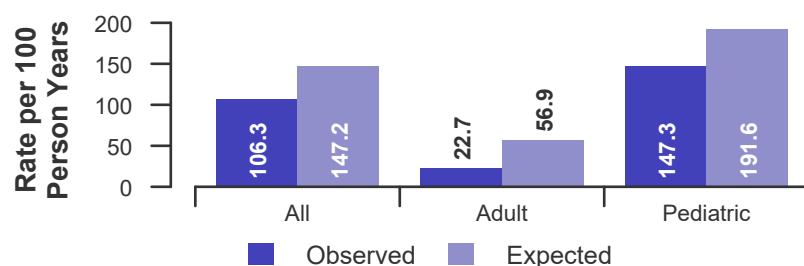


Figure B2. Transplant rate ratio estimate

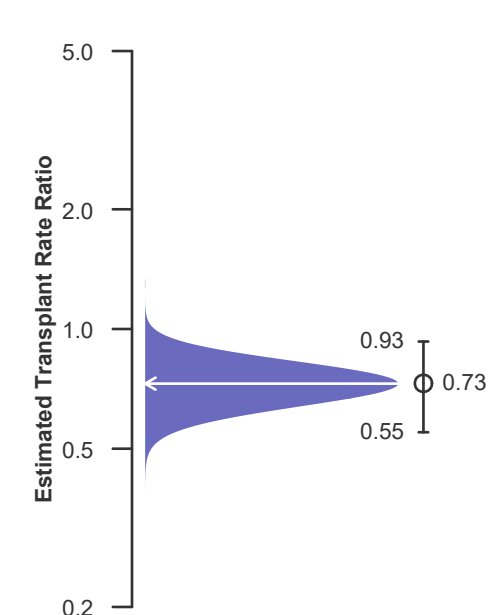
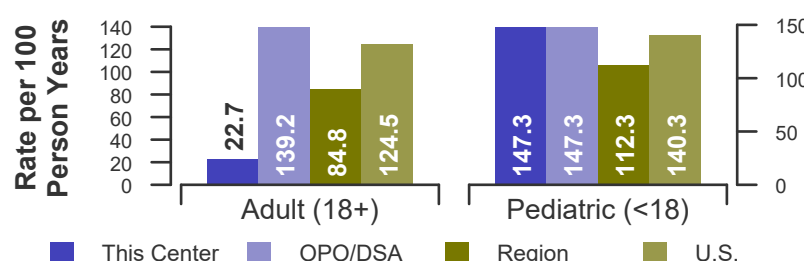


Figure B3. Observed adult (18+) and pediatric (<18) transplant rates: 01/01/2024 - 12/31/2025





B. Waiting List Information

Table B4D. Deceased donor transplant rates: 01/01/2024 - 12/31/2025

Waiting List Registrations	This Center	OPO/DSA	Region	U.S.
All Candidates				
Count on waiting list at start*	21	182	1,371	10,135
Person Years**	53.6	365.8	2,547.6	19,006.5
Removals for Transplant	39	333	1,882	22,488
Adult (18+) Candidates				
Count on waiting list at start*	6	167	1,315	9,731
Person Years**	17.7	329.8	2,413.2	18,177.0
Removals for transplant	4	298	1,771	21,541
Pediatric (<18) Candidates				
Count on waiting list at start*	15	15	56	404
Person Years**	36.0	36.0	134.4	829.5
Removals for transplant	35	35	111	947

* Counts in this table may be lower than similar counts in other waiting list tables, such as Table B1. A small percentage (~1%) of patients are found to have died or been transplanted before being removed from the waiting list, so these patients are excluded if the event occurs prior to the start of the study period. Inactive time on the waiting list is included in the calculations for this table.

** Person years are calculated as days (converted to fractional years). The number of days from January 1 or from the date of first wait listing until death, transplant, removal from the waiting list or December 31.

Figure B1D. Observed and expected deceased donor transplant rates: 01/01/2024 - 12/31/2025

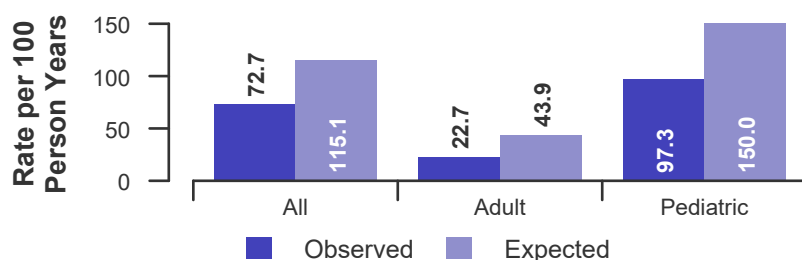


Figure B2D. Deceased donor transplant rate ratio estimate

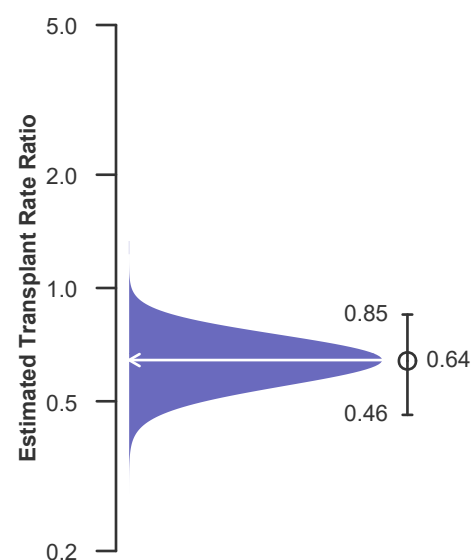
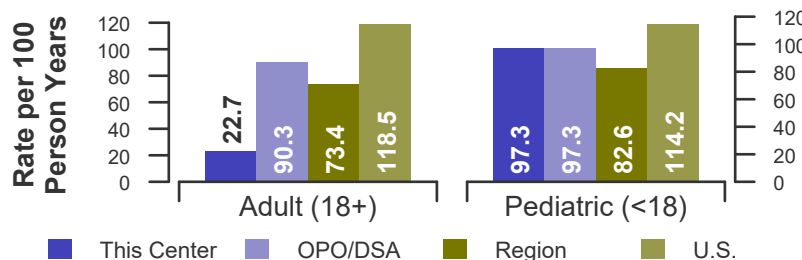


Figure B3D. Observed adult (18+) and pediatric (<18) deceased donor transplant rates: 01/01/2024 - 12/31/2025





B. Waiting List Information

Table B5. Pre-transplant mortality rates: 01/01/2024 - 12/31/2025

Waiting List Registrations	This Center	OPO/DSA	Region	U.S.
All Candidates				
Count on waiting list at start*	21	182	1,371	10,135
Person Years**	54.9	420.3	2,939.2	22,423.8
Number of deaths	1	49	378	2,751
Adult (18+) Candidates				
Count on waiting list at start*	6	167	1,315	9,731
Person Years**	17.7	383.1	2,794.9	21,516.7
Number of deaths	0	48	371	2,718
Pediatric (<18) Candidates				
Count on waiting list at start*	15	15	56	404
Person Years**	37.3	37.3	144.3	907.1
Number of deaths	1	1	7	33

* Counts in this table may be lower than similar counts in other waiting list tables, such as Table B1. A small percentage (~1%) of patients are found to have died or been transplanted before being removed from the waiting list, so these patients are excluded if the event occurs prior to the start of the study period. Inactive time on the waiting list is included in the calculations for this table.

** Person years are calculated as days (converted to fractional years). The number of days from January 1 or from the date of first wait listing until death, transplant, 60 days after recovery, transfer or December 31.

Figure B4. Observed and expected pre-transplant mortality rates: 01/01/2024 - 12/31/2025

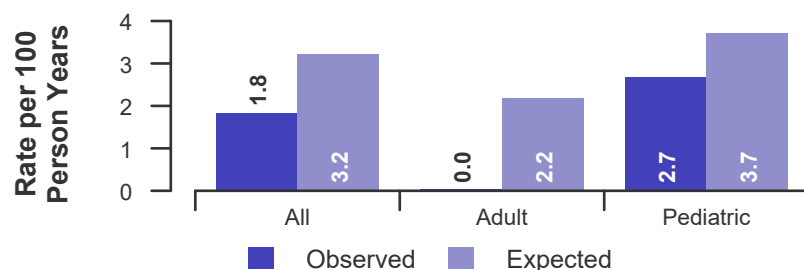


Figure B6. Observed adult (18+) and pediatric (<18) pre-transplant mortality rates: 01/01/2024 - 12/31/2025

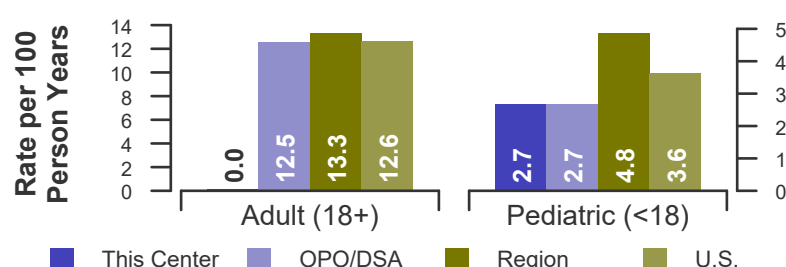
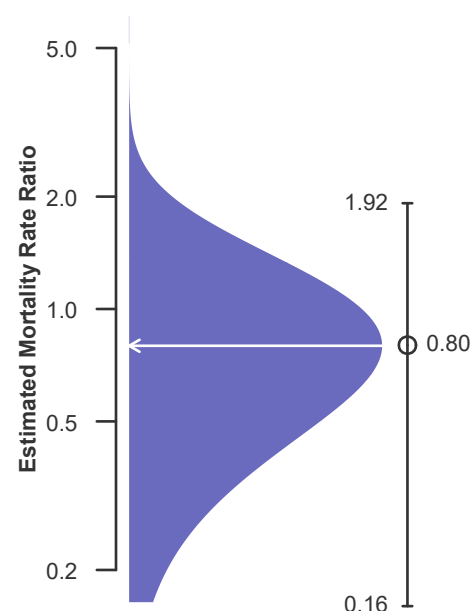


Figure B5. Pre-transplant mortality rate ratio estimate





B. Waiting List Information

Table B6. Rates of patient mortality after listing: 01/01/2024 - 12/31/2025

Waiting List Registrations	This Center	OPO/DSA	Region	U.S.
All Patients				
Count at risk during the evaluation period	219	1,926	8,986	85,845
Person-years*	305.9	2,579.4	12,096.4	115,619.4
Number of Deaths	6	143	753	6,194
Adult (18+) Patients				
Count at risk during the evaluation period	33	1,738	8,390	81,423
Person-years*	45.0	2,316.2	11,268.7	109,420.1
Number of Deaths	0	136	731	6,071
Pediatric (<18) Patients				
Count at risk during the evaluation period	186	188	596	4,422
Person-years*	260.9	263.2	827.7	6,199.3
Number of Deaths	6	7	22	123

* Person-years are calculated as days (converted to fractional years). The number of days from 01/01/2024, or from the date of first wait listing until death, reaching 5 years after listing or December 31, 2025.

** Patient mortality after listing describes the relative survival experience of patients after listing. It depends on many factors, some of which are outside of the control of the transplant program. For example, availability of organs may not be the same in every part of the country.

Figure B7. Observed and expected rates of patient mortality after listing: 01/01/2024 - 12/31/2025

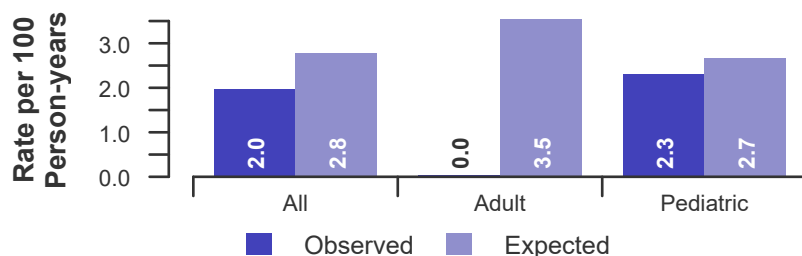


Figure B8. HR estimate of patient mortality after listing

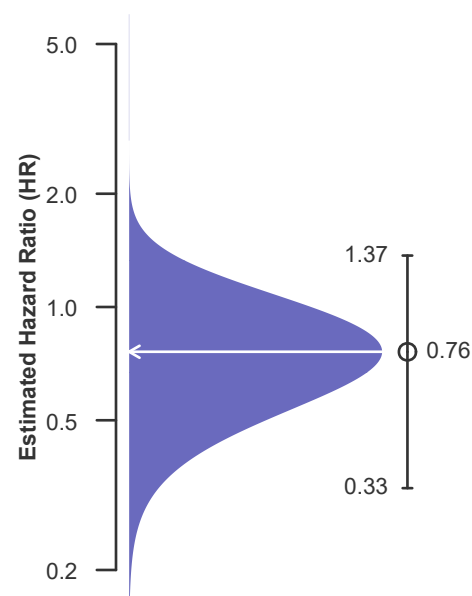
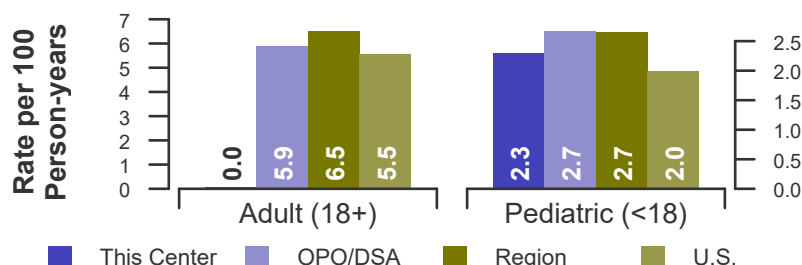


Figure B9. Observed adult (18+) and pediatric (<18) rates of patient mortality after listing: 01/01/2024 - 12/31/2025





B. Waiting List Information

Table B7. Waiting list candidate status after listing
Candidates registered on waiting list between 07/01/2023 and 06/30/2024

Waiting list status (survival status)	This Center (N=43)			U.S. (N=15,116)		
	Months Since Listing			Months Since Listing		
	6	12	18	6	12	18
Alive on waiting list (%)	39.5	25.6	23.3	32.7	17.0	10.7
Died on the waiting list without transplant (%)	7.0	7.0	7.0	3.7	4.5	4.9
Removed without transplant (%):						
Condition worsened (status unknown)	0.0	0.0	0.0	2.8	4.0	4.6
Condition improved (status unknown)	0.0	2.3	2.3	1.5	2.7	3.5
Refused transplant (status unknown)	0.0	0.0	0.0	0.2	0.3	0.5
Other	0.0	0.0	0.0	1.6	2.8	3.8
Transplant (living donor from waiting list only) (%):						
Functioning (alive)	23.3	27.9	9.3	2.5	3.1	2.0
Failed-Retransplanted (alive)	0.0	0.0	0.0	0.1	0.1	0.1
Failed-alive not retransplanted	0.0	0.0	0.0	0.0	0.0	0.0
Died	0.0	2.3	2.3	0.1	0.1	0.1
Status Yet Unknown**	0.0	0.0	18.6	0.0	0.1	1.4
Transplant (deceased donor) (%):						
Functioning (alive)	27.9	27.9	16.3	50.6	56.1	38.4
Failed-Retransplanted (alive)	0.0	0.0	0.0	0.5	0.7	0.9
Failed-alive not retransplanted	0.0	0.0	0.0	0.0	0.0	0.0
Died	0.0	2.3	2.3	2.2	3.3	4.5
Status Yet Unknown*	2.3	4.7	16.3	1.1	4.4	23.8
Lost or Transferred (status unknown) (%)	0.0	0.0	2.3	0.3	0.6	0.7
TOTAL (%)	100.0	100.0	100.0	100.0	100.0	100.0
Total % known died on waiting list or after transplant	7.0	11.6	11.6	5.9	8.0	9.5
Total % known died or removed as unstable	7.0	11.6	11.6	8.8	12.0	14.2
Total % removed for transplant	53.5	65.1	65.1	57.2	68.0	71.3
Total % with known functioning transplant (alive)	51.2	55.8	25.6	53.2	59.2	40.5

* Follow-up form covering specified time period not yet completed, and possibly has not become due.



B. Waiting List Information

Table B7S1. Medical urgency status 1 candidate status after listing

Candidates registered on the waiting list between 07/01/2023 and 06/30/2024

Waiting list status (survival status)	This Center (N=6)			U.S. (N=436)		
	Months Since listing			Months Since listing		
	6	12	18	6	12	18
Alive on waiting list (%)	0.0	0.0	0.0	0.7	0.2	0.0
Died on the waiting list without transplant (%)	33.3	33.3	33.3	6.7	6.7	6.7
Removed without transplant (%):						
Condition worsened (status unknown)	0.0	0.0	0.0	4.1	4.1	4.1
Condition improved (status unknown)	0.0	0.0	0.0	17.9	17.9	18.1
Refused transplant (status unknown)	0.0	0.0	0.0	0.0	0.0	0.0
Other	0.0	0.0	0.0	0.7	0.9	0.9
Transplant (living donor from waiting list only) (%):						
Functioning (alive)	16.7	16.7	0.0	1.4	1.4	0.2
Failed-Retransplanted (alive)	0.0	0.0	0.0	0.2	0.2	0.2
Failed-alive not retransplanted	0.0	0.0	0.0	0.0	0.0	0.0
Died	0.0	0.0	0.0	0.2	0.2	0.2
Status Yet Unknown**	0.0	0.0	16.7	0.0	0.0	1.1
Transplant (deceased donor) (%):						
Functioning (alive)	50.0	50.0	16.7	58.3	52.3	34.9
Failed-Retransplanted (alive)	0.0	0.0	0.0	0.5	0.5	0.5
Failed-alive not retransplanted	0.0	0.0	0.0	0.0	0.0	0.0
Died	0.0	0.0	0.0	8.9	10.6	11.7
Status Yet Unknown*	0.0	0.0	33.3	0.5	4.8	21.1
Lost or Transferred (status unknown) (%)	0.0	0.0	0.0	0.0	0.2	0.2
TOTAL (%)	100.0	100.0	100.0	100.0	100.0	100.0
Total % known died on waiting list or after transplant	33.3	33.3	33.3	15.8	17.4	18.6
Total % known died or removed as unstable	33.3	33.3	33.3	20.0	21.6	22.7
Total % removed for transplant	66.7	66.7	66.7	70.0	70.0	70.0
Total % with known functioning transplant (alive)	66.7	66.7	16.7	59.6	53.7	35.1

* Follow-up form covering specified time period not yet completed, and possibly has not become due.



B. Waiting List Information

Table B8. Percent of candidates with deceased donor transplants: demographic characteristics
Candidates registered on the waiting list between 01/01/2020 and 12/31/2022

Characteristic	Percent transplanted at time periods since listing									
	This Center					United States				
	N	30 day	1 year	2 years	3 years	N	30 day	1 year	2 years	3 years
All	67	20.9	71.6	82.1	85.1	39,310	31.5	58.3	63.4	65.0
Ethnicity/Race*										
White	52	21.2	76.9	88.5	88.5	26,919	32.3	59.3	64.1	65.6
African-American	10	30.0	70.0	80.0	80.0	2,875	35.0	60.1	65.0	66.5
Hispanic/Latino	4	0.0	0.0	0.0	50.0	7,094	27.8	54.4	60.1	61.8
Asian	1	0.0	100.0	100.0	100.0	1,719	27.4	55.7	63.4	65.3
Other	0	--	--	--	--	701	33.2	56.5	61.9	63.8
Unknown	0	--	--	--	--	2	0.0	0.0	0.0	0.0
Age										
<2 years	29	24.1	69.0	82.8	86.2	782	26.0	71.0	74.0	75.1
2-11 years	20	25.0	85.0	85.0	90.0	588	29.4	67.3	71.4	73.8
12-17 years	10	10.0	70.0	80.0	80.0	501	27.9	62.1	67.5	68.5
18-34 years	8	12.5	50.0	75.0	75.0	2,627	43.6	62.5	66.7	67.9
35-49 years	0	--	--	--	--	8,163	43.6	63.5	67.3	68.5
50-64 years	0	--	--	--	--	18,162	29.8	57.5	63.0	64.8
65-69 years	0	--	--	--	--	6,238	21.1	52.6	58.9	60.5
70+ years	0	--	--	--	--	2,249	19.5	48.5	54.3	55.4
Gender										
Male	36	11.1	75.0	86.1	86.1	24,261	31.4	59.2	64.4	65.9
Female	31	32.3	67.7	77.4	83.9	15,049	31.6	56.7	61.7	63.4

* Race and ethnicity are reported together as a single data element, reflecting their data collection (either race or ethnicity is required, but not both). Patients formerly coded as white and Hispanic are coded as Hispanic. Race and ethnicity sum to 100%.



B. Waiting List Information

Table B9. Percent of candidates with deceased donor transplants: medical characteristics

Candidates registered on the waiting list between 01/01/2020 and 12/31/2022

Characteristic	N	Percent transplanted at time periods since listing									
		This Center				N	United States				
		30 day	1 year	2 years	3 years		30 day	1 year	2 years	3 years	
All	67	20.9	71.6	82.1	85.1	39,310	31.5	58.3	63.4	65.0	
Blood Type											
O	30	20.0	66.7	86.7	86.7	18,395	30.2	56.1	61.5	63.1	
A	27	22.2	66.7	70.4	77.8	14,547	30.7	56.2	61.5	63.2	
B	7	28.6	100.0	100.0	100.0	4,794	34.5	66.3	70.7	72.0	
AB	3	0.0	100.0	100.0	100.0	1,574	45.2	77.9	80.2	80.7	
Previous Transplant											
Yes	6	16.7	50.0	66.7	66.7	1,794	36.0	58.2	62.4	64.4	
No	61	21.3	73.8	83.6	86.9	37,516	31.3	58.3	63.4	65.0	
Primary Disease											
Acute Hepatic Necrosis	3	100.0	100.0	100.0	100.0	1,914	59.5	67.8	68.6	68.8	
Non-Cholestatic Cirrhosis	3	0.0	66.7	66.7	66.7	24,341	32.8	57.7	62.3	63.8	
Cholestatic Liver Disease/Cirrhosis	0	--	--	--	--	2,417	25.9	56.1	63.6	66.1	
Biliary Atresia	8	37.5	87.5	87.5	87.5	632	18.4	70.1	74.8	76.6	
Metabolic Diseases	33	12.1	72.7	90.9	90.9	722	30.1	70.4	76.7	78.0	
Malignant Neoplasms	3	33.3	66.7	66.7	66.7	4,443	8.9	48.8	58.1	60.3	
Other	17	17.6	58.8	64.7	76.5	4,830	39.6	63.6	67.8	69.5	
Missing	0	--	--	--	--	11	45.5	54.5	63.6	63.6	
Medical Urgency Status/MELD/PELD at Listing*											
Status 1	0	--	--	--	--	0	--	--	--	--	
Status 1A	5	100.0	100.0	100.0	100.0	1,141	62.3	62.5	62.5	62.5	
Status 1B	4	25.0	75.0	75.0	75.0	149	50.3	79.2	79.2	79.2	
Status 2A	0	--	--	--	--	0	--	--	--	--	
Status 2B	0	--	--	--	--	0	--	--	--	--	
Status 3	0	--	--	--	--	0	--	--	--	--	
MELD 6-10	7	0.0	57.1	71.4	71.4	5,912	4.0	38.4	48.2	50.9	
MELD 11-14	1	0.0	100.0	100.0	100.0	4,528	4.9	37.0	46.9	50.0	
MELD 15-20	4	0.0	75.0	75.0	75.0	8,047	13.0	48.9	56.0	58.3	
MELD 21-30	4	0.0	25.0	75.0	75.0	9,720	39.9	69.6	72.1	72.8	
MELD 31-40	0	--	--	--	--	5,423	82.4	85.9	86.0	86.0	
PELD less than or equal to 10	29	6.9	69.0	82.8	86.2	565	13.6	69.9	76.3	79.1	
PELD 11-14	1	0.0	100.0	100.0	100.0	99	16.2	76.8	82.8	84.8	
PELD 15-20	5	40.0	80.0	80.0	100.0	129	21.7	82.9	83.7	85.3	
PELD 21-30	3	66.7	100.0	100.0	100.0	124	32.3	72.6	75.0	75.0	
PELD 31 or greater	4	50.0	75.0	75.0	75.0	57	54.4	71.9	71.9	71.9	
Temporarily Inactive	0	--	--	--	--	3,416	45.8	60.1	63.7	65.1	

* MELD/PELD score based on laboratory measures is shown for listings beginning 2/27/2002 unless patient is Status 1 or Temporarily Inactive. MELD/PELD scores based on exception rules are not used. Status 1 separated into 1A and 1B in August 2005.



B. Waiting List Information

Table B10. Time to transplant for waiting list candidates*

Candidates registered on the waiting list between 01/01/2020 and 06/30/2025

Percentile	Center	Months to Transplant**		U.S.
		OPO/DSA	Region	
5th	0.3	0.1	0.1	0.1
10th	0.7	0.2	0.2	0.2
25th	1.5	0.7	0.5	0.5
50th (median time to transplant)	4.3	3.8	6.1	4.0
75th	10.9	24.8	Not Observed	Not Observed

* If cells contain "Not Observed" fewer than that percentile of patients had received a transplant. For example, the 50th percentile of time to transplant is the time when 50% of candidates have received transplants. If waiting times are long, then the 50th percentile may not be observed during the follow-up period for this table. Also, if more than 50% of candidates are removed from the list due to death or other reasons before receiving transplants, then the 50th percentile of time to transplant will not be observed.

** Censored on 12/31/2025. Calculated as the months after listing, during which the corresponding percent of all patients initially listed had received a transplant.



B. Waiting List Information

Table B11. Offer Acceptance Practices: 01/01/2025 - 12/31/2025

Offers Acceptance Characteristics	This Center	OPO/DSA	Region	U.S.
Overall				
Number of Offers	1,319	4,570	34,682	235,608
Number of Acceptances	20	143	836	10,311
Expected Acceptances	36.4	240.9	1,229.8	10,310.0
Offer Acceptance Ratio*	0.57	0.60	0.68	1.00
95% Credible Interval**	[0.36, 0.84]	--	--	--
PHS increased infectious risk				
Number of Offers	159	433	3,577	21,582
Number of Acceptances	0	18	120	1,417
Expected Acceptances	2.4	26.5	161.7	1,414.2
Offer Acceptance Ratio*	0.45	0.70	0.75	1.00
95% Credible Interval**	[0.05, 1.26]	--	--	--
DCD donor				
Number of Offers	55	2,102	19,214	143,677
Number of Acceptances	1	59	281	4,151
Expected Acceptances	3.4	112.1	454.1	4,152.2
Offer Acceptance Ratio*	0.55	0.53	0.62	1.00
95% Credible Interval**	[0.11, 1.33]	--	--	--
HCV+ donor				
Number of Offers	0	90	774	6,087
Number of Acceptances	0	1	26	269
Expected Acceptances	0.0	4.4	36.5	269.6
Offer Acceptance Ratio*	--	0.47	0.73	1.00
95% Credible Interval**	[--, --]	--	--	--
Hard-to-Place Livers (Over 50 Offers)				
Number of Offers	63	1,402	16,533	112,282
Number of Acceptances	0	8	71	1,037
Expected Acceptances	0.6	28.7	188.9	1,220.8
Offer Acceptance Ratio*	0.78	0.33	0.38	0.85
95% Credible Interval**	[0.10, 2.19]	--	--	--
Donor more than 500 miles away				
Number of Offers	313	562	3,186	53,033
Number of Acceptances	3	12	64	1,220
Expected Acceptances	5.5	17.6	90.7	1,091.7
Offer Acceptance Ratio*	0.66	0.71	0.71	1.12
95% Credible Interval**	[0.22, 1.36]	--	--	--

* The offer acceptance ratio estimates the relative offer acceptance practice of UPMC Children's Hospital of Pittsburgh compared to the national offer acceptance practice. A ratio above one indicates the program accepts more offers compared to national offer acceptance practices (e.g., an offer acceptance ratio of 1.25 indicates a center accepts 25% more offers than is expected based on national offer acceptance practices), while a ratio below one indicates the program accepts fewer offers compared to national offer acceptance practices (e.g., an offer acceptance ratio of 0.75 indicates a center accepts 25% fewer offers than is expected based on national offer acceptance practices).

** As an example, the 95% Credible Interval for the overall offer acceptance ratio, [0.36, 0.84], indicates the location of PACH's true offer acceptance ratio with 95% probability. The best estimate is 43% less likely to accept an offer compared to national acceptance behavior, but PACH's performance could plausibly range from 64% reduced acceptance up to 16% reduced acceptance.



B. Waiting List Information

Figure B10. Offer acceptance: Overall

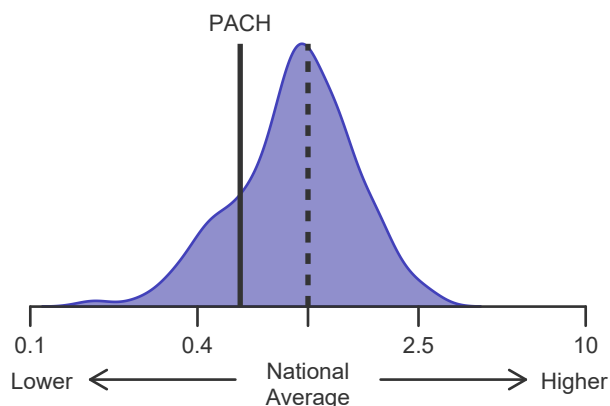


Figure B11. Offer acceptance:
PHS increased infectious risk

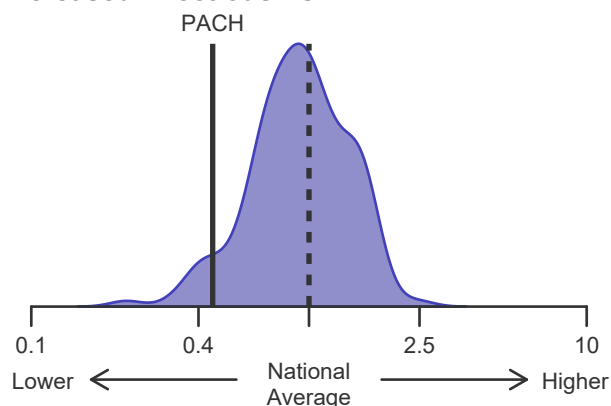


Figure B12. Offer acceptance: DCD Donor

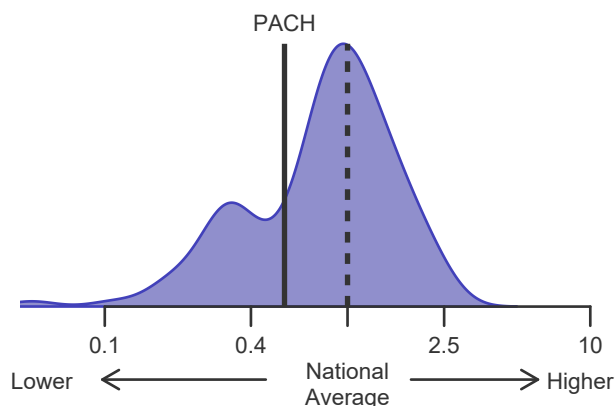


Figure B13. Offer acceptance: HCV+ Donor

This program received no offers.

Figure B14. Offer acceptance: Offer number > 50

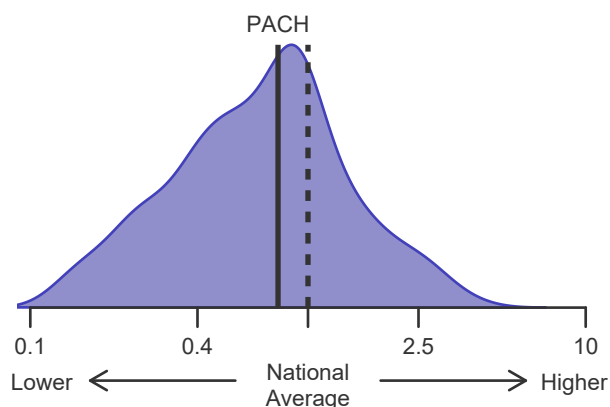
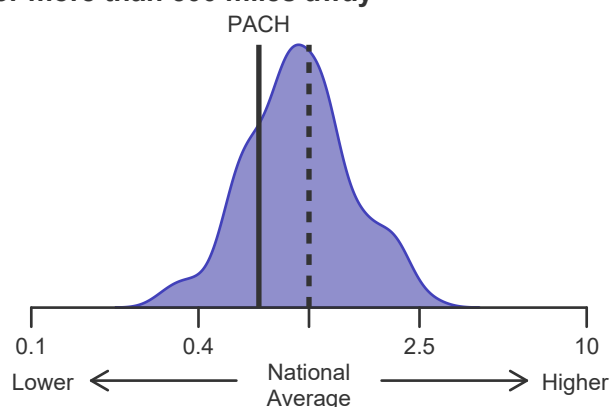


Figure B15. Offer acceptance:
Donor more than 500 miles away





C. Transplant Information

Table C1D. Deceased donor transplant recipient demographic characteristics

Patients transplanted between 01/01/2025 and 12/31/2025

Characteristic	Percentage in each category		
	Center (N=22)	Region (N=897)	U.S. (N=10,771)
Ethnicity/Race (%)*			
White	77.3	71.6	66.3
African-American	0.0	11.9	6.8
Hispanic/Latino	9.1	9.6	18.0
Asian	4.5	3.8	4.1
Other	4.5	0.7	2.1
Unknown	4.5	2.5	2.7
Age (%)			
<2 years	13.6	1.7	1.6
2-11 years	54.5	2.8	1.5
12-17	22.7	1.2	1.1
18-34	4.5	6.1	7.1
35-49 years	4.5	24.4	24.0
50-64 years	0.0	42.4	42.8
65-69 years	0.0	15.7	15.1
70+ years	0.0	5.7	6.9
Gender (%)			
Male	36.4	60.3	59.2
Female	63.6	39.7	40.8

* Race and ethnicity are reported together as a single data element, reflecting their data collection (either race or ethnicity is required, but not both). Patients formerly coded as white and Hispanic are coded as Hispanic. Race and ethnicity sum to 100%.



C. Transplant Information

Table C1L. Living donor transplant recipient demographic characteristics

Patients transplanted between 01/01/2025 and 12/31/2025

Characteristic	Percentage in each category		
	Center (N=7)	Region (N=173)	U.S. (N=707)
Ethnicity/Race (%)*			
White	85.7	89.0	74.5
African-American	0.0	5.2	4.1
Hispanic/Latino	0.0	2.9	15.1
Asian	0.0	2.3	3.5
Other	0.0	0.0	0.6
Unknown	14.3	0.6	2.1
Age (%)			
<2 years	57.1	6.4	9.9
2-11 years	42.9	4.0	3.7
12-17	0.0	0.0	1.3
18-34	0.0	8.7	8.6
35-49 years	0.0	25.4	21.8
50-64 years	0.0	34.1	36.2
65-69 years	0.0	13.9	12.4
70+ years	0.0	7.5	6.1
Gender (%)			
Male	57.1	50.9	49.5
Female	42.9	49.1	50.5

* Race and ethnicity are reported together as a single data element, reflecting their data collection (either race or ethnicity is required, but not both). Patients formerly coded as white and Hispanic are coded as Hispanic. Race and ethnicity sum to 100%.



C. Transplant Information

Table C2D. Deceased donor transplant recipient medical characteristics
Patients transplanted between 01/01/2025 and 12/31/2025

Characteristic	Percentage in each category		
	Center (N=22)	Region (N=897)	U.S. (N=10,771)
Blood Type (%)			
O	54.5	43.5	45.7
A	36.4	37.6	36.3
B	0.0	15.3	13.5
AB	9.1	3.7	4.5
Previous Transplant (%)			
Yes	0.0	4.8	4.2
No	100.0	95.2	95.8
Body Mass Index (%)			
0-20	63.6	11.5	9.4
21-25	22.7	26.4	25.9
26-30	0.0	30.5	30.5
31-35	0.0	17.7	19.3
36-40	0.0	8.4	8.6
41+	0.0	3.9	4.3
Unknown	13.6	1.6	1.9
Primary Disease (%)			
Acute Hepatic Necrosis	4.5	2.6	2.0
Non-Cholestatic Cirrhosis	0.0	7.0	10.2
Cholestatic Liver Disease/Cirrhosis	9.1	4.8	6.2
Biliary Atresia	4.5	1.2	1.5
Metabolic Diseases	59.1	3.7	2.1
Malignant Neoplasms	13.6	16.5	12.5
Other	9.1	64.1	65.5
Missing	0.0	0.1	0.1
Medical Urgency Statust/MELD/PELD at Transplant (%)*			
Status 1A	4.5	3.5	2.6
Status 1B	59.1	3.0	1.2
MELD 6-10	4.5	11.6	10.2
MELD 11-14	0.0	6.5	9.2
MELD 15-20	9.1	14.9	19.7
MELD 21-30	4.5	25.3	29.0
MELD 31-40	0.0	25.1	20.1
PELD less than or equal to 10	13.6	1.1	0.8
PELD 11-14	0.0	0.2	0.3
PELD 15-20	0.0	0.0	0.2
PELD 21-30	0.0	0.3	0.3
PELD 31 or greater	0.0	0.0	0.1
Temporarily Inactive	0.0	0.0	0.0
Recipient Medical Condition at Transplant (%)			
Not Hospitalized	72.7	51.6	59.7
Hospitalized	4.5	34.8	24.7
ICU	22.7	13.5	15.6
Unknown	0.0	0.1	0.0

* MELD/PELD score based on laboratory measures at the time of transplant is shown unless recipient is Status 1 or Temporarily Inactive. MELD/PELD scores based on exception rules are not used. Status 1 separated into 1A and 1B in August 2005



C. Transplant Information

Table C2L. Living donor transplant recipient medical characteristics
Patients transplanted between 01/01/2025 and 12/31/2025

Characteristic	Percentage in each category		
	Center (N=7)	Region (N=173)	U.S. (N=707)
Blood Type (%)			
O	57.1	41.6	44.4
A	28.6	47.4	44.1
B	14.3	9.2	9.3
AB	0.0	1.7	2.1
Previous Transplant (%)			
Yes	0.0	1.2	1.8
No	100.0	98.8	98.2
Body Mass Index (%)			
0-20	85.7	16.2	21.5
21-25	14.3	34.7	31.5
26-30	0.0	27.2	27.6
31-35	0.0	13.3	12.4
36-40	0.0	5.8	4.5
41+	0.0	2.3	1.7
Unknown	0.0	0.6	0.7
Primary Disease (%)			
Acute Hepatic Necrosis	0.0	0.0	0.4
Non-Cholestatic Cirrhosis	0.0	6.4	9.2
Cholestatic Liver Disease/Cirrhosis	42.9	23.7	18.0
Biliary Atresia	28.6	6.4	10.3
Metabolic Diseases	28.6	2.3	2.3
Malignant Neoplasms	0.0	17.3	12.4
Other	0.0	43.9	47.4
Missing	0.0	0.0	0.0
Medical Urgency Statust/MELD/PELD at Transplant (%)*			
Status 1A	0.0	0.0	0.7
Status 1B	14.3	1.2	1.7
MELD 6-10	0.0	25.4	23.8
MELD 11-14	0.0	23.1	21.8
MELD 15-20	0.0	26.0	24.3
MELD 21-30	0.0	10.4	11.2
MELD 31-40	0.0	0.6	0.8
PELD less than or equal to 10	28.6	4.6	4.8
PELD 11-14	0.0	1.2	1.6
PELD 15-20	0.0	0.6	1.7
PELD 21-30	28.6	1.7	2.1
PELD 31 or greater	14.3	0.6	0.8
Temporarily Inactive	14.3	4.6	4.7
Recipient Medical Condition at Transplant (%)			
Not Hospitalized	57.1	90.2	87.6
Hospitalized	42.9	8.7	10.2
ICU	0.0	1.2	2.1
Unknown	0.0	0.0	0.1

* MELD/PELD score based on laboratory measures at the time of transplant is shown unless recipient is Status 1 or Temporarily Inactive. MELD/PELD scores based on exception rules are not used. Status 1 separated into 1A and 1B in August 2005



C. Transplant Information

Table C3D. Deceased donor characteristics

Transplants performed between 01/01/2025 and 12/31/2025

Donor Characteristic	Percentage in each category		
	Center (N=22)	Region (N=897)	U.S. (N=10,771)
Cause of Death (%)			
Deceased: Stroke	27.3	27.9	29.4
Deceased: MVA	4.5	9.6	9.6
Deceased: Other	68.2	62.5	61.0
Ethnicity/Race (%)*			
White	72.7	64.3	63.7
African-American	13.6	22.7	17.1
Hispanic/Latino	9.1	8.7	13.9
Asian	4.5	2.3	3.1
Other	0.0	0.2	1.2
Not Reported	0.0	1.7	1.0
Age (%)			
<2 years	0.0	0.7	0.6
2-11 years	18.2	2.0	1.7
12-17	31.8	3.8	2.9
18-34	36.4	17.3	18.0
35-49 years	9.1	28.5	27.6
50-64 years	4.5	32.1	32.2
65-69 years	0.0	9.5	9.7
70+ years	0.0	6.1	7.3
Gender (%)			
Male	59.1	59.8	61.0
Female	40.9	40.2	39.0
Blood Type (%)			
O	59.1	48.4	48.9
A	36.4	38.0	36.7
B	4.5	12.0	11.7
AB	0.0	1.6	2.7
Unknown	0.0	0.0	0.0

* Race and ethnicity are reported together as a single data element, reflecting their data collection (either race or ethnicity is required, but not both). Patients formerly coded as white and Hispanic are coded as Hispanic. Race and ethnicity sum to 100%.



C. Transplant Information

Table C3L. Living donor characteristics

Transplants performed between 01/01/2025 and 12/31/2025

Donor Characteristic	Percentage in each category		
	Center (N=7)	Region (N=173)	U.S. (N=707)
Ethnicity/Race (%)*			
White	71.4	73.4	72.6
African-American	0.0	5.2	3.3
Hispanic/Latino	0.0	0.6	13.0
Asian	0.0	0.0	3.1
Other	0.0	0.6	1.1
Not Reported	28.6	20.2	6.9
Age (%)			
0-11 years	0.0	0.6	0.6
12-17	0.0	0.0	0.1
18-34	57.1	34.7	37.2
35-49 years	28.6	48.0	46.0
50-64 years	14.3	16.2	16.0
65-69 years	0.0	0.0	0.0
70+ years	0.0	0.6	0.1
Gender (%)			
Male	42.9	41.6	41.6
Female	57.1	58.4	58.4
Blood Type (%)			
O	71.4	59.5	62.0
A	28.6	35.3	32.5
B	0.0	5.2	5.0
AB	0.0	0.0	0.6
Unknown	0.0	0.0	0.0

* Race and ethnicity are reported together as a single data element, reflecting their data collection (either race or ethnicity is required, but not both). Patients formerly coded as white and Hispanic are coded as Hispanic. Race and ethnicity sum to 100%.



C. Transplant Information

Table C4D. Deceased donor transplant characteristics
Transplants performed between 01/01/2025 and 12/31/2025

Transplant Characteristic	Percentage in each category		
	Center (N=22)	Region (N=897)	U.S. (N=10,771)
Cold Ischemic Time (Hours): Local (%)			
Deceased: 0-5 hr	80.0	46.1	32.5
Deceased: 6-10 hr	0.0	18.6	16.9
Deceased: 11-15 hr	0.0	15.9	17.3
Deceased: 16-20 hr	20.0	13.6	20.6
Deceased: 21+ hr	0.0	5.1	12.1
Not Reported	0.0	0.7	0.6
Cold Ischemic Time (Hours): Shared (%)			
Deceased: 0-5 hr	64.7	19.6	18.2
Deceased: 6-10 hr	23.5	41.2	28.8
Deceased: 11-15 hr	5.9	17.1	17.6
Deceased: 16-20 hr	5.9	13.5	20.9
Deceased: 21+ hr	0.0	8.5	14.2
Not Reported	0.0	0.2	0.4
Procedure Type (%)			
Single organ	90.9	97.1	98.3
Multi organ	9.1	2.9	1.7
Donor Location (%)			
Local Donation Service Area (DSA)	22.7	32.9	36.8
Another Donation Service Area (DSA)	77.3	67.1	63.2
Median Time in Hospital After Transplant	25.0 Days	11.5 Days	10.0 Days



C. Transplant Information

Table C4L. Living donor transplant characteristics
Transplants performed between 01/01/2025 and 12/31/2025

Transplant Characteristic	Percentage in each category		
	Center (N=7)	Region (N=173)	U.S. (N=707)
Relation with Donor (%)			
Related	71.4	46.8	48.4
Unrelated	28.6	52.0	51.2
Not Reported	0.0	1.2	0.4
Procedure Type (%)			
Single organ	100.0	99.4	99.9
Multi organ	0.0	0.6	0.1
Median Time in Hospital After Transplant	26.0 Days	9.0 Days	10.0 Days



C. Transplant Information

Table C5. Adult (18+) 1-month survival with a functioning graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	7	24,637
Estimated probability of surviving with a functioning graft at 1 month & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	97.29% [97.09%-97.50%]
Expected probability of surviving with a functioning graft at 1 month (adjusted for patient and donor characteristics)	98.16%	--
Number of observed graft failures (including deaths) during the first month after transplant	0	667
Number of expected graft failures (including deaths) during the first month after transplant	0.13	--
Estimated hazard ratio*	0.94	--
95% credible interval for the hazard ratio**	[0.11, 2.62]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.11, 2.62], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 6% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 89% reduced risk up to 162% increased risk.

Figure C1. Adult (18+) 1-month graft failure HR estimate

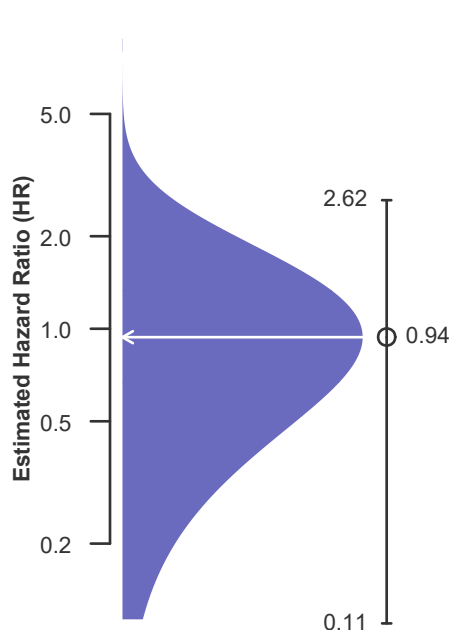
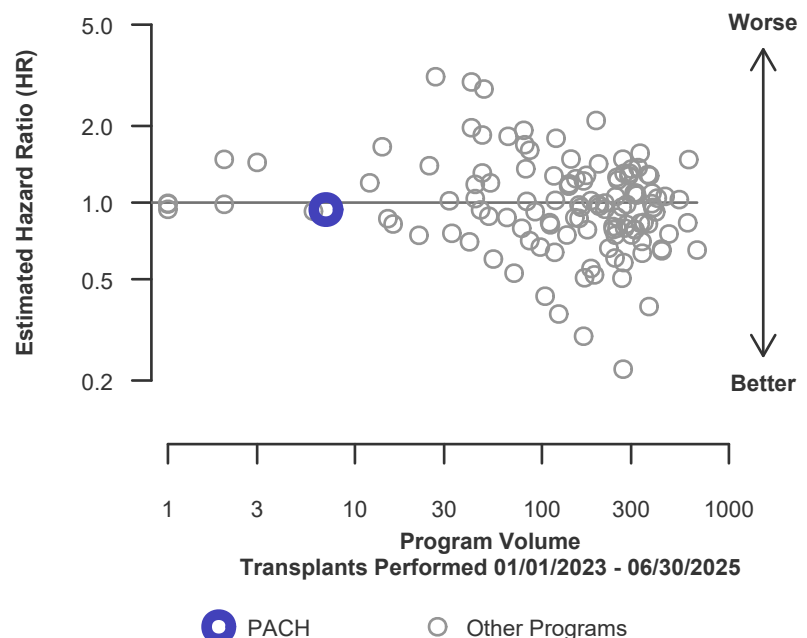


Figure C2. Adult (18+) 1-month graft failure HR program comparison





C. Transplant Information

Table C5D. Adult (18+) 1-month survival with a functioning deceased donor graft

Single organ transplants performed between 01/01/2023 and 06/30/2025

Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	5	23,288
Estimated probability of surviving with a functioning graft at 1 month & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	97.31% [97.10%-97.52%]
Expected probability of surviving with a functioning graft at 1 month (adjusted for patient and donor characteristics)	98.57%	--
Number of observed graft failures (including deaths) during the first month after transplant	0	626
Number of expected graft failures (including deaths) during the first month after transplant	0.07	--
Estimated hazard ratio*	0.97	--
95% credible interval for the hazard ratio**	[0.12, 2.69]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.12, 2.69], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 3% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 88% reduced risk up to 169% increased risk.

Figure C1D. Adult (18+) 1-month deceased donor graft failure HR estimate

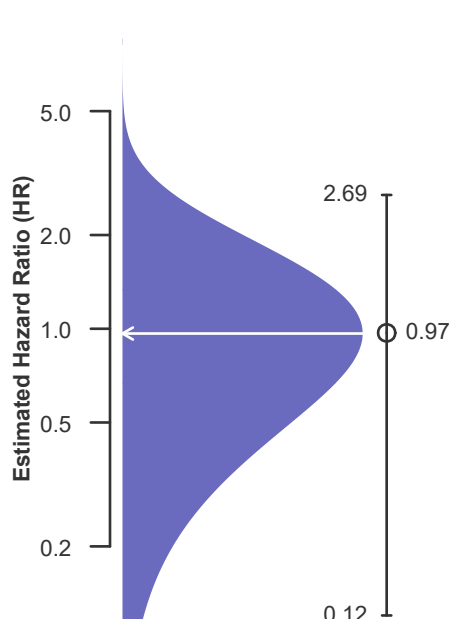
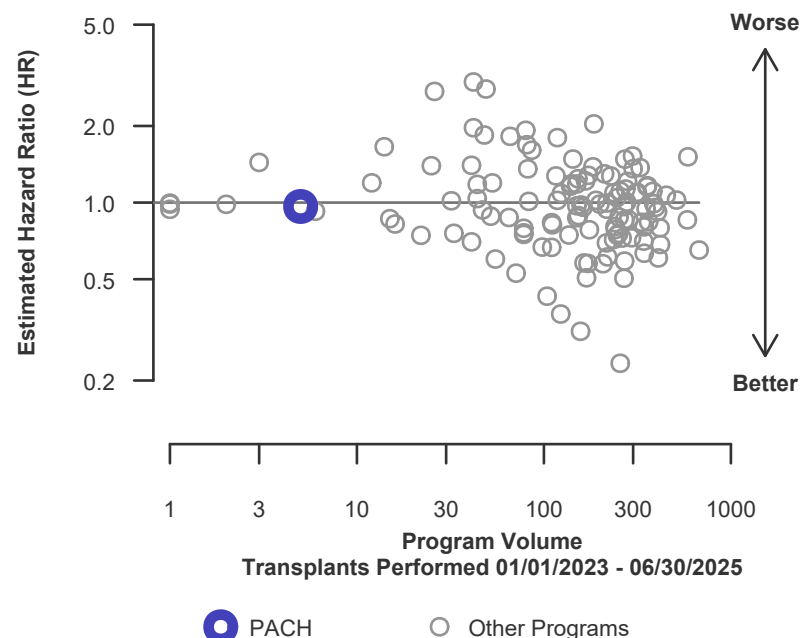


Figure C2D. Adult (18+) 1-month deceased donor graft failure HR program comparison





C. Transplant Information

Table C5L. Adult (18+) 1-month survival with a functioning living donor graft

Single organ transplants performed between 01/01/2023 and 06/30/2025

Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	2	1,349
Estimated probability of surviving with a functioning graft at 1 month & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	96.96% [96.05%-97.88%]
Expected probability of surviving with a functioning graft at 1 month (adjusted for patient and donor characteristics)	97.12%	--
Number of observed graft failures (including deaths) during the first month after transplant	0	41
Number of expected graft failures (including deaths) during the first month after transplant	0.06	--
Estimated hazard ratio*	0.97	--
95% credible interval for the hazard ratio**	[0.12, 2.71]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.12, 2.71], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 3% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 88% reduced risk up to 171% increased risk.

Figure C1L. Adult (18+) 1-month living donor graft failure HR estimate

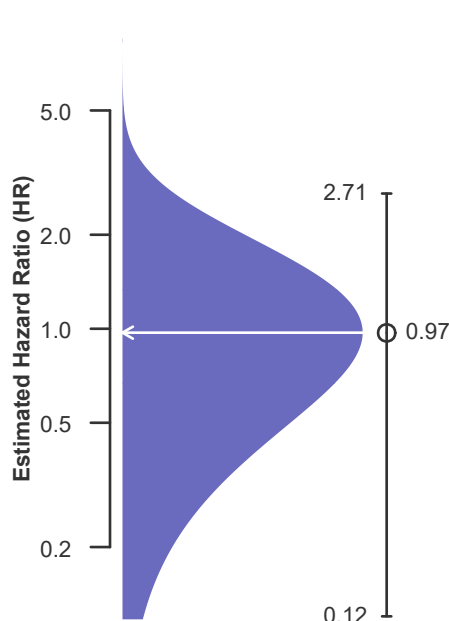
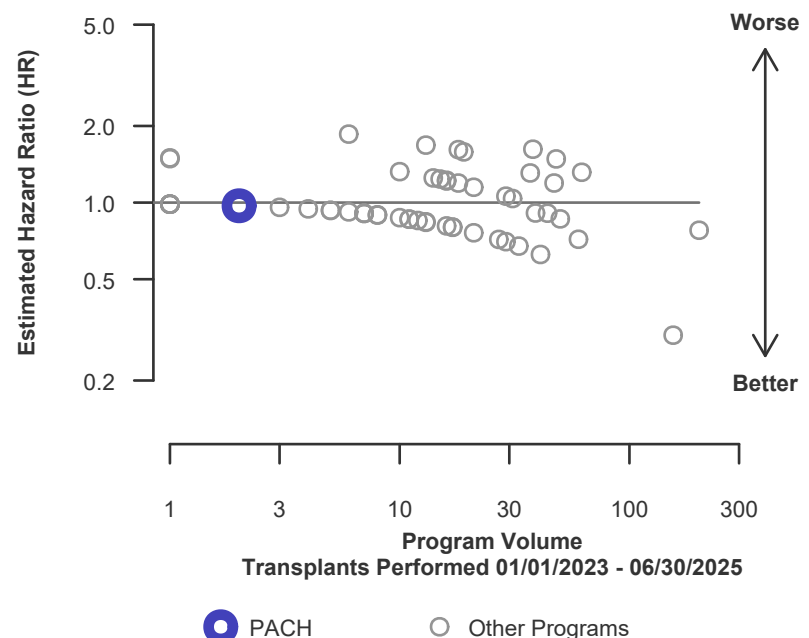


Figure C2L. Adult (18+) 1-month living donor graft failure HR program comparison





C. Transplant Information

Table C6. Adult (18+) 90-Day survival with a functioning graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	7	24,637
Estimated probability of surviving with a functioning graft at 90 days & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	95.59% [95.34%-95.85%]
Expected probability of surviving with a functioning graft at 90 days (adjusted for patient and donor characteristics)	97.16%	--
Number of observed graft failures (including deaths) during the first 90 days after transplant	0	1,086
Number of expected graft failures (including deaths) during the first 90 days after transplant	0.20	--
Estimated hazard ratio*	0.91	--
95% credible interval for the hazard ratio**	[0.11, 2.53]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.11, 2.53], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 9% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 89% reduced risk up to 153% increased risk.

Figure C3. Adult (18+) 90-Day graft failure HR estimate

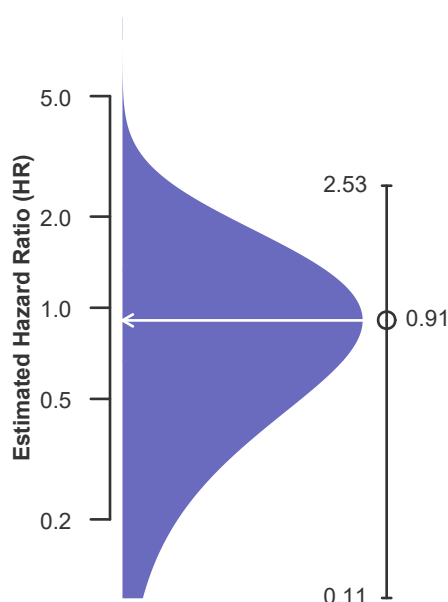
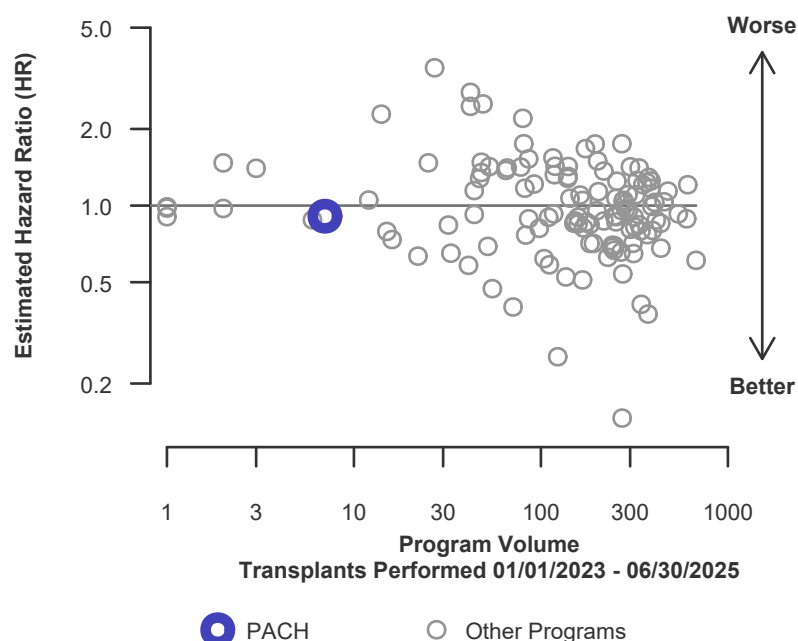


Figure C4. Adult (18+) 90-Day graft failure HR program comparison





C. Transplant Information

Table C6D. Adult (18+) 90-Day survival with a functioning deceased donor graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	5	23,288
Estimated probability of surviving with a functioning graft at 90 days & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	95.58% [95.31%-95.84%]
Expected probability of surviving with a functioning graft at 90 days (adjusted for patient and donor characteristics)	97.62%	--
Number of observed graft failures (including deaths) during the first 90 days after transplant	0	1,030
Number of expected graft failures (including deaths) during the first 90 days after transplant	0.12	--
Estimated hazard ratio*	0.94	--
95% credible interval for the hazard ratio**	[0.11, 2.63]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.11, 2.63], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 6% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 89% reduced risk up to 163% increased risk.

Figure C3D. Adult (18+) 90-Day deceased donor graft failure HR estimate

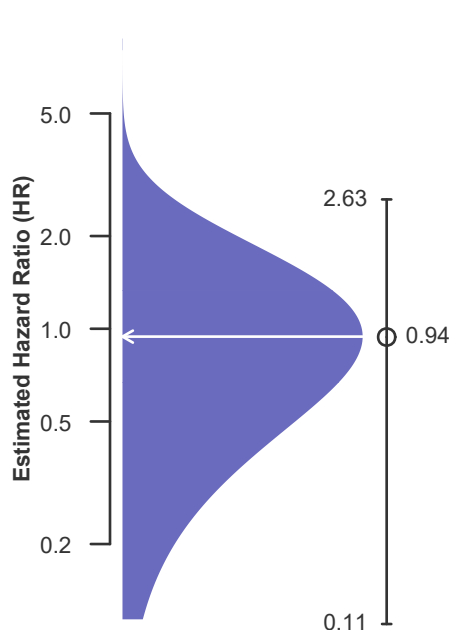
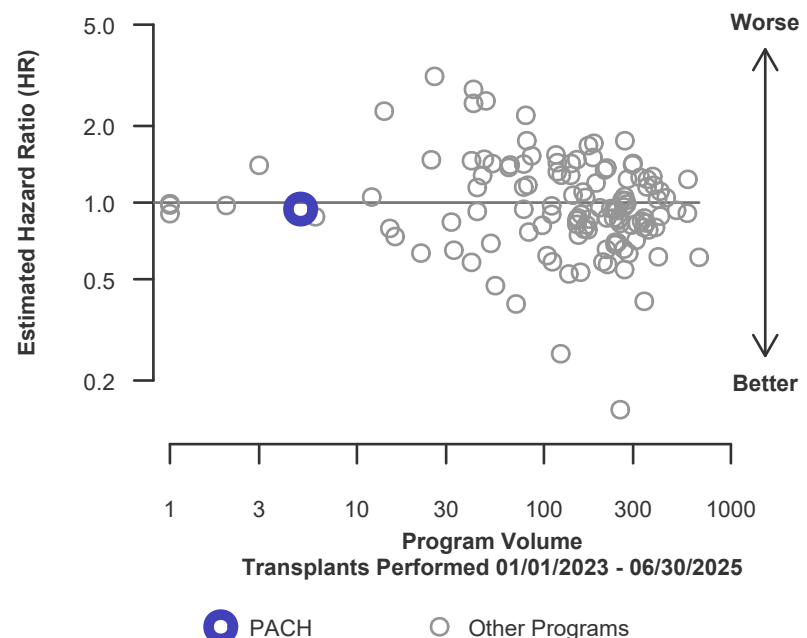


Figure C4D. Adult (18+) 90-Day deceased donor graft failure HR program comparison





C. Transplant Information

Table C6L. Adult (18+) 90-Day survival with a functioning living donor graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	2	1,349
Estimated probability of surviving with a functioning graft at 90 days & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	95.85% [94.79%-96.92%]
Expected probability of surviving with a functioning graft at 90 days (adjusted for patient and donor characteristics)	96.03%	--
Number of observed graft failures (including deaths) during the first 90 days after transplant	0	56
Number of expected graft failures (including deaths) during the first 90 days after transplant	0.08	--
Estimated hazard ratio*	0.96	--
95% credible interval for the hazard ratio**	[0.12, 2.68]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.12, 2.68], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 4% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 88% reduced risk up to 168% increased risk.

Figure C3L. Adult (18+) 90-Day living donor graft failure HR estimate

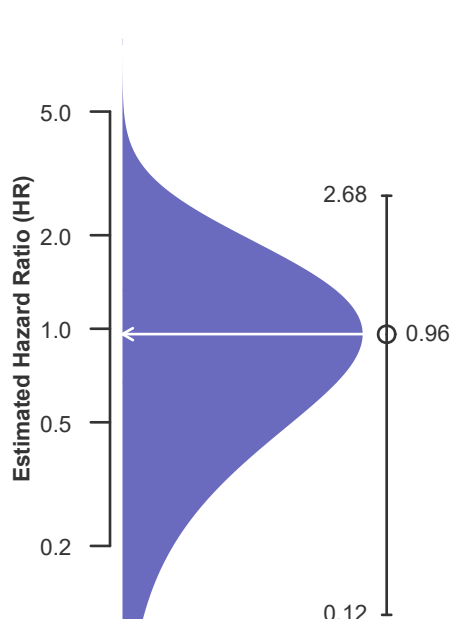
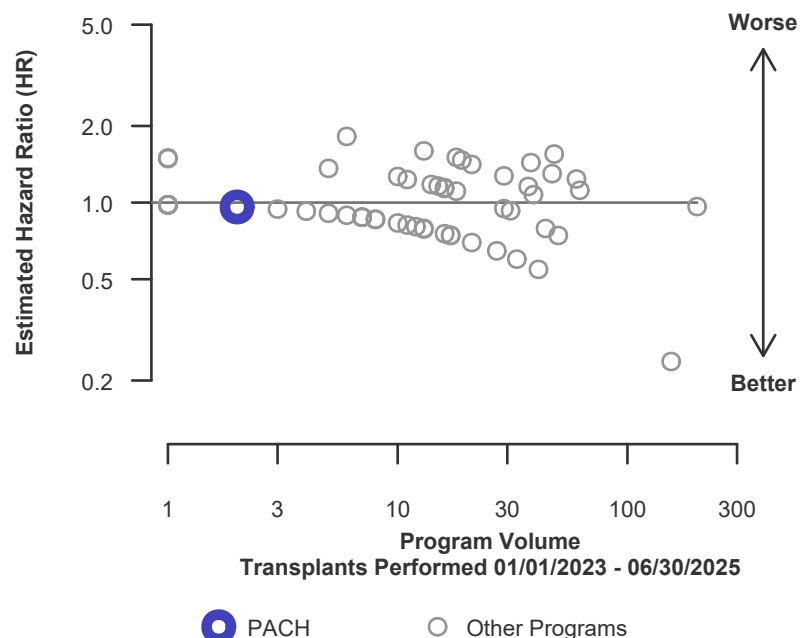


Figure C4L. Adult (18+) 90-Day living donor graft failure HR program comparison





C. Transplant Information

Table C7. Adult (18+) 1-year survival with a functioning graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	7	24,637
Estimated probability of surviving with a functioning graft at 1 year & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	92.29% [91.94%-92.63%]
Expected probability of surviving with a functioning graft at 1 year (adjusted for patient and donor characteristics)	95.22%	--
Number of observed graft failures (including deaths) during the first year after transplant	0	1,798
Number of expected graft failures (including deaths) during the first year after transplant	0.34	--
Estimated hazard ratio*	0.85	--
95% credible interval for the hazard ratio**	[0.10, 2.38]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.10, 2.38], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 15% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 90% reduced risk up to 138% increased risk.

Figure C5. Adult (18+) 1-year graft failure HR estimate

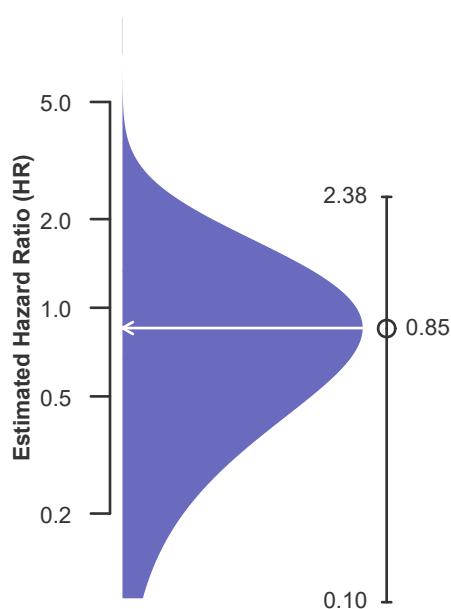
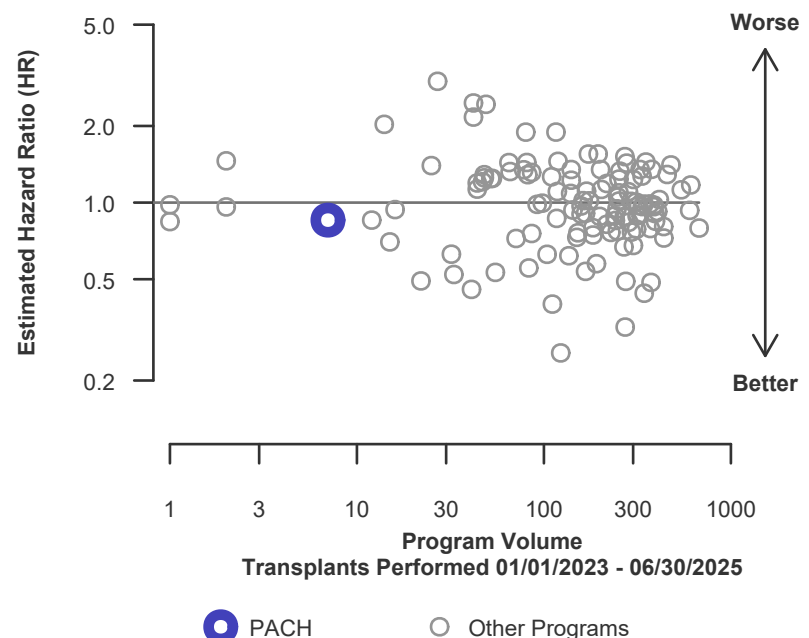


Figure C6. Adult (18+) 1-year graft failure HR program comparison





C. Transplant Information

Table C7D. Adult (18+) 1-year survival with a functioning deceased donor graft

Single organ transplants performed between 01/01/2023 and 06/30/2025

Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	5	23,288
Estimated probability of surviving with a functioning graft at 1 year & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	92.21% [91.86%-92.57%]
Expected probability of surviving with a functioning graft at 1 year (adjusted for patient and donor characteristics)	95.80%	--
Number of observed graft failures (including deaths) during the first year after transplant	0	1,715
Number of expected graft failures (including deaths) during the first year after transplant	0.22	--
Estimated hazard ratio*	0.90	--
95% credible interval for the hazard ratio**	[0.11, 2.52]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.11, 2.52], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 10% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 89% reduced risk up to 152% increased risk.

Figure C5D. Adult (18+) 1-year deceased donor graft failure HR estimate

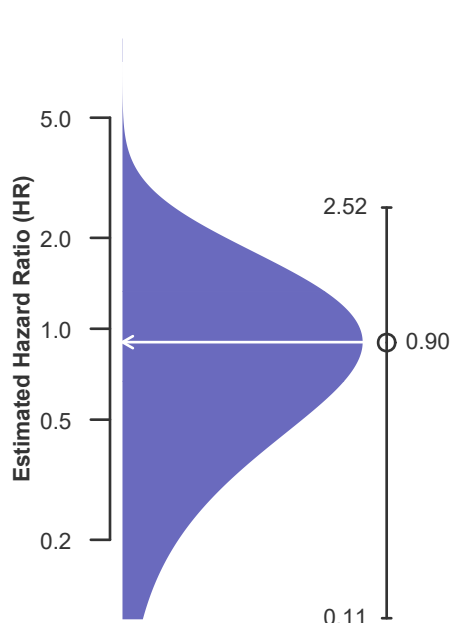
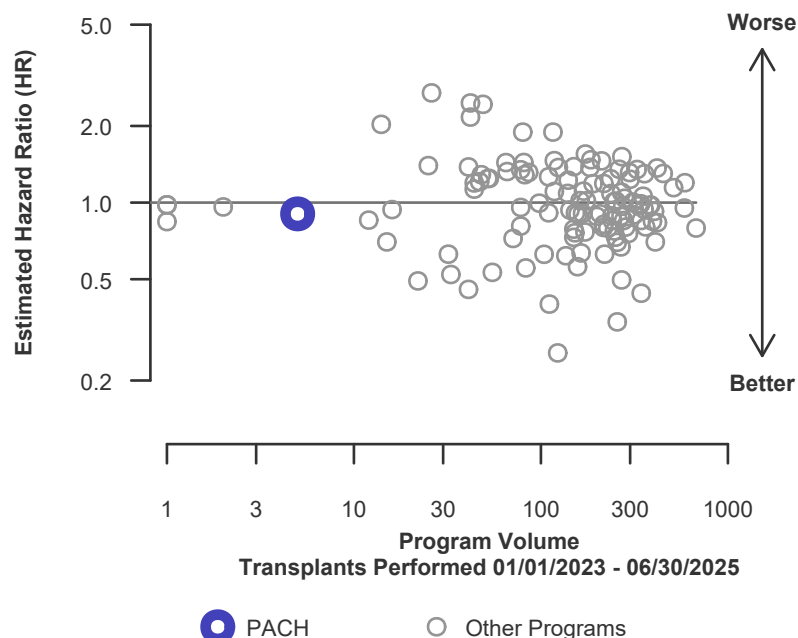


Figure C6D. Adult (18+) 1-year deceased donor graft failure HR program comparison





C. Transplant Information

Table C7L. Adult (18+) 1-year survival with a functioning living donor graft

Single organ transplants performed between 01/01/2023 and 06/30/2025

Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	2	1,349
Estimated probability of surviving with a functioning graft at 1 year & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	93.55% [92.21%-94.91%]
Expected probability of surviving with a functioning graft at 1 year (adjusted for patient and donor characteristics)	93.76%	--
Number of observed graft failures (including deaths) during the first year after transplant	0	83
Number of expected graft failures (including deaths) during the first year after transplant	0.13	--
Estimated hazard ratio*	0.94	--
95% credible interval for the hazard ratio**	[0.11, 2.62]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.11, 2.62], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 6% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 89% reduced risk up to 162% increased risk.

Figure C5L. Adult (18+) 1-year living donor graft failure HR estimate

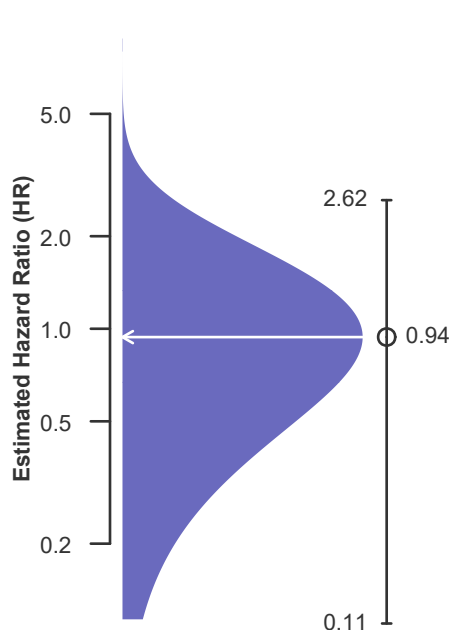
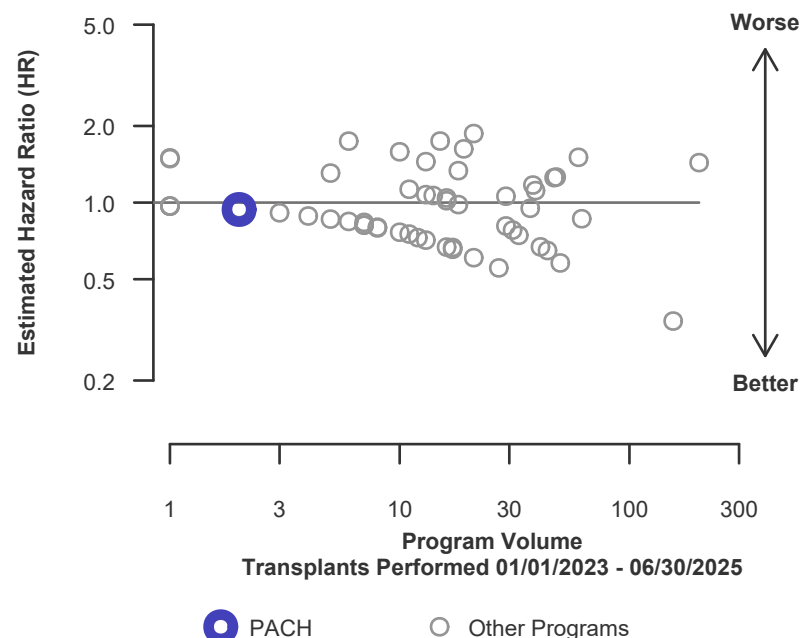


Figure C6L. Adult (18+) 1-year living donor graft failure HR program comparison





C. Transplant Information

Table C8. Adult (18+) 1-year Conditional survival with a functioning graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	7	23,551
Estimated probability of surviving with a functioning graft at 1 year, among patients with a functioning graft at day 90 & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	96.54% [96.44%-96.65%]
Expected probability of surviving with a functioning graft at 1 year, among patients with a functioning graft at day 90 (adjusted for patient and donor characteristics)	98.00%	--
Number of observed graft failures (including deaths) from day 91 through day 365 after transplant	0	712
Number of expected graft failures (including deaths) from day 91 through day 365 after transplant	0.14	--
Estimated hazard ratio*	0.93	--
95% credible interval for the hazard ratio**	[0.11, 2.60]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.11, 2.60], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 7% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 89% reduced risk up to 160% increased risk.

Figure C7. Adult (18+) 1-year Conditional graft failure HR estimate

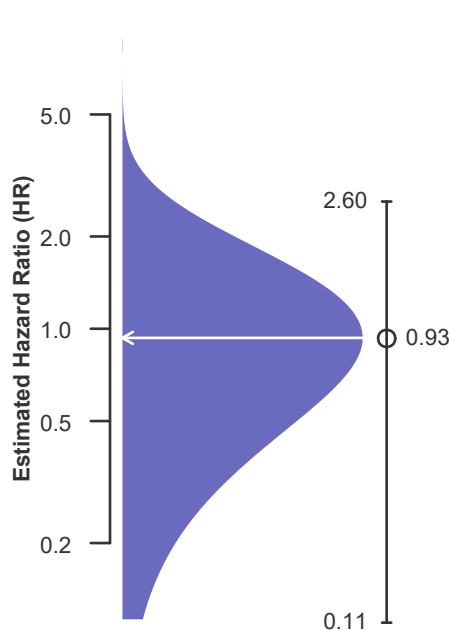
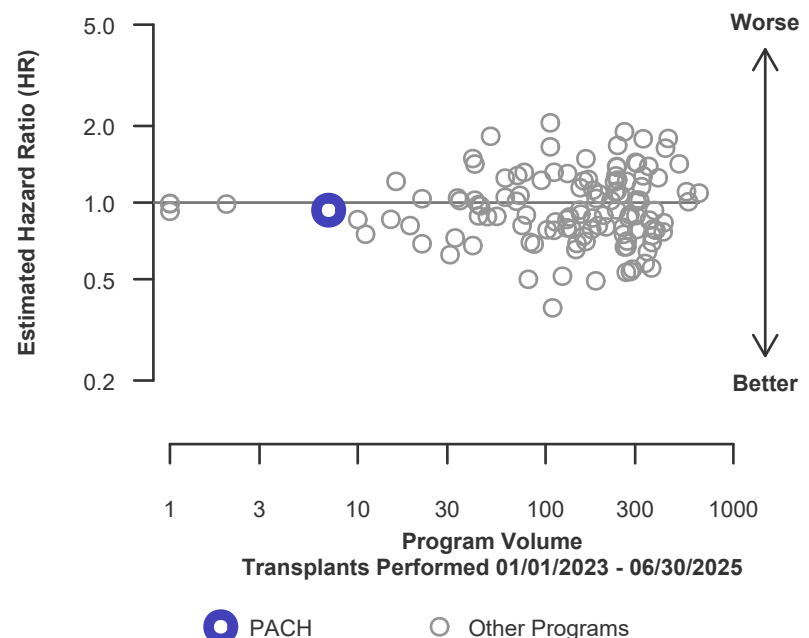


Figure C8. Adult (18+) 1-year Conditional graft failure HR program comparison





C. Transplant Information

Table C8D. Adult (18+) 1-year Conditional survival with a functioning deceased donor graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	5	22,258
Estimated probability of surviving with a functioning graft at 1 year, among patients with a functioning graft at day 90 & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	96.48% [96.37%-96.59%]
Expected probability of surviving with a functioning graft at 1 year, among patients with a functioning graft at day 90 (adjusted for patient and donor characteristics)	98.14%	--
Number of observed graft failures (including deaths) from day 91 through day 365 after transplant	0	685
Number of expected graft failures (including deaths) from day 91 through day 365 after transplant	0.09	--
Estimated hazard ratio*	0.96	--
95% credible interval for the hazard ratio**	[0.12, 2.66]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.12, 2.66], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 4% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 88% reduced risk up to 166% increased risk.

Figure C7D. Adult (18+) 1-year Conditional deceased donor graft failure HR estimate

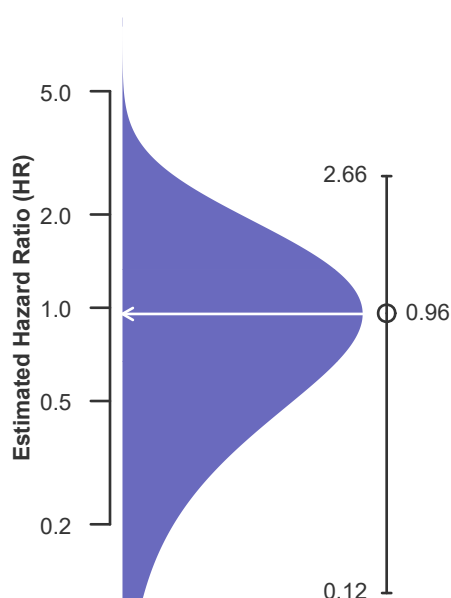
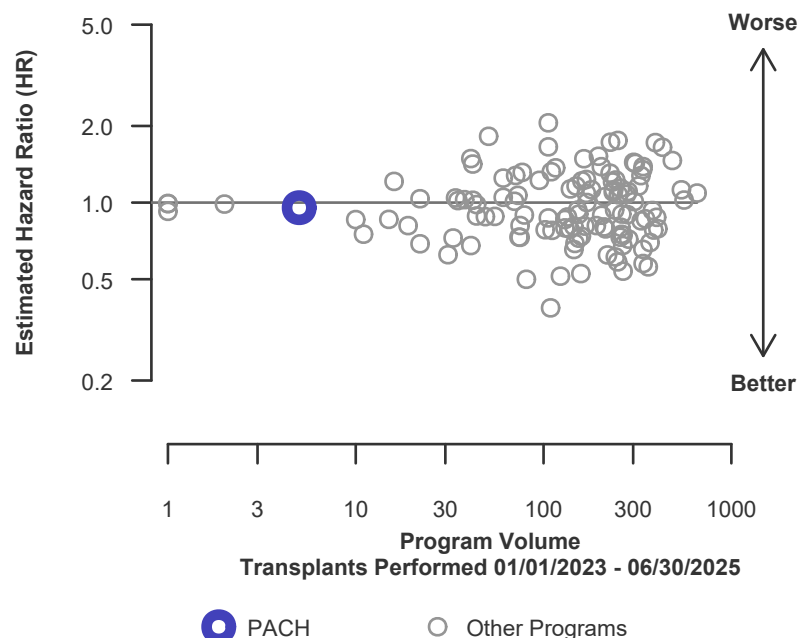


Figure C8D. Adult (18+) 1-year Conditional deceased donor graft failure HR program comparison





C. Transplant Information

Table C8L. Adult (18+) 1-year Conditional survival with a functioning living donor graft

Single organ transplants performed between 01/01/2023 and 06/30/2025

Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	2	1,293
Estimated probability of surviving with a functioning graft at 1 year, among patients with a functioning graft at day 90 & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	97.60% [97.28%-97.93%]
Expected probability of surviving with a functioning graft at 1 year, among patients with a functioning graft at day 90 (adjusted for patient and donor characteristics)	97.63%	--
Number of observed graft failures (including deaths) from day 91 through day 365 after transplant	0	27
Number of expected graft failures (including deaths) from day 91 through day 365 after transplant	0.05	--
Estimated hazard ratio*	0.98	--
95% credible interval for the hazard ratio**	[0.12, 2.72]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.12, 2.72], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 2% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 88% reduced risk up to 172% increased risk.

Figure C7L. Adult (18+) 1-year Conditional living donor graft failure HR estimate

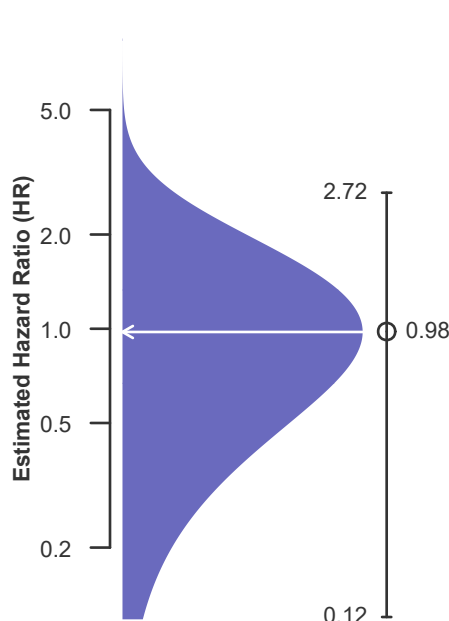
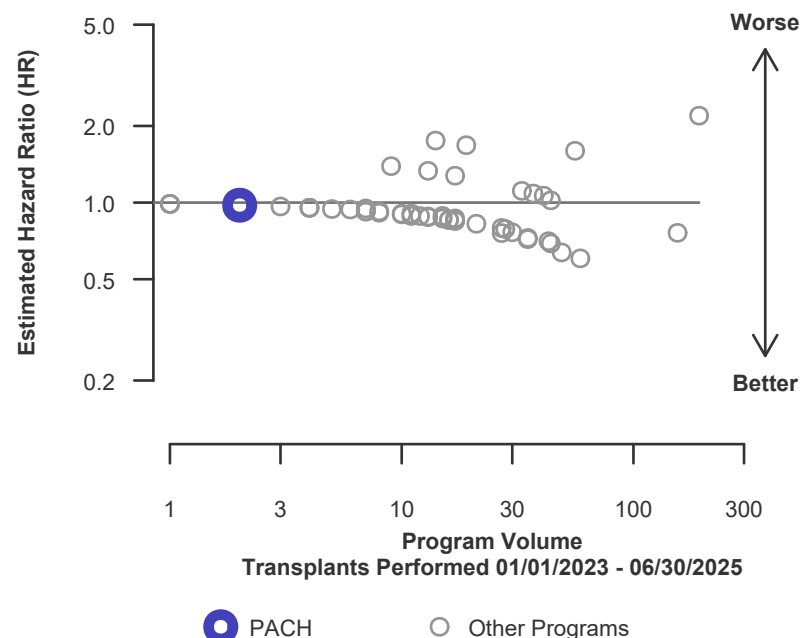


Figure C8L. Adult (18+) 1-year Conditional living donor graft failure HR program comparison





C. Transplant Information

Table C9. Adult (18+) 3-year survival with a functioning graft

Single organ transplants performed between 07/01/2020 and 12/31/2022

Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	2	19,856
Estimated probability of surviving with a functioning graft at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	86.50% [86.02%-86.97%]
Expected probability of surviving with a functioning graft at 3 years (adjusted for patient and donor characteristics)	92.00%	--
Number of observed graft failures (including deaths) during the first 3 years after transplant	0	2,681
Number of expected graft failures (including deaths) during the first 3 years after transplant	0.17	--
Estimated hazard ratio*	0.92	--
95% credible interval for the hazard ratio**	[0.11, 2.57]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.11, 2.57], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 8% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 89% reduced risk up to 157% increased risk.

Figure C9. Adult (18+) 3-year graft failure HR estimate

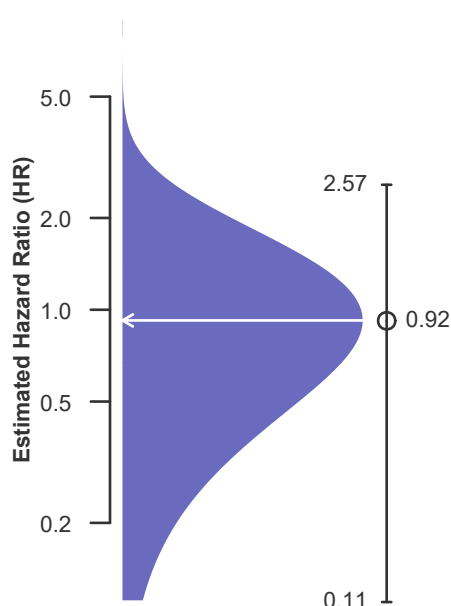
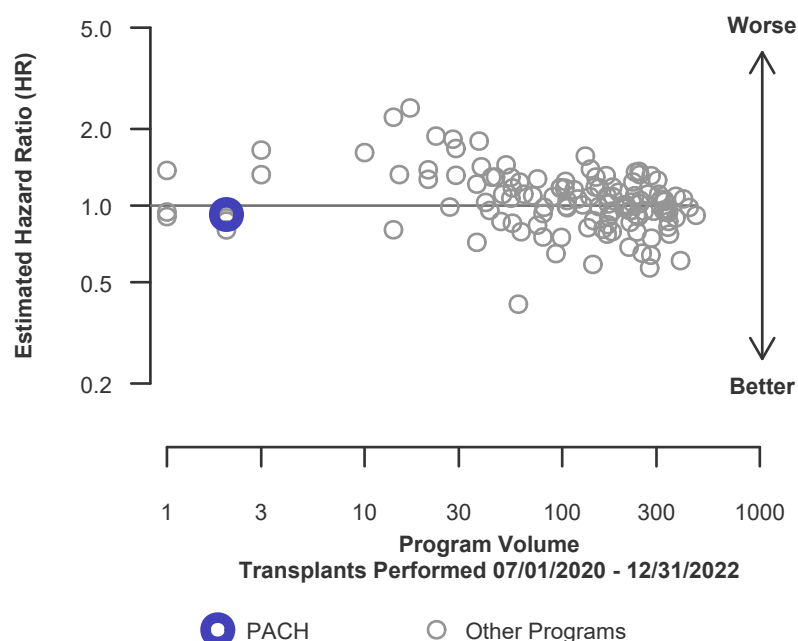


Figure C10. Adult (18+) 3-year graft failure HR program comparison





C. Transplant Information

Table C9D. Adult (18+) 3-year survival with a functioning deceased donor graft

Single organ transplants performed between 07/01/2020 and 12/31/2022

Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	1	18,615
Estimated probability of surviving with a functioning graft at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	86.34% [85.85%-86.83%]
Expected probability of surviving with a functioning graft at 3 years (adjusted for patient and donor characteristics)	95.10%	--
Number of observed graft failures (including deaths) during the first 3 years after transplant	0	2,543
Number of expected graft failures (including deaths) during the first 3 years after transplant	0.05	--
Estimated hazard ratio*	0.98	--
95% credible interval for the hazard ratio**	[0.12, 2.72]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.12, 2.72], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 2% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 88% reduced risk up to 172% increased risk.

Figure C9D. Adult (18+) 3-year deceased donor graft failure HR estimate

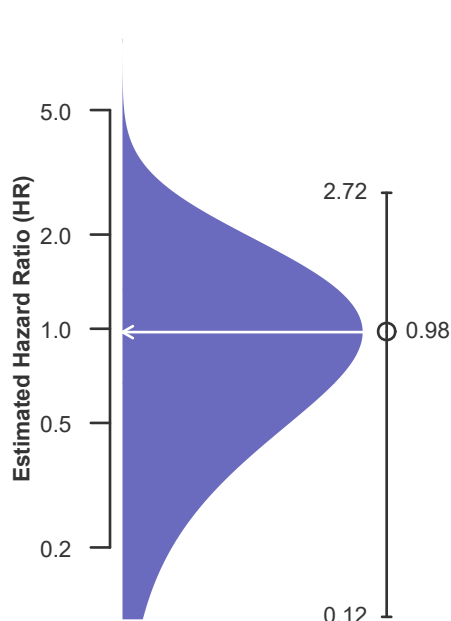
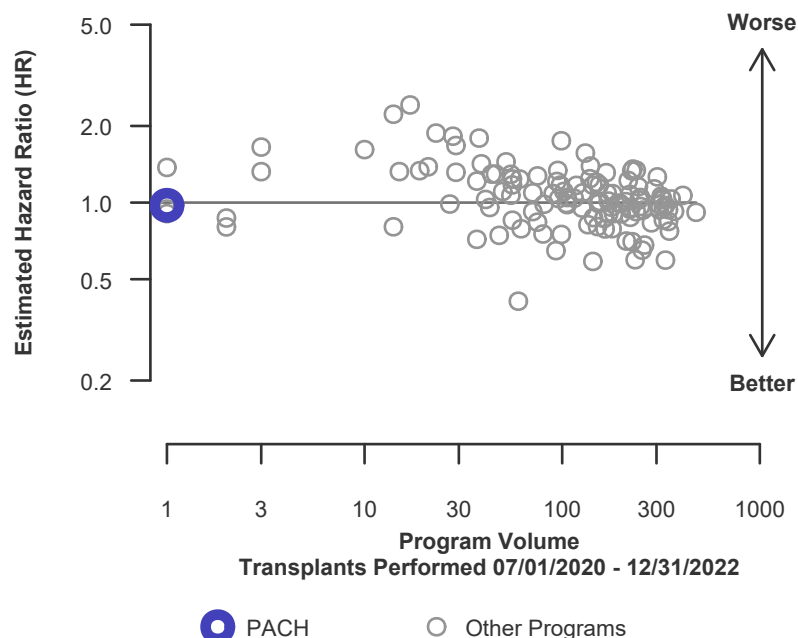


Figure C10D. Adult (18+) 3-year deceased donor graft failure HR program comparison





C. Transplant Information

Table C9L. Adult (18+) 3-year survival with a functioning living donor graft

Single organ transplants performed between 07/01/2020 and 12/31/2022

Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	1	1,241
Estimated probability of surviving with a functioning graft at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	88.88% [87.15%-90.65%]
Expected probability of surviving with a functioning graft at 3 years (adjusted for patient and donor characteristics)	88.89%	--
Number of observed graft failures (including deaths) during the first 3 years after transplant	0	138
Number of expected graft failures (including deaths) during the first 3 years after transplant	0.12	--
Estimated hazard ratio*	0.94	--
95% credible interval for the hazard ratio**	[0.11, 2.63]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.11, 2.63], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 6% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 89% reduced risk up to 163% increased risk.

Figure C9L. Adult (18+) 3-year living donor graft failure HR estimate

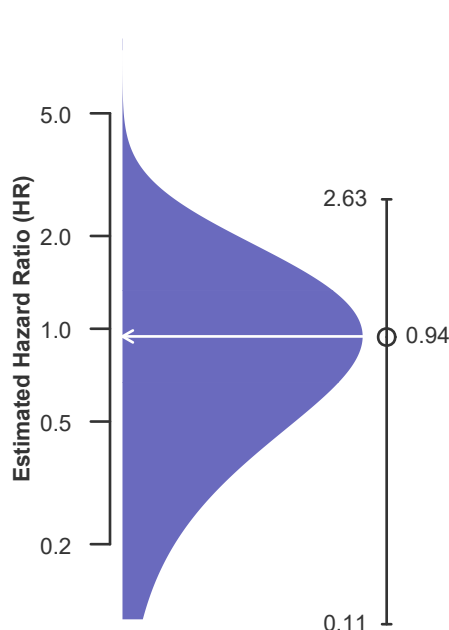
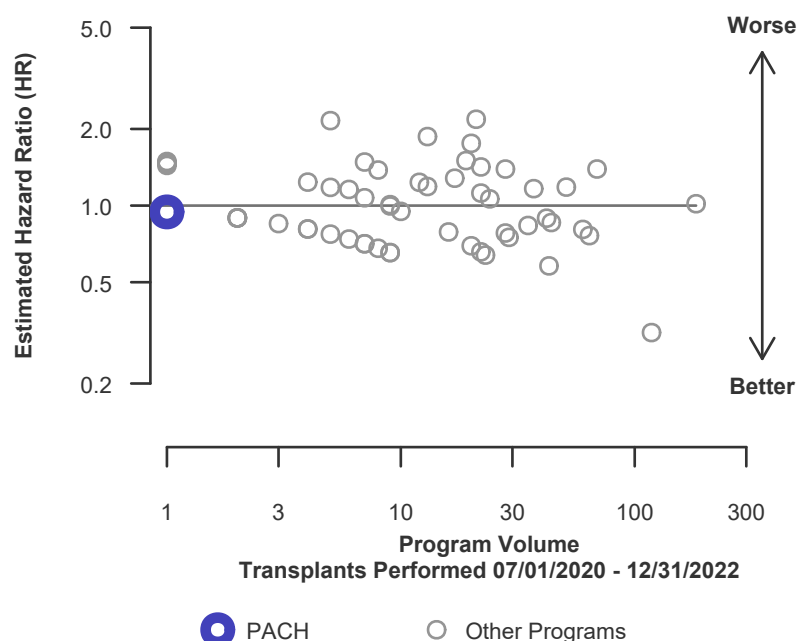


Figure C10L. Adult (18+) 3-year living donor graft failure HR program comparison





C. Transplant Information

Table C10. Pediatric (<18) 1-month survival with a functioning graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	54	1,290
Estimated probability of surviving with a functioning graft at 1 month & [95% CI] (unadjusted for patient and donor characteristics)	98.15% [94.62%-100.00%]	97.44% [96.58%-98.31%]
Expected probability of surviving with a functioning graft at 1 month (adjusted for patient and donor characteristics)	97.51%	--
Number of observed graft failures (including deaths) during the first month after transplant	1	33
Number of expected graft failures (including deaths) during the first month after transplant	1.35	--
Estimated hazard ratio*	0.89	--
95% credible interval for the hazard ratio**	[0.18, 2.16]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.18, 2.16], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 11% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 82% reduced risk up to 116% increased risk.

Figure C11. Pediatric (<18) 1-month graft failure HR estimate

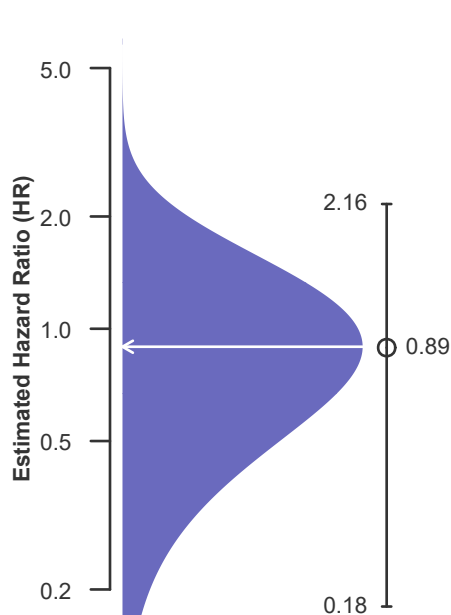
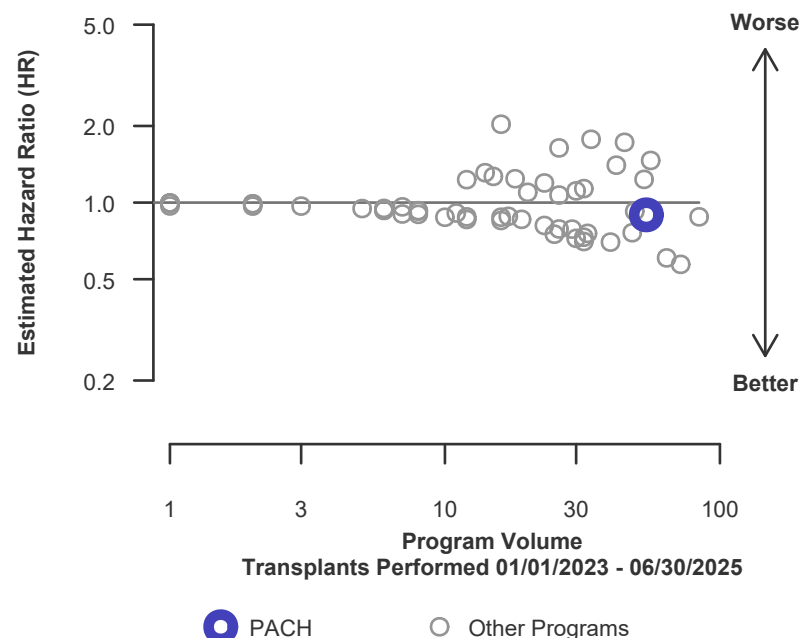


Figure C12. Pediatric (<18) 1-month graft failure HR program comparison





C. Transplant Information

Table C10D. Pediatric (<18) 1-month survival with a functioning deceased donor graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	34	1,048
Estimated probability of surviving with a functioning graft at 1 month & [95% CI] (unadjusted for patient and donor characteristics)	97.06% [91.54%-100.00%]	97.33% [96.36%-98.31%]
Expected probability of surviving with a functioning graft at 1 month (adjusted for patient and donor characteristics)	97.25%	--
Number of observed graft failures (including deaths) during the first month after transplant	1	28
Number of expected graft failures (including deaths) during the first month after transplant	0.94	--
Estimated hazard ratio*	1.02	--
95% credible interval for the hazard ratio**	[0.21, 2.46]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.21, 2.46], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 2% higher risk of graft failure compared to an average program, but PACH's performance could plausibly range from 79% reduced risk up to 146% increased risk.

Figure C11D. Pediatric (<18) 1-month deceased donor graft failure HR estimate

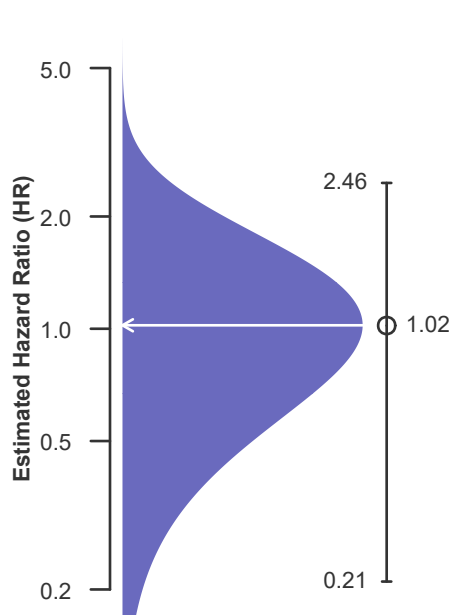
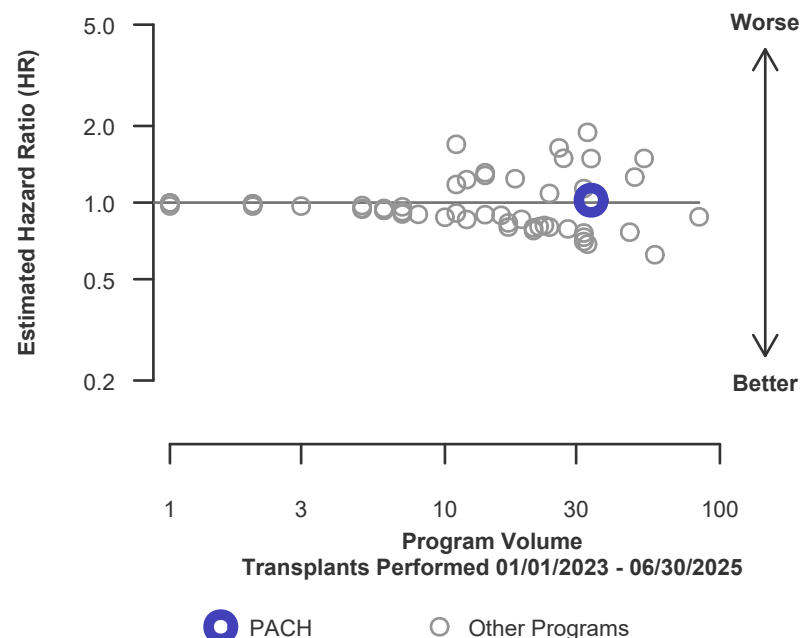


Figure C12D. Pediatric (<18) 1-month deceased donor graft failure HR program comparison





C. Transplant Information

Table C10L. Pediatric (<18) 1-month survival with a functioning living donor graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	20	242
Estimated probability of surviving with a functioning graft at 1 month & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	97.93% [96.16%-99.74%]
Expected probability of surviving with a functioning graft at 1 month (adjusted for patient and donor characteristics)	97.94%	--
Number of observed graft failures (including deaths) during the first month after transplant	0	5
Number of expected graft failures (including deaths) during the first month after transplant	0.42	--
Estimated hazard ratio*	0.83	--
95% credible interval for the hazard ratio**	[0.10, 2.31]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.10, 2.31], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 17% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 90% reduced risk up to 131% increased risk.

Figure C11L. Pediatric (<18) 1-month living donor graft failure HR estimate

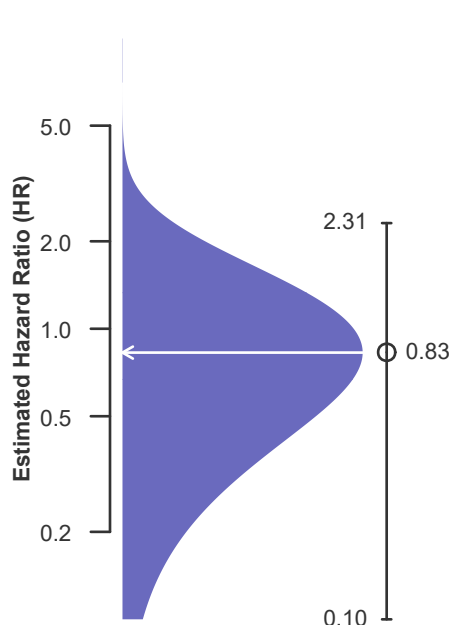
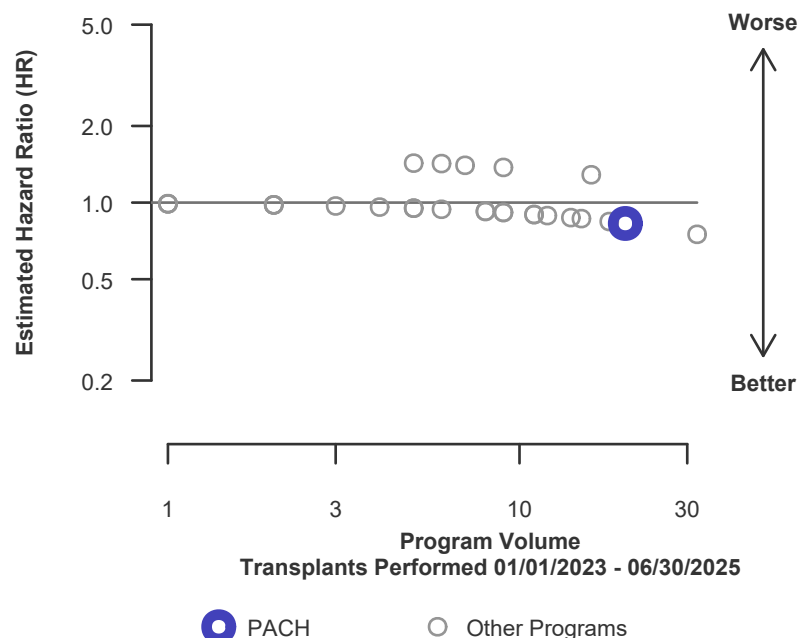


Figure C12L. Pediatric (<18) 1-month living donor graft failure HR program comparison





C. Transplant Information

Table C11. Pediatric (<18) 90-Day survival with a functioning graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	54	1,290
Estimated probability of surviving with a functioning graft at 90 days & [95% CI] (unadjusted for patient and donor characteristics)	98.15% [94.62%-100.00%]	95.81% [94.73%-96.91%]
Expected probability of surviving with a functioning graft at 90 days (adjusted for patient and donor characteristics)	95.59%	--
Number of observed graft failures (including deaths) during the first 90 days after transplant	1	54
Number of expected graft failures (including deaths) during the first 90 days after transplant	2.41	--
Estimated hazard ratio*	0.68	--
95% credible interval for the hazard ratio**	[0.14, 1.64]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.14, 1.64], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 32% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 86% reduced risk up to 64% increased risk.

Figure C13. Pediatric (<18) 90-Day graft failure HR estimate

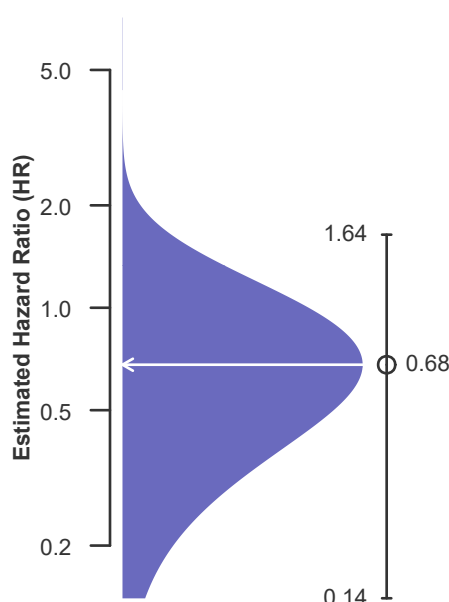
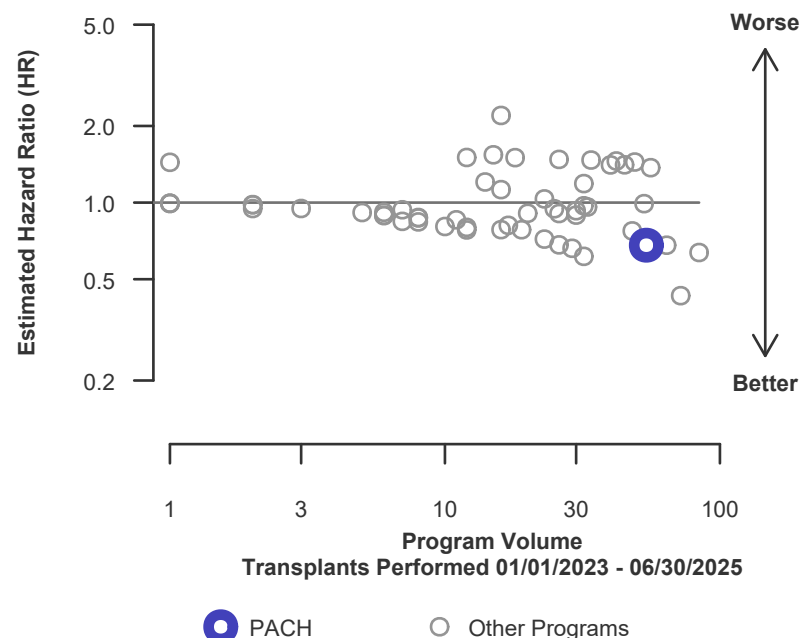


Figure C14. Pediatric (<18) 90-Day graft failure HR program comparison





C. Transplant Information

Table C11D. Pediatric (<18) 90-Day survival with a functioning deceased donor graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	34	1,048
Estimated probability of surviving with a functioning graft at 90 days & [95% CI] (unadjusted for patient and donor characteristics)	97.06% [91.54%-100.00%]	95.80% [94.59%-97.02%]
Expected probability of surviving with a functioning graft at 90 days (adjusted for patient and donor characteristics)	95.41%	--
Number of observed graft failures (including deaths) during the first 90 days after transplant	1	44
Number of expected graft failures (including deaths) during the first 90 days after transplant	1.57	--
Estimated hazard ratio*	0.84	--
95% credible interval for the hazard ratio**	[0.17, 2.02]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.17, 2.02], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 16% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 83% reduced risk up to 102% increased risk.

Figure C13D. Pediatric (<18) 90-Day deceased donor graft failure HR estimate

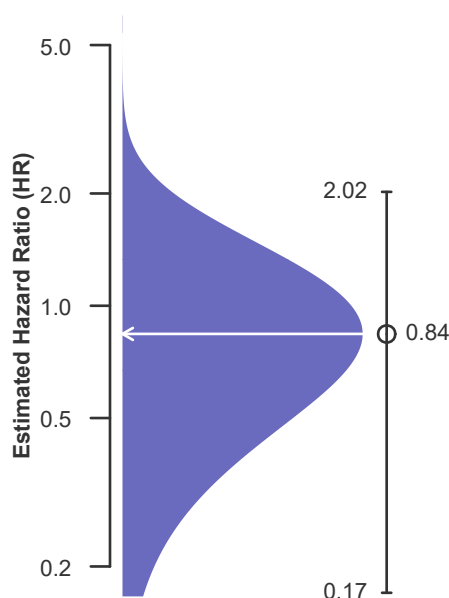
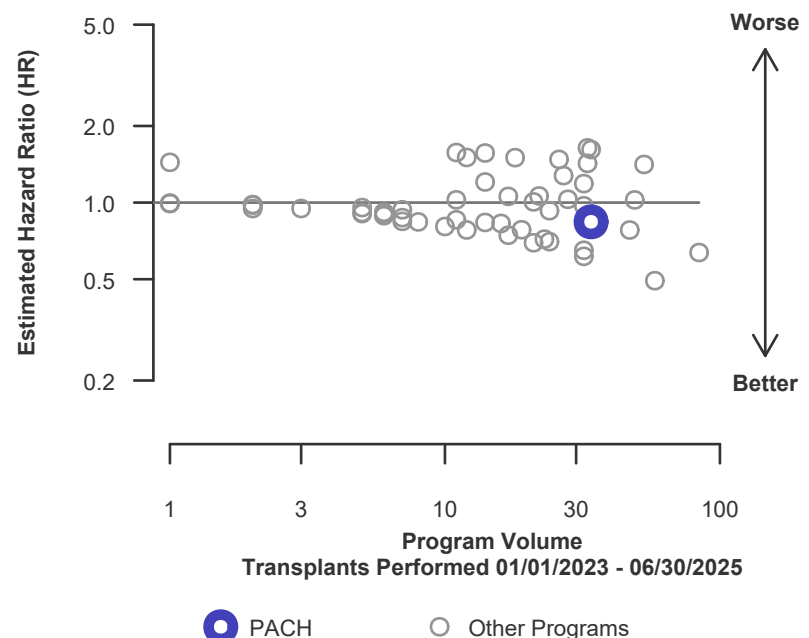


Figure C14D. Pediatric (<18) 90-Day deceased donor graft failure HR program comparison





C. Transplant Information

Table C11L. Pediatric (<18) 90-Day survival with a functioning living donor graft

Single organ transplants performed between 01/01/2023 and 06/30/2025

Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	20	242
Estimated probability of surviving with a functioning graft at 90 days & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	95.87% [93.39%-98.41%]
Expected probability of surviving with a functioning graft at 90 days (adjusted for patient and donor characteristics)	95.88%	--
Number of observed graft failures (including deaths) during the first 90 days after transplant	0	10
Number of expected graft failures (including deaths) during the first 90 days after transplant	0.84	--
Estimated hazard ratio*	0.70	--
95% credible interval for the hazard ratio**	[0.09, 1.96]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.09, 1.96], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 30% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 91% reduced risk up to 96% increased risk.

Figure C13L. Pediatric (<18) 90-Day living donor graft failure HR estimate

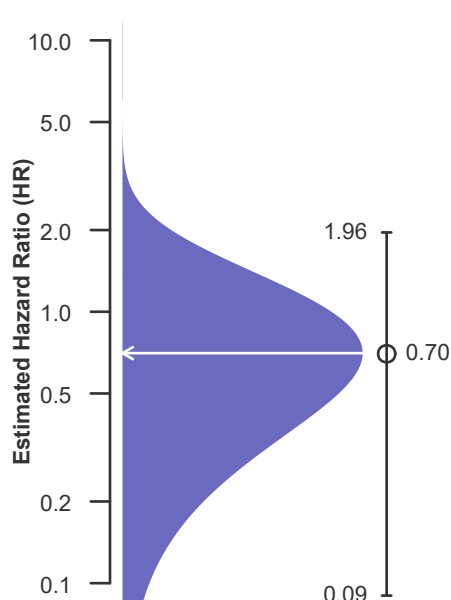
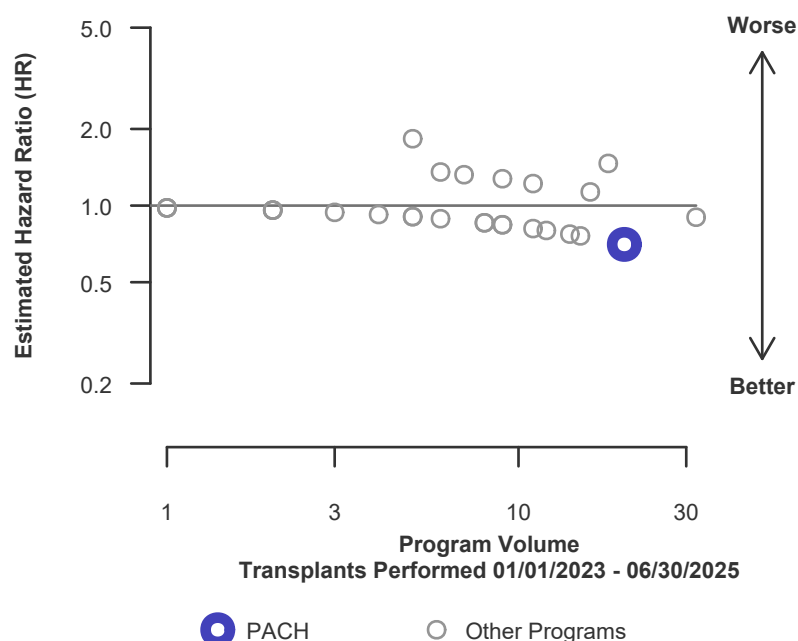


Figure C14L. Pediatric (<18) 90-Day living donor graft failure HR program comparison





C. Transplant Information

Table C12. Pediatric (<18) 1-year survival with a functioning graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	54	1,290
Estimated probability of surviving with a functioning graft at 1 year & [95% CI] (unadjusted for patient and donor characteristics)	94.44% [88.53%-100.00%]	93.78% [92.44%-95.14%]
Expected probability of surviving with a functioning graft at 1 year (adjusted for patient and donor characteristics)	93.40%	--
Number of observed graft failures (including deaths) during the first year after transplant	3	77
Number of expected graft failures (including deaths) during the first year after transplant	3.48	--
Estimated hazard ratio*	0.91	--
95% credible interval for the hazard ratio**	[0.30, 1.87]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.30, 1.87], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 9% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 70% reduced risk up to 87% increased risk.

Figure C15. Pediatric (<18) 1-year graft failure HR estimate

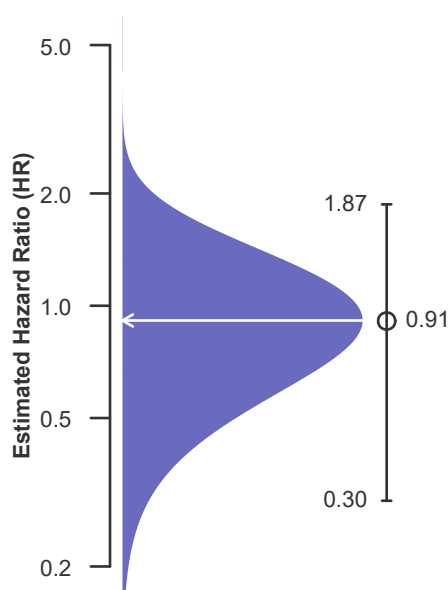
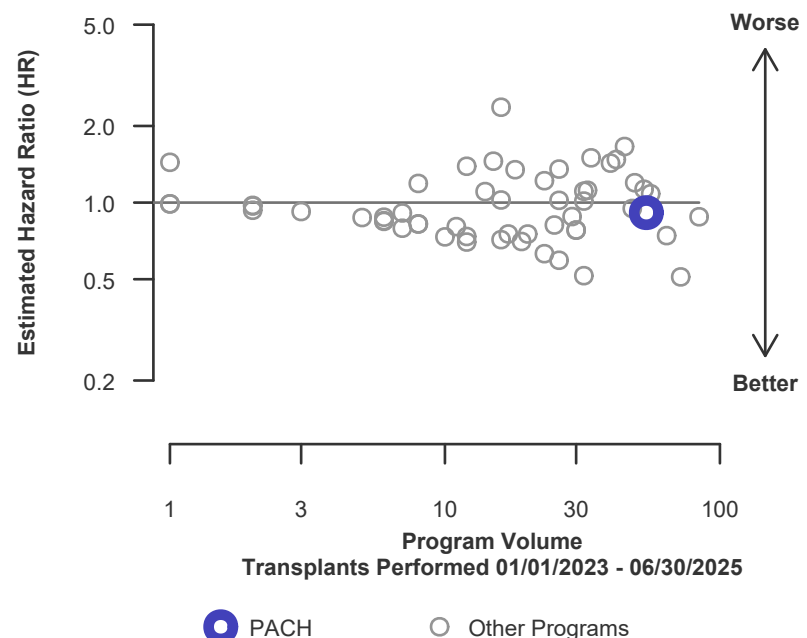


Figure C16. Pediatric (<18) 1-year graft failure HR program comparison





C. Transplant Information

Table C12D. Pediatric (<18) 1-year survival with a functioning deceased donor graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	34	1,048
Estimated probability of surviving with a functioning graft at 1 year & [95% CI] (unadjusted for patient and donor characteristics)	94.12% [86.53%-100.00%]	93.76% [92.27%-95.27%]
Expected probability of surviving with a functioning graft at 1 year (adjusted for patient and donor characteristics)	93.11%	--
Number of observed graft failures (including deaths) during the first year after transplant	2	63
Number of expected graft failures (including deaths) during the first year after transplant	2.28	--
Estimated hazard ratio*	0.93	--
95% credible interval for the hazard ratio**	[0.25, 2.05]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.25, 2.05], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 7% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 75% reduced risk up to 105% increased risk.

Figure C15D. Pediatric (<18) 1-year deceased donor graft failure HR estimate

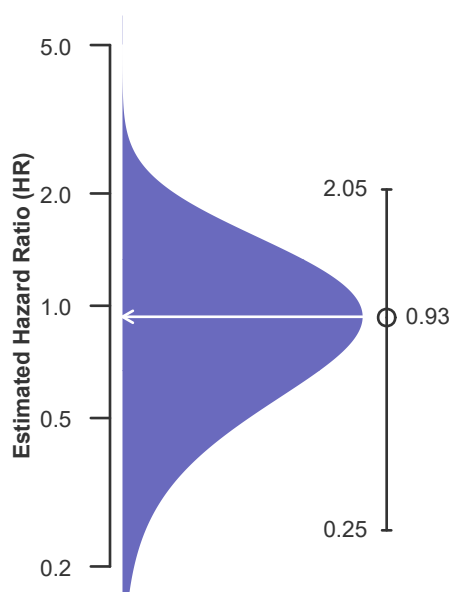
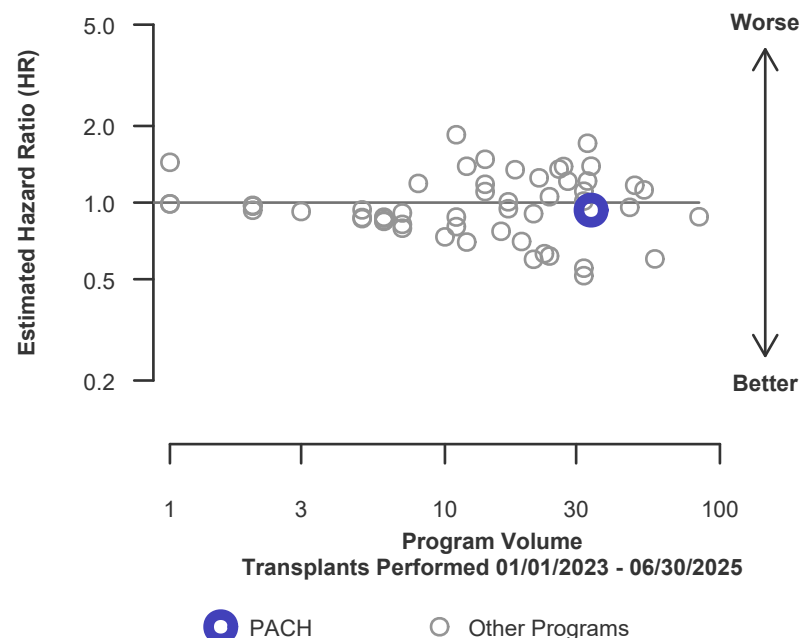


Figure C16D. Pediatric (<18) 1-year deceased donor graft failure HR program comparison





C. Transplant Information

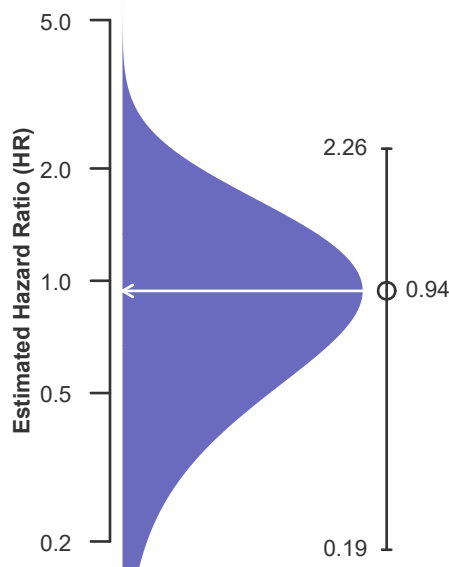
Table C12L. Pediatric (<18) 1-year survival with a functioning living donor graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	20	242
Estimated probability of surviving with a functioning graft at 1 year & [95% CI] (unadjusted for patient and donor characteristics)	95.00% [85.91%-100.00%]	93.87% [90.79%-97.06%]
Expected probability of surviving with a functioning graft at 1 year (adjusted for patient and donor characteristics)	93.89%	--
Number of observed graft failures (including deaths) during the first year after transplant	1	14
Number of expected graft failures (including deaths) during the first year after transplant	1.19	--
Estimated hazard ratio*	0.94	--
95% credible interval for the hazard ratio**	[0.19, 2.26]	--

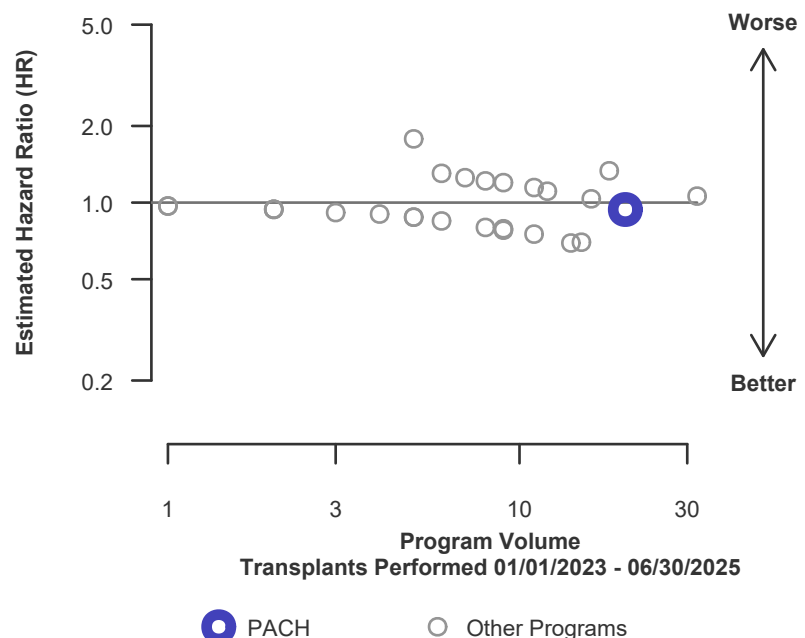
* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.19, 2.26], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 6% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 81% reduced risk up to 126% increased risk.

**Figure C15L. Pediatric (<18)
1-year living donor graft failure
HR estimate**



**Figure C16L. Pediatric (<18) 1-year living donor graft failure
HR program comparison**





C. Transplant Information

Table C13. Pediatric (<18) 1-year Conditional survival with a functioning graft

Single organ transplants performed between 01/01/2023 and 06/30/2025

Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	53	1,236
Estimated probability of surviving with a functioning graft at 1 year, among patients with a functioning graft at day 90 & [95% CI] (unadjusted for patient and donor characteristics)	96.23% [93.56%-100.00%]	97.88% [97.58%-98.17%]
Expected probability of surviving with a functioning graft at 1 year, among patients with a functioning graft at day 90 (adjusted for patient and donor characteristics)	97.71%	--
Number of observed graft failures (including deaths) from day 91 through day 365 after transplant	2	23
Number of expected graft failures (including deaths) from day 91 through day 365 after transplant	1.06	--
Estimated hazard ratio*	1.31	--
95% credible interval for the hazard ratio**	[0.36, 2.86]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.36, 2.86], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 31% higher risk of graft failure compared to an average program, but PACH's performance could plausibly range from 64% reduced risk up to 186% increased risk.

Figure C17. Pediatric (<18) 1-year Conditional graft failure HR estimate

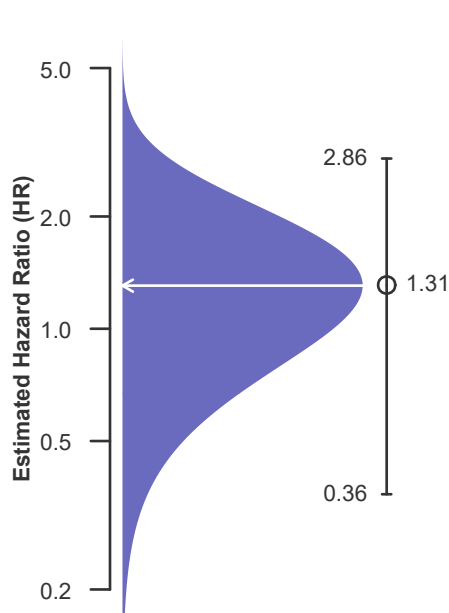
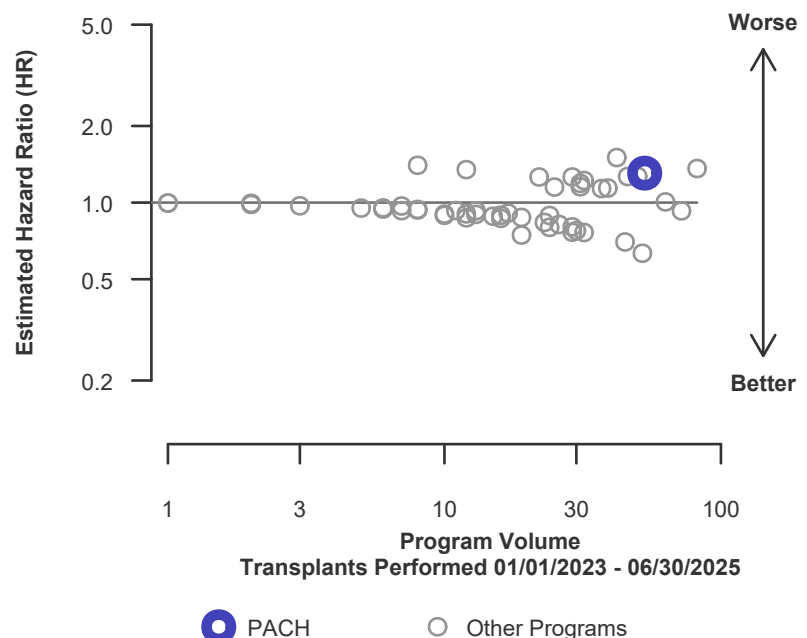


Figure C18. Pediatric (<18) 1-year Conditional graft failure HR program comparison





C. Transplant Information

Table C13D. Pediatric (<18) 1-year Conditional survival with a functioning deceased donor graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	33	1,004
Estimated probability of surviving with a functioning graft at 1 year, among patients with a functioning graft at day 90 & [95% CI] (unadjusted for patient and donor characteristics)	96.97% [94.53%-100.00%]	97.87% [97.54%-98.19%]
Expected probability of surviving with a functioning graft at 1 year, among patients with a functioning graft at day 90 (adjusted for patient and donor characteristics)	97.59%	--
Number of observed graft failures (including deaths) from day 91 through day 365 after transplant	1	19
Number of expected graft failures (including deaths) from day 91 through day 365 after transplant	0.71	--
Estimated hazard ratio*	1.11	--
95% credible interval for the hazard ratio**	[0.23, 2.66]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.23, 2.66], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 11% higher risk of graft failure compared to an average program, but PACH's performance could plausibly range from 77% reduced risk up to 166% increased risk.

Figure C17D. Pediatric (<18) 1-year Conditional deceased donor graft failure HR estimate

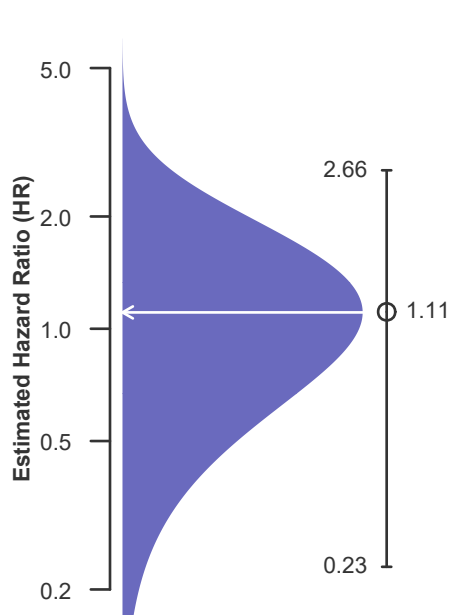
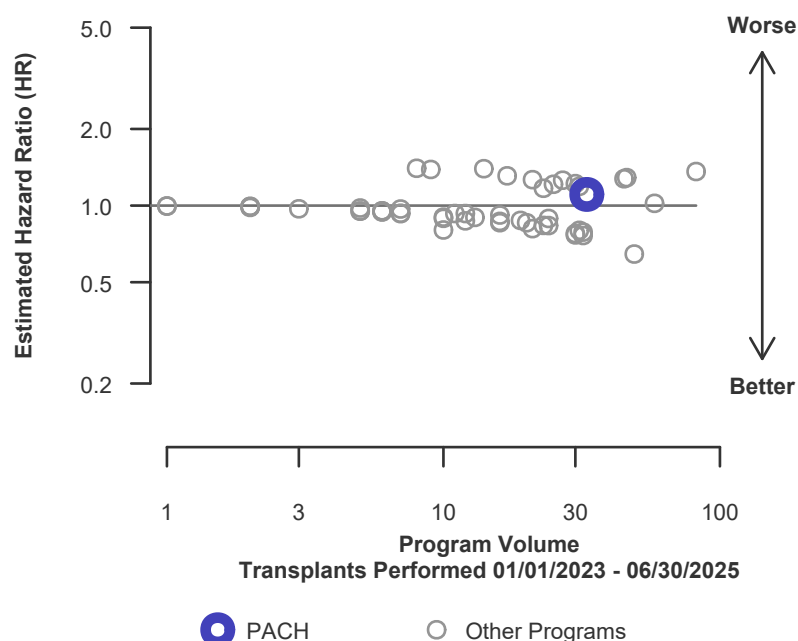


Figure C18D. Pediatric (<18) 1-year Conditional deceased donor graft failure HR program comparison





C. Transplant Information

Table C13L. Pediatric (<18) 1-year Conditional survival with a functioning living donor graft

Single organ transplants performed between 01/01/2023 and 06/30/2025

Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	20	232
Estimated probability of surviving with a functioning graft at 1 year, among patients with a functioning graft at day 90 & [95% CI] (unadjusted for patient and donor characteristics)	95.00% [85.91%-100.00%]	97.92% [97.21%-98.63%]
Expected probability of surviving with a functioning graft at 1 year, among patients with a functioning graft at day 90 (adjusted for patient and donor characteristics)	97.92%	--
Number of observed graft failures (including deaths) from day 91 through day 365 after transplant	1	4
Number of expected graft failures (including deaths) from day 91 through day 365 after transplant	0.35	--
Estimated hazard ratio*	1.28	--
95% credible interval for the hazard ratio**	[0.26, 3.07]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.26, 3.07], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 28% higher risk of graft failure compared to an average program, but PACH's performance could plausibly range from 74% reduced risk up to 207% increased risk.

Figure C17L. Pediatric (<18) 1-year Conditional living donor graft failure HR estimate

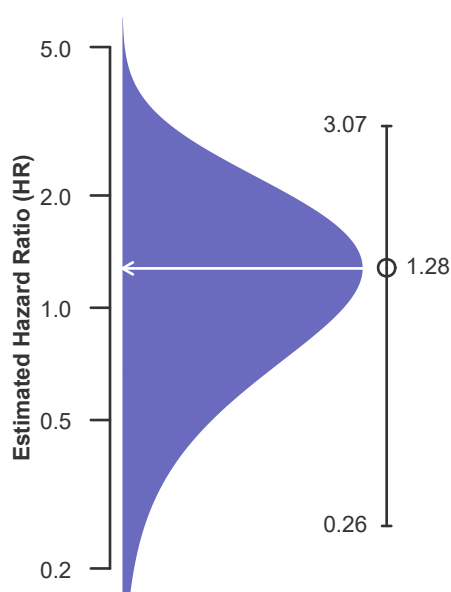
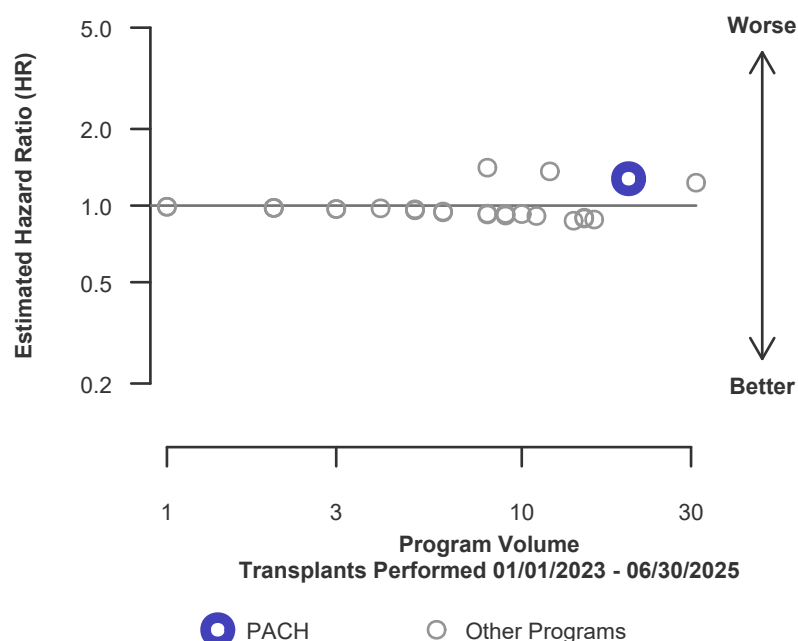


Figure C18L. Pediatric (<18) 1-year Conditional living donor graft failure HR program comparison





C. Transplant Information

Table C14. Pediatric (<18) 3-year survival with a functioning graft
Single organ transplants performed between 07/01/2020 and 12/31/2022
Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	57	1,230
Estimated probability of surviving with a functioning graft at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	94.74% [89.11%-100.00%]	89.67% [87.99%-91.39%]
Expected probability of surviving with a functioning graft at 3 years (adjusted for patient and donor characteristics)	90.55%	--
Number of observed graft failures (including deaths) during the first 3 years after transplant	3	127
Number of expected graft failures (including deaths) during the first 3 years after transplant	5.36	--
Estimated hazard ratio*	0.68	--
95% credible interval for the hazard ratio**	[0.22, 1.39]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.22, 1.39], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 32% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 78% reduced risk up to 39% increased risk.

Figure C19. Pediatric (<18) 3-year graft failure HR estimate

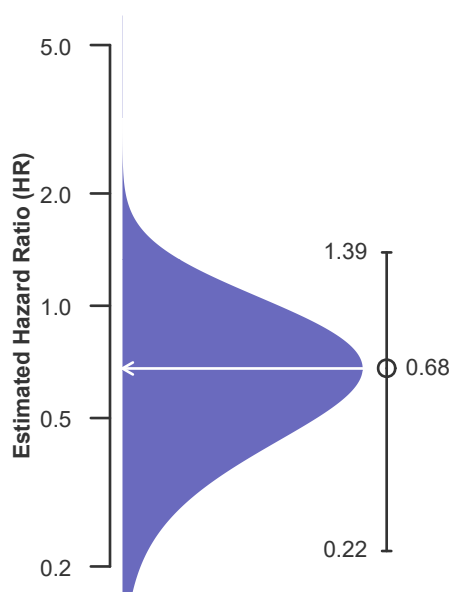
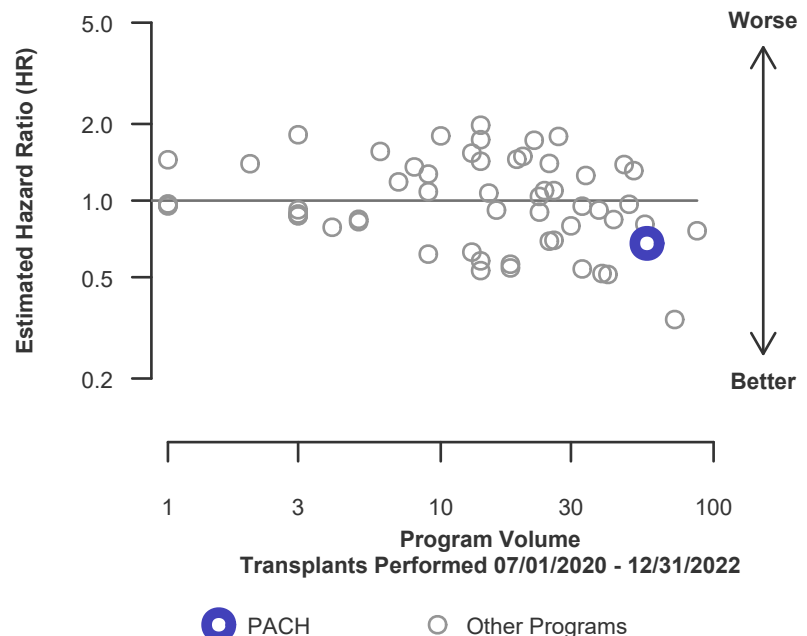


Figure C20. Pediatric (<18) 3-year graft failure HR program comparison





C. Transplant Information

Table C14D. Pediatric (<18) 3-year survival with a functioning deceased donor graft

Single organ transplants performed between 07/01/2020 and 12/31/2022

Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	40	1,032
Estimated probability of surviving with a functioning graft at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	92.50% [84.69%-100.00%]	89.24% [87.37%-91.15%]
Expected probability of surviving with a functioning graft at 3 years (adjusted for patient and donor characteristics)	89.96%	--
Number of observed graft failures (including deaths) during the first 3 years after transplant	3	111
Number of expected graft failures (including deaths) during the first 3 years after transplant	3.94	--
Estimated hazard ratio*	0.84	--
95% credible interval for the hazard ratio**	[0.27, 1.73]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.27, 1.73], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 16% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 73% reduced risk up to 73% increased risk.

Figure C19D. Pediatric (<18) 3-year deceased donor graft failure HR estimate

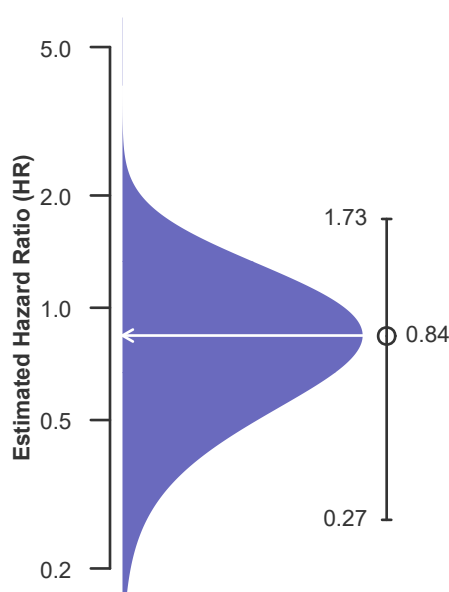
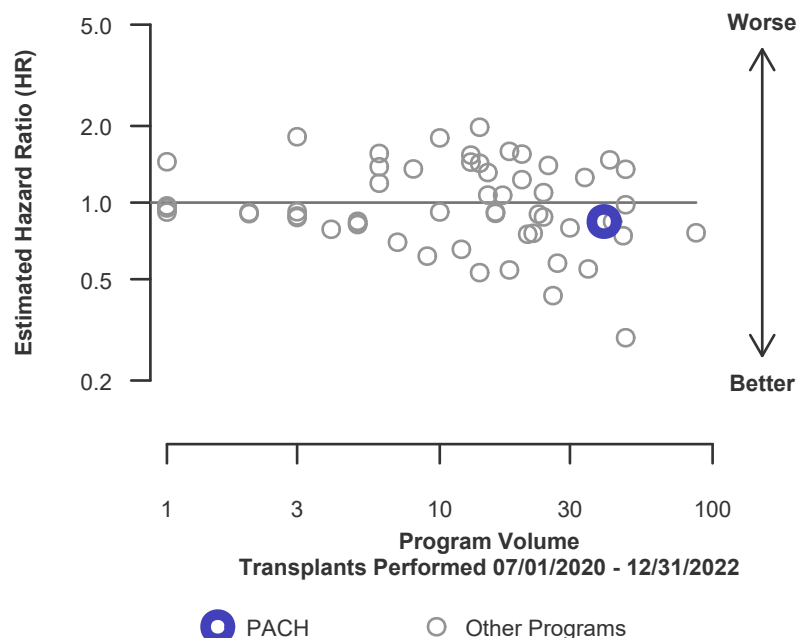


Figure C20D. Pediatric (<18) 3-year deceased donor graft failure HR program comparison





C. Transplant Information

Table C14L. Pediatric (<18) 3-year survival with a functioning living donor graft

Single organ transplants performed between 07/01/2020 and 12/31/2022

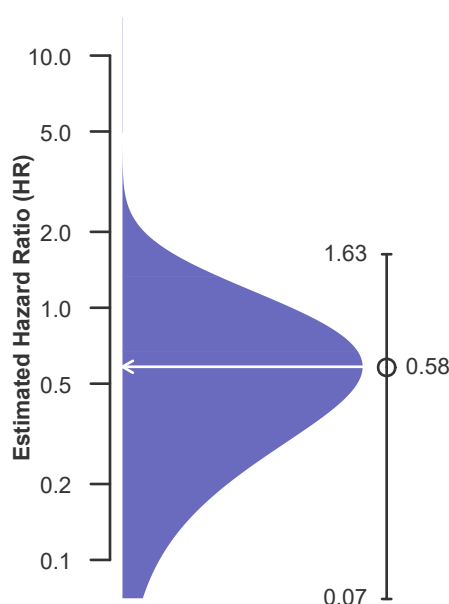
Deaths and retransplants are considered graft failures

	PACH	U.S.
Number of transplants evaluated	17	198
Estimated probability of surviving with a functioning graft at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	91.92% [88.20%-95.79%]
Expected probability of surviving with a functioning graft at 3 years (adjusted for patient and donor characteristics)	91.94%	--
Number of observed graft failures (including deaths) during the first 3 years after transplant	0	16
Number of expected graft failures (including deaths) during the first 3 years after transplant	1.43	--
Estimated hazard ratio*	0.58	--
95% credible interval for the hazard ratio**	[0.07, 1.63]	--

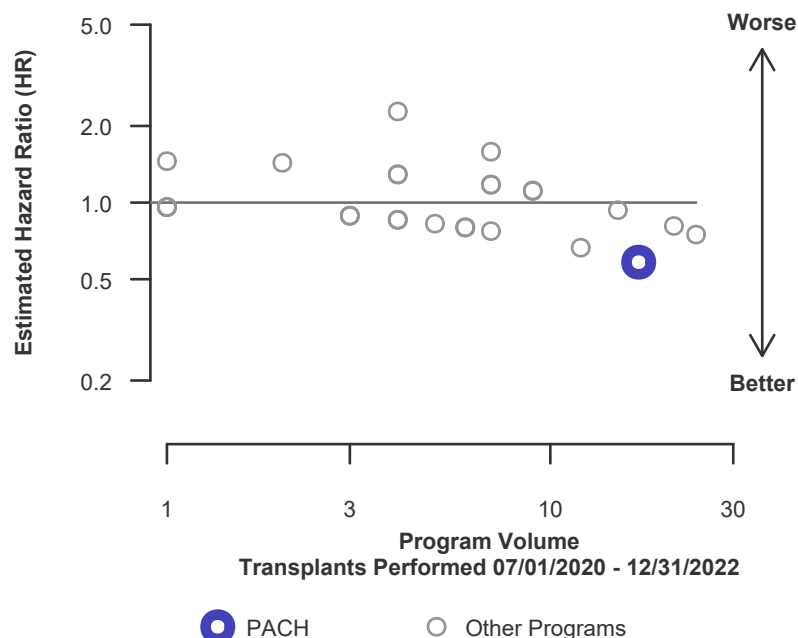
* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.07, 1.63], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 42% lower risk of graft failure compared to an average program, but PACH's performance could plausibly range from 93% reduced risk up to 63% increased risk.

**Figure C19L. Pediatric (<18)
3-year living donor graft failure
HR estimate**



**Figure C20L. Pediatric (<18) 3-year living donor graft failure
HR program comparison**





C. Transplant Information

Table C15. Adult (18+) 1-month patient survival

Single organ transplants performed between 01/01/2023 and 06/30/2025

Retransplants excluded

	PACH	U.S.
Number of transplants evaluated	7	23,811
Estimated probability of surviving at 1 month & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	98.47% [98.32%-98.63%]
Expected probability of surviving at 1 month (adjusted for patient and donor characteristics)	99.33%	--
Number of observed deaths during the first month after transplant	0	364
Number of expected deaths during the first month after transplant	0.05	--
Estimated hazard ratio*	0.98	--
95% credible interval for the hazard ratio**	[0.12, 2.72]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.12, 2.72], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 2% lower risk of patient death compared to an average program, but PACH's performance could plausibly range from 88% reduced risk up to 172% increased risk.

Figure C21. Adult (18+) 1-month patient death HR estimate

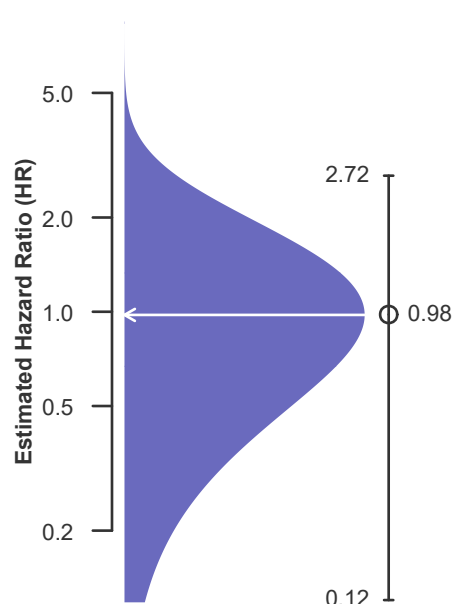
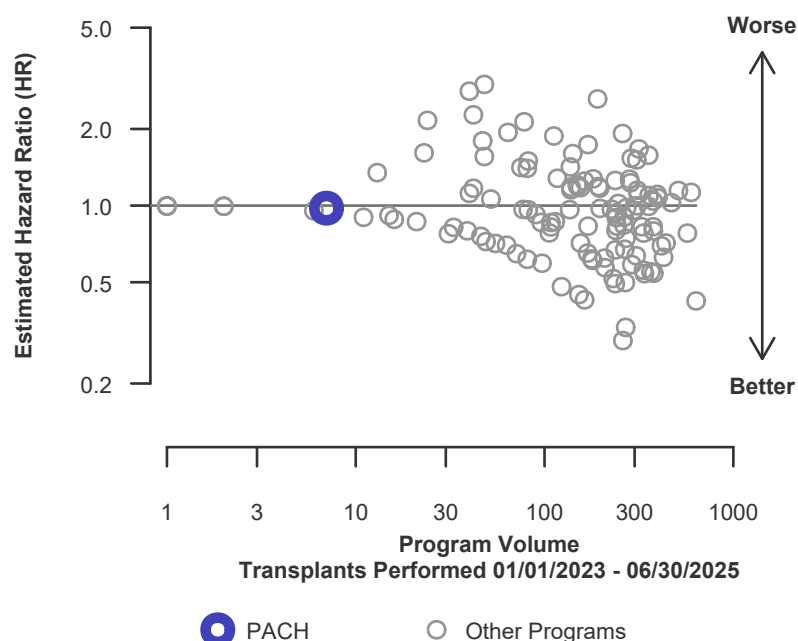


Figure C22. Adult (18+) 1-month patient death HR program comparison





C. Transplant Information

Table C15D. Adult (18+) 1-month patient survival (deceased donor graft recipients)

Single organ transplants performed between 01/01/2023 and 06/30/2025

Retransplants excluded

	PACH	U.S.
Number of transplants evaluated	5	22,475
Estimated probability of surviving at 1 month & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	98.44% [98.28%-98.60%]
Expected probability of surviving at 1 month (adjusted for patient and donor characteristics)	99.25%	--
Number of observed deaths during the first month after transplant	0	351
Number of expected deaths during the first month after transplant	0.04	--
Estimated hazard ratio*	0.98	--
95% credible interval for the hazard ratio**	[0.12, 2.73]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.12, 2.73], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 2% lower risk of patient death compared to an average program, but PACH's performance could plausibly range from 88% reduced risk up to 173% increased risk.

Figure C21D. Adult (18+) 1-month patient death HR estimate (deceased donor grafts)

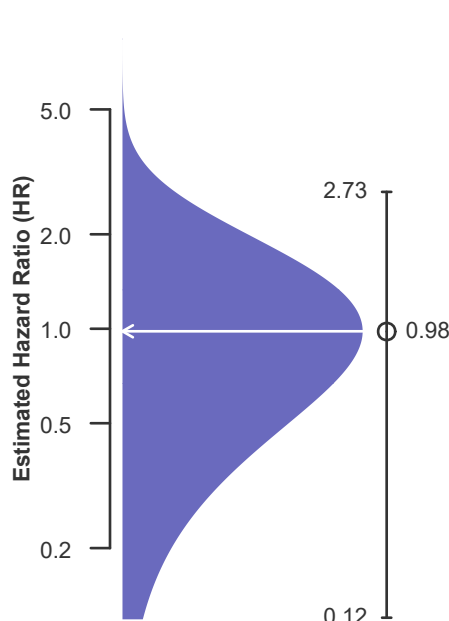
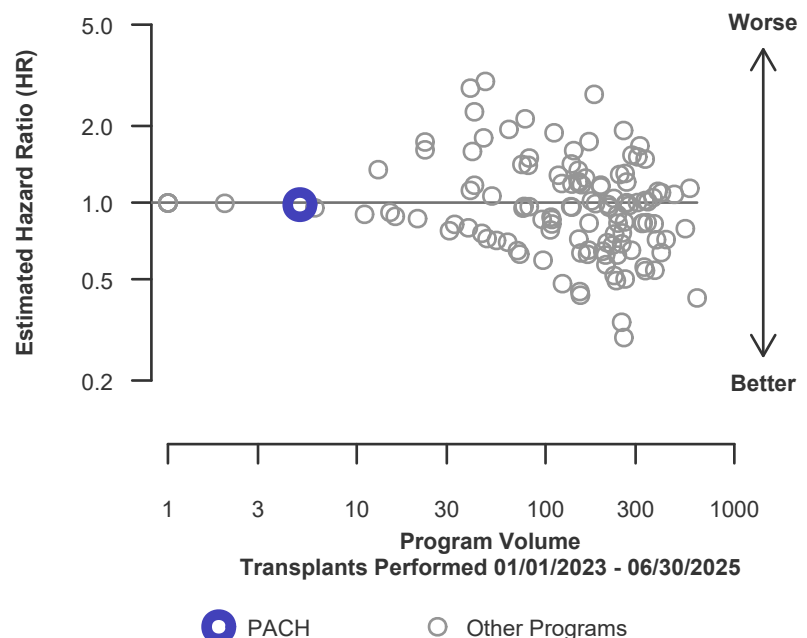


Figure C22D. Adult (18+) 1-month patient death HR program comparison (deceased donor grafts)





C. Transplant Information

Table C15L. Adult (18+) 1-month patient survival (living donor graft recipients)

Single organ transplants performed between 01/01/2023 and 06/30/2025

Retransplants excluded

	PACH	U.S.
Number of transplants evaluated	2	1,336
Estimated probability of surviving at 1 month & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	99.03% [98.50%-99.55%]
Expected probability of surviving at 1 month (adjusted for patient and donor characteristics)	99.52%	--
Number of observed deaths during the first month after transplant	0	13
Number of expected deaths during the first month after transplant	0.01	--
Estimated hazard ratio*	1.00	--
95% credible interval for the hazard ratio**	[0.12, 2.77]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.12, 2.77], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 0% lower risk of patient death compared to an average program, but PACH's performance could plausibly range from 88% reduced risk up to 177% increased risk.

Figure C21L. Adult (18+) 1-month patient death HR estimate (living donor grafts)

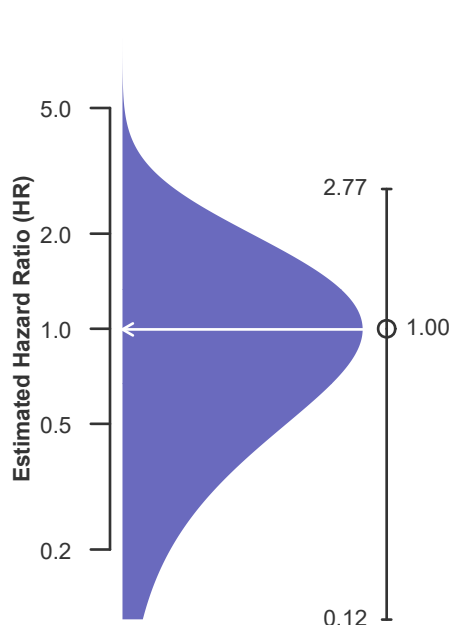
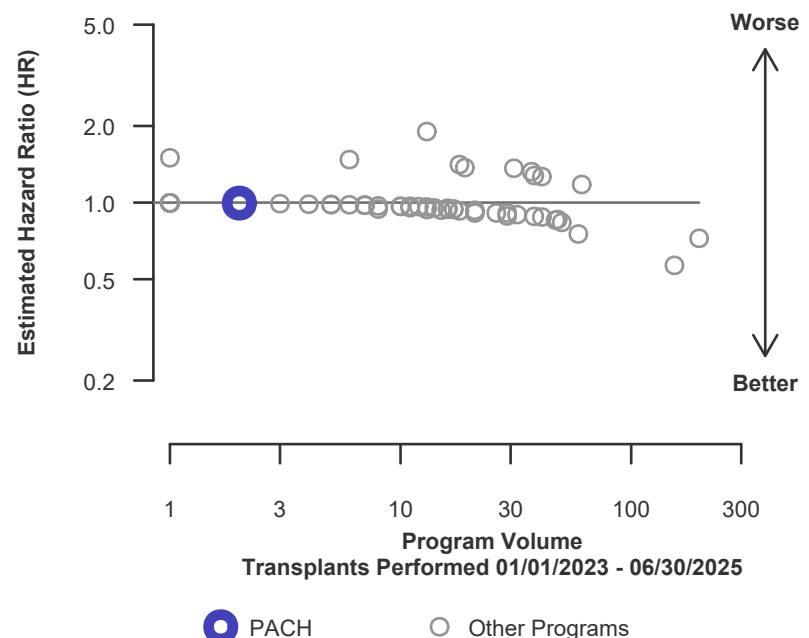


Figure C22L. Adult (18+) 1-month patient death HR program comparison (living donor grafts)





C. Transplant Information

Table C16. Adult (18+) 1-year patient survival

Single organ transplants performed between 01/01/2023 and 06/30/2025

Retransplants excluded

	PACH	U.S.
Number of transplants evaluated	7	23,811
Estimated probability of surviving at 1 year & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	94.27% [93.97%-94.58%]
Expected probability of surviving at 1 year (adjusted for patient and donor characteristics)	97.43%	--
Number of observed deaths during the first year after transplant	0	1,277
Number of expected deaths during the first year after transplant	0.18	--
Estimated hazard ratio*	0.92	--
95% credible interval for the hazard ratio**	[0.11, 2.55]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.11, 2.55], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 8% lower risk of patient death compared to an average program, but PACH's performance could plausibly range from 89% reduced risk up to 155% increased risk.

Figure C23. Adult (18+) 1-year patient death HR estimate

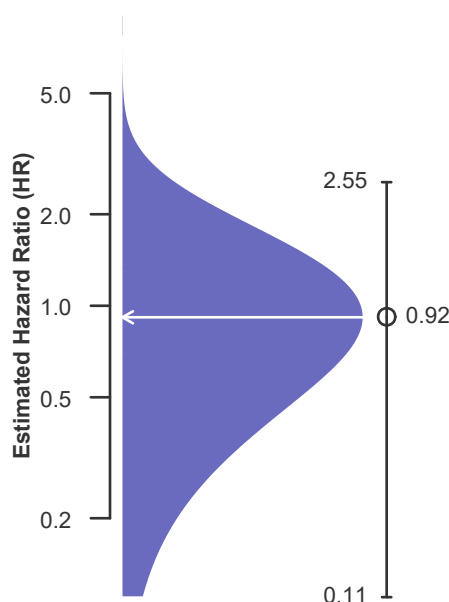
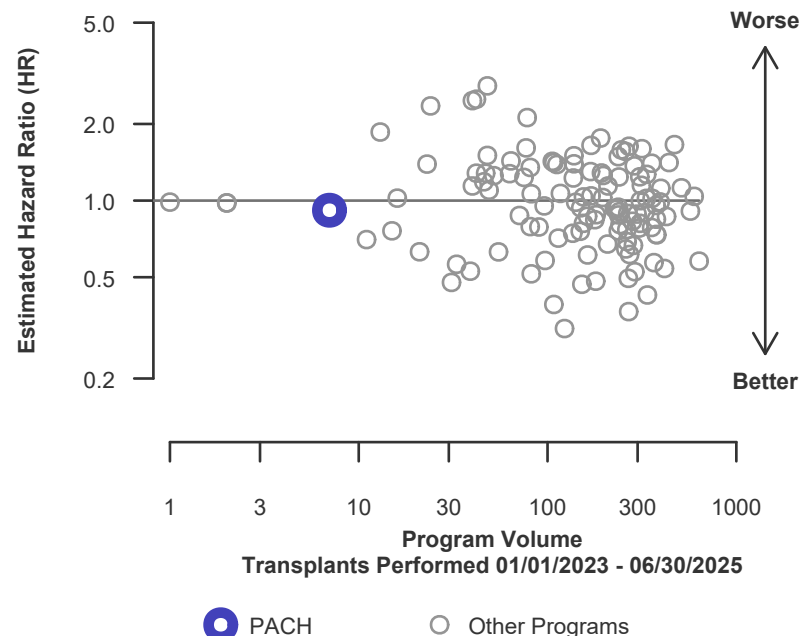


Figure C24. Adult (18+) 1-year patient death HR program comparison





C. Transplant Information

Table C16D. Adult (18+) 1-year patient survival (deceased donor graft recipients)

Single organ transplants performed between 01/01/2023 and 06/30/2025

Retransplants excluded

	PACH	U.S.
Number of transplants evaluated	5	22,475
Estimated probability of surviving at 1 year & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	94.16% [93.84%-94.48%]
Expected probability of surviving at 1 year (adjusted for patient and donor characteristics)	97.22%	--
Number of observed deaths during the first year after transplant	0	1,230
Number of expected deaths during the first year after transplant	0.14	--
Estimated hazard ratio*	0.93	--
95% credible interval for the hazard ratio**	[0.11, 2.60]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.11, 2.60], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 7% lower risk of patient death compared to an average program, but PACH's performance could plausibly range from 89% reduced risk up to 160% increased risk.

Figure C23D. Adult (18+) 1-year patient death HR estimate (deceased donor grafts)

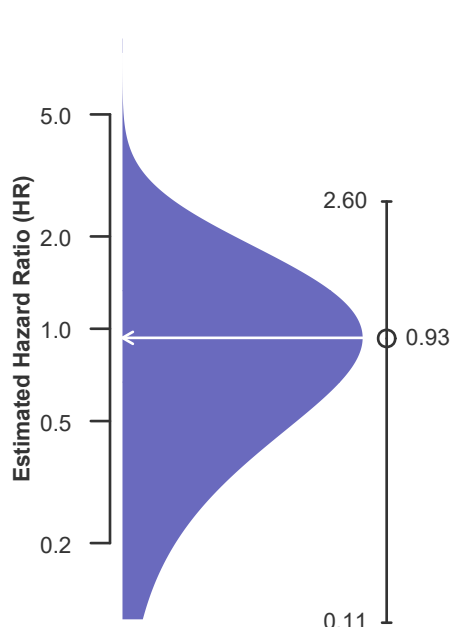
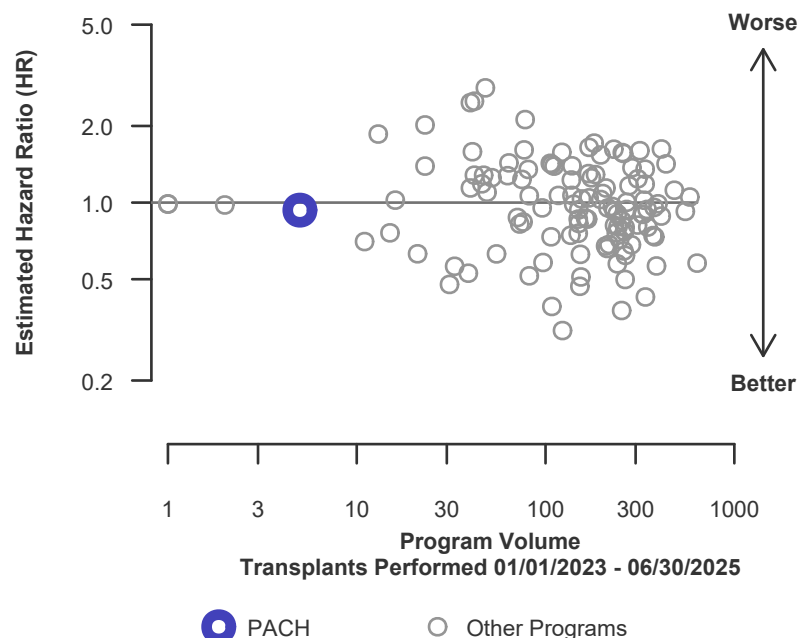


Figure C24D. Adult (18+) 1-year patient death HR program comparison (deceased donor grafts)





C. Transplant Information

Table C16L. Adult (18+) 1-year patient survival (living donor graft recipients)
Single organ transplants performed between 01/01/2023 and 06/30/2025
Retransplants excluded

	PACH	U.S.
Number of transplants evaluated	2	1,336
Estimated probability of surviving at 1 year & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	96.21% [95.14%-97.28%]
Expected probability of surviving at 1 year (adjusted for patient and donor characteristics)	97.97%	--
Number of observed deaths during the first year after transplant	0	47
Number of expected deaths during the first year after transplant	0.04	--
Estimated hazard ratio*	0.98	--
95% credible interval for the hazard ratio**	[0.12, 2.73]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.12, 2.73], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 2% lower risk of patient death compared to an average program, but PACH's performance could plausibly range from 88% reduced risk up to 173% increased risk.

Figure C23L. Adult (18+) 1-year patient death HR estimate (living donor grafts)

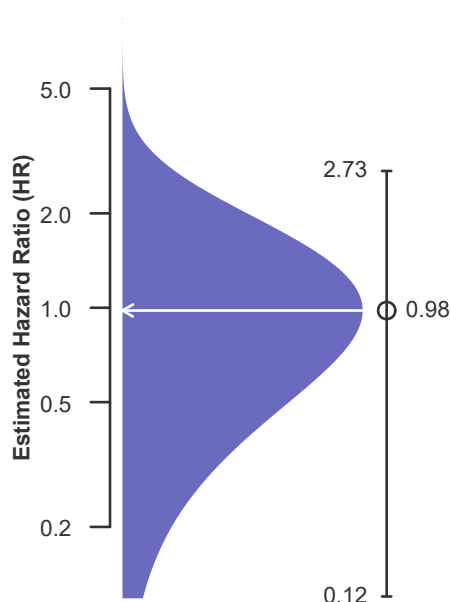
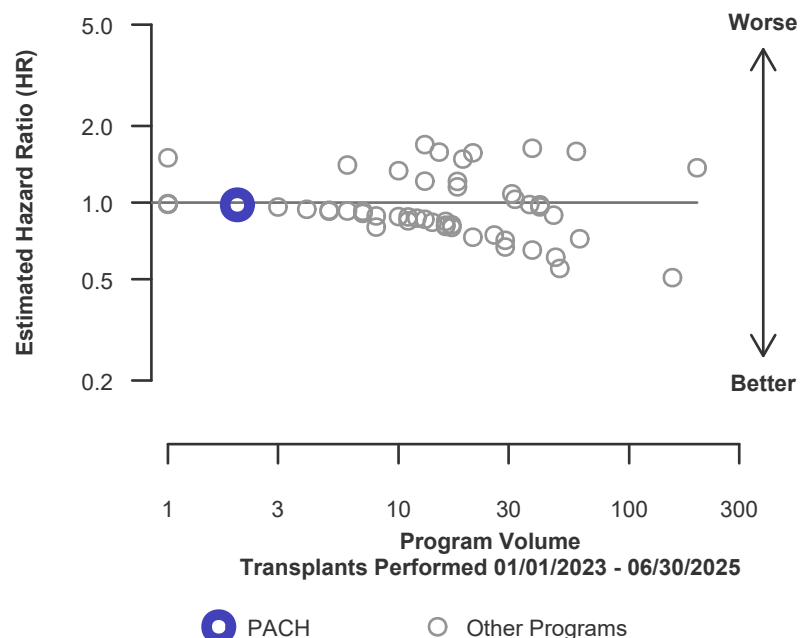


Figure C24L. Adult (18+) 1-year patient death HR program comparison (living donor grafts)





C. Transplant Information

Table C17. Adult (18+) 3-year patient survival

Single organ transplants performed between 07/01/2020 and 12/31/2022

Retransplants excluded

	PACH	U.S.
Number of transplants evaluated	2	19,158
Estimated probability of surviving at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	88.78% [88.33%-89.23%]
Expected probability of surviving at 3 years (adjusted for patient and donor characteristics)	95.92%	--
Number of observed deaths during the first 3 years after transplant	0	2,150
Number of expected deaths during the first 3 years after transplant	0.08	--
Estimated hazard ratio*	0.96	--
95% credible interval for the hazard ratio**	[0.12, 2.67]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.12, 2.67], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 4% lower risk of patient death compared to an average program, but PACH's performance could plausibly range from 88% reduced risk up to 167% increased risk.

Figure C25. Adult (18+) 3-year patient death HR estimate

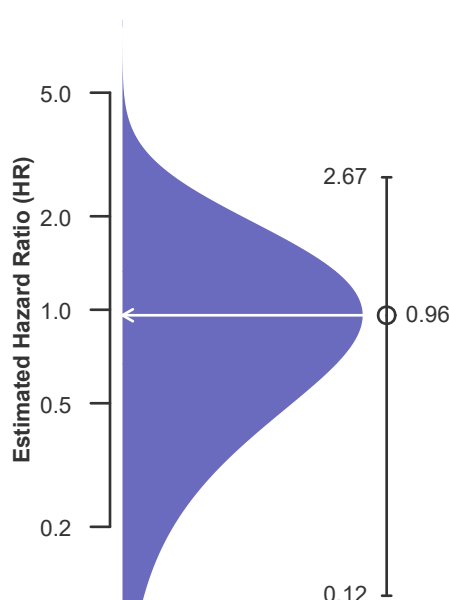
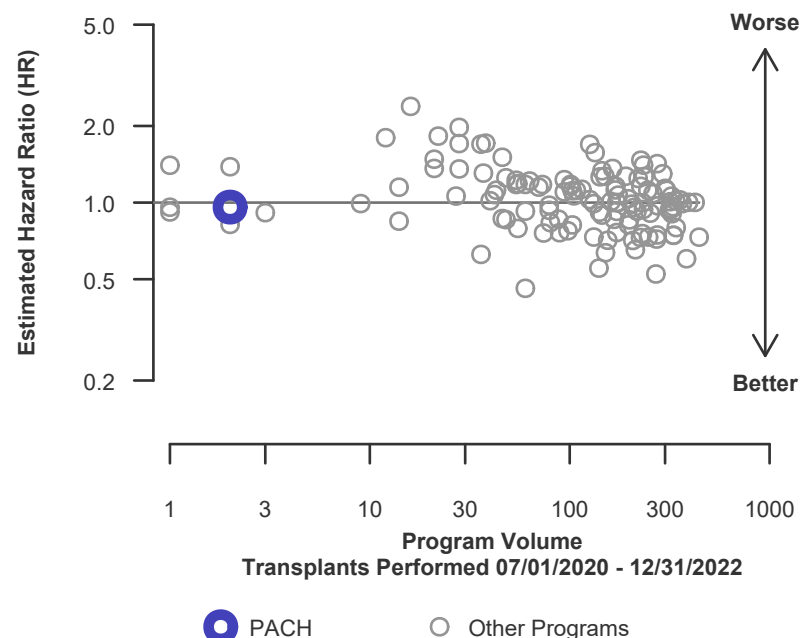


Figure C26. Adult (18+) 3-year patient death HR program comparison





C. Transplant Information

Table C17D. Adult (18+) 3-year patient survival (deceased donor graft recipients)

Single organ transplants performed between 07/01/2020 and 12/31/2022

Retransplants excluded

	PACH	U.S.
Number of transplants evaluated	1	17,930
Estimated probability of surviving at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	88.57% [88.10%-89.03%]
Expected probability of surviving at 3 years (adjusted for patient and donor characteristics)	95.89%	--
Number of observed deaths during the first 3 years after transplant	0	2,050
Number of expected deaths during the first 3 years after transplant	0.04	--
Estimated hazard ratio*	0.98	--
95% credible interval for the hazard ratio**	[0.12, 2.73]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.12, 2.73], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 2% lower risk of patient death compared to an average program, but PACH's performance could plausibly range from 88% reduced risk up to 173% increased risk.

Figure C25D. Adult (18+) 3-year patient death HR estimate (deceased donor grafts)

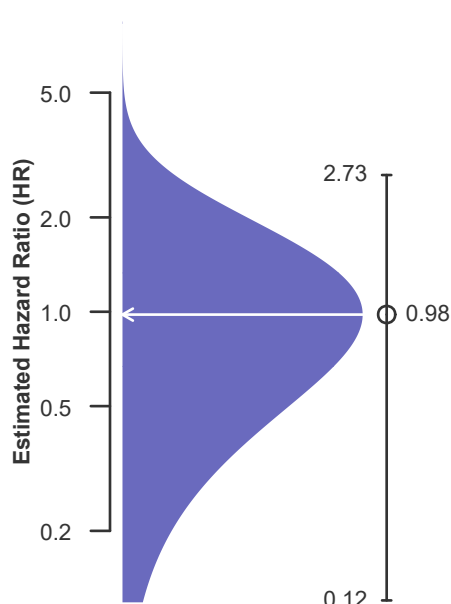
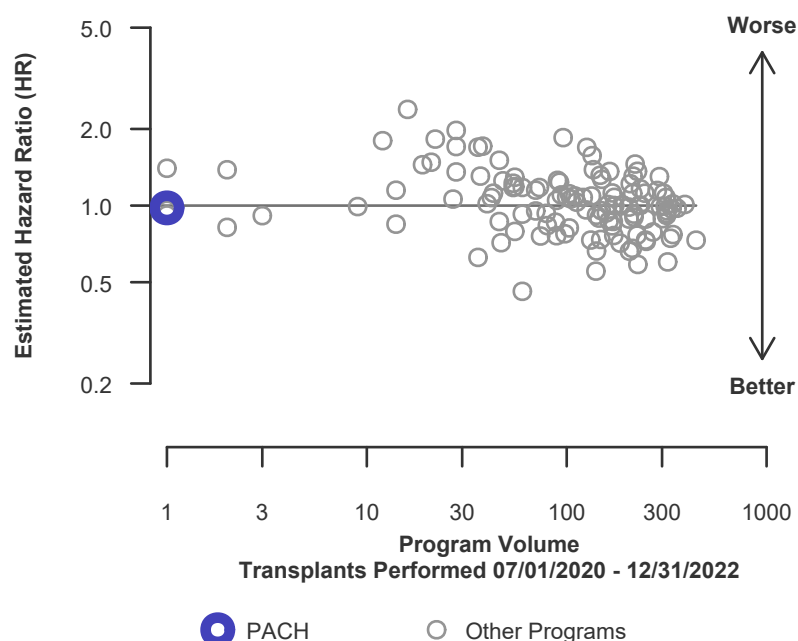


Figure C26D. Adult (18+) 3-year patient death HR program comparison (deceased donor grafts)





C. Transplant Information

Table C17L. Adult (18+) 3-year patient survival (living donor graft recipients)
Single organ transplants performed between 07/01/2020 and 12/31/2022
Retransplants excluded

	PACH	U.S.
Number of transplants evaluated	1	1,228
Estimated probability of surviving at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	91.86% [90.34%-93.40%]
Expected probability of surviving at 3 years (adjusted for patient and donor characteristics)	95.96%	--
Number of observed deaths during the first 3 years after transplant	0	100
Number of expected deaths during the first 3 years after transplant	0.04	--
Estimated hazard ratio*	0.98	--
95% credible interval for the hazard ratio**	[0.12, 2.73]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.12, 2.73], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 2% lower risk of patient death compared to an average program, but PACH's performance could plausibly range from 88% reduced risk up to 173% increased risk.

Figure C25L. Adult (18+) 3-year patient death HR estimate (living donor grafts)

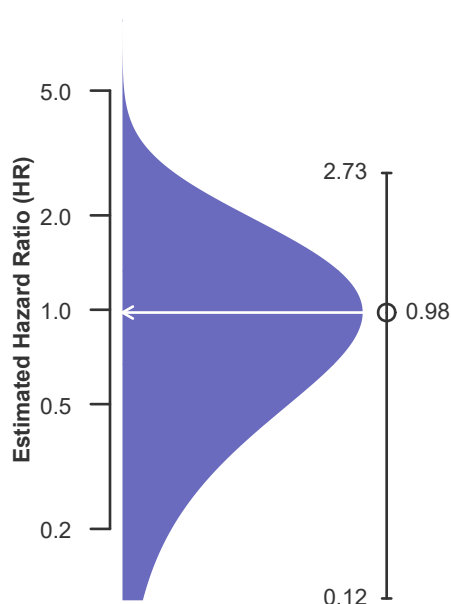
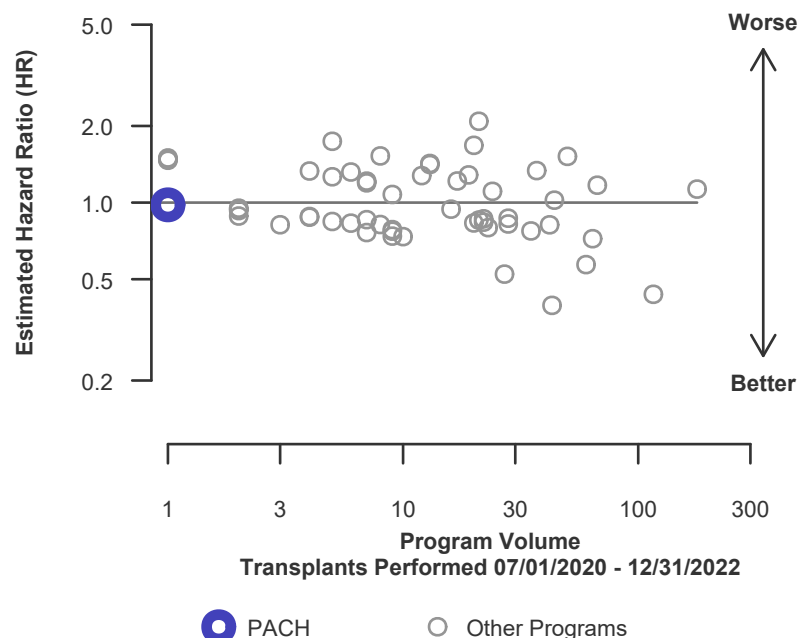


Figure C26L. Adult (18+) 3-year patient death HR program comparison (living donor grafts)





C. Transplant Information

Table C18. Pediatric (<18) 1-month patient survival
Single organ transplants performed between 01/01/2023 and 06/30/2025
Retransplants excluded

	PACH	U.S.
Number of transplants evaluated	51	1,234
Estimated probability of surviving at 1 month & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	98.78% [98.17%-99.40%]
Expected probability of surviving at 1 month (adjusted for patient and donor characteristics)	98.99%	--
Number of observed deaths during the first month after transplant	0	15
Number of expected deaths during the first month after transplant	0.52	--
Estimated hazard ratio*	0.79	--
95% credible interval for the hazard ratio**	[0.10, 2.21]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.10, 2.21], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 21% lower risk of patient death compared to an average program, but PACH's performance could plausibly range from 90% reduced risk up to 121% increased risk.

Figure C27. Pediatric (<18) 1-month patient death HR estimate

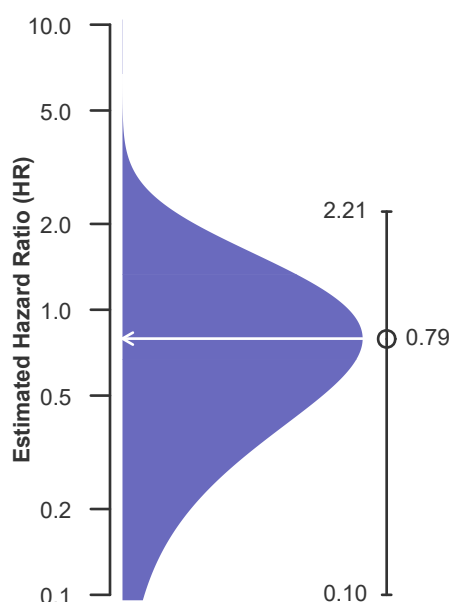
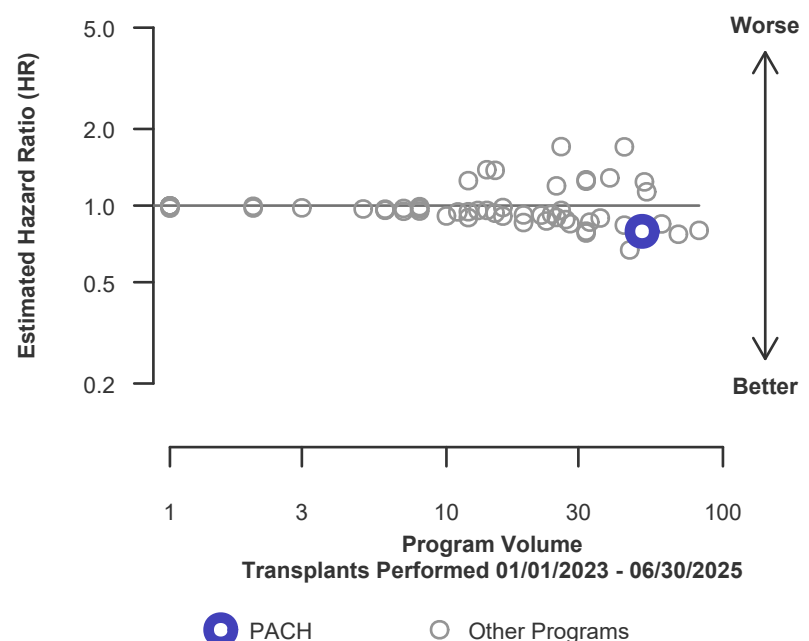


Figure C28. Pediatric (<18) 1-month patient death HR program comparison





C. Transplant Information

Table C18D. Pediatric (<18) 1-month patient survival (deceased donor graft recipients)
Single organ transplants performed between 01/01/2023 and 06/30/2025
Retransplants excluded

	PACH	U.S.
Number of transplants evaluated	31	996
Estimated probability of surviving at 1 month & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	98.49% [97.74%-99.25%]
Expected probability of surviving at 1 month (adjusted for patient and donor characteristics)	98.34%	--
Number of observed deaths during the first month after transplant	0	15
Number of expected deaths during the first month after transplant	0.52	--
Estimated hazard ratio*	0.79	--
95% credible interval for the hazard ratio**	[0.10, 2.21]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.10, 2.21], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 21% lower risk of patient death compared to an average program, but PACH's performance could plausibly range from 90% reduced risk up to 121% increased risk.

Figure C27D. Pediatric (<18) 1-month patient death HR estimate (deceased donor grafts)

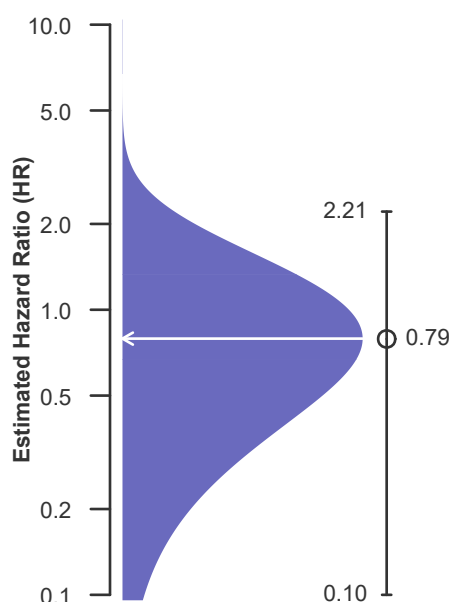
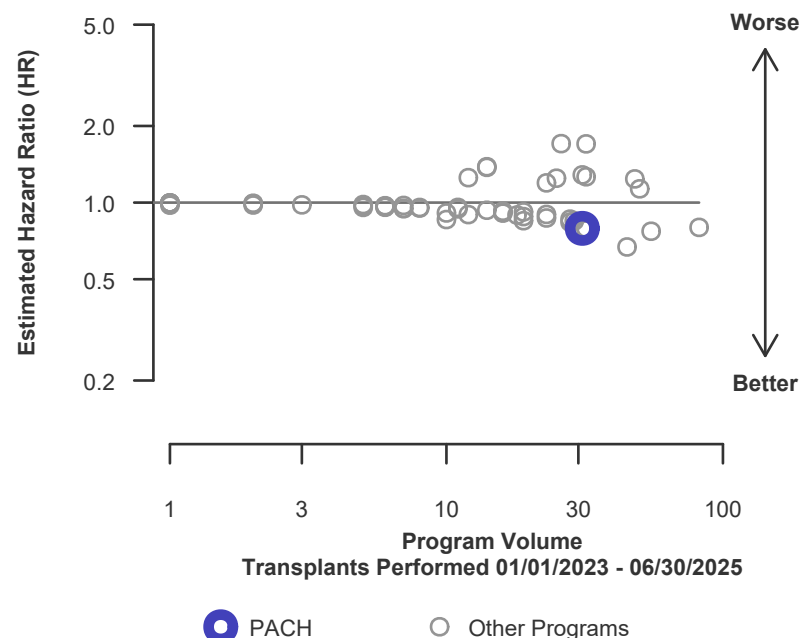


Figure C28D. Pediatric (<18) 1-month patient death HR program comparison (deceased donor grafts)





C. Transplant Information

Table C18L. Pediatric (<18) 1-month patient survival (living donor graft recipients)

Single organ transplants performed between 01/01/2023 and 06/30/2025

Retransplants excluded

	PACH	U.S.
Number of transplants evaluated	20	238
Estimated probability of surviving at 1 month & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	100.00% [100.00%-100.00%]
Expected probability of surviving at 1 month (adjusted for patient and donor characteristics)	100.00%	--
Number of observed deaths during the first month after transplant	0	0
Number of expected deaths during the first month after transplant	0.00	--
Estimated hazard ratio*	1.00	--
95% credible interval for the hazard ratio**	[0.12, 2.79]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.12, 2.79], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 0% lower risk of patient death compared to an average program, but PACH's performance could plausibly range from 88% reduced risk up to 179% increased risk.

Figure C27L. Pediatric (<18) 1-month patient death HR estimate (living donor grafts)

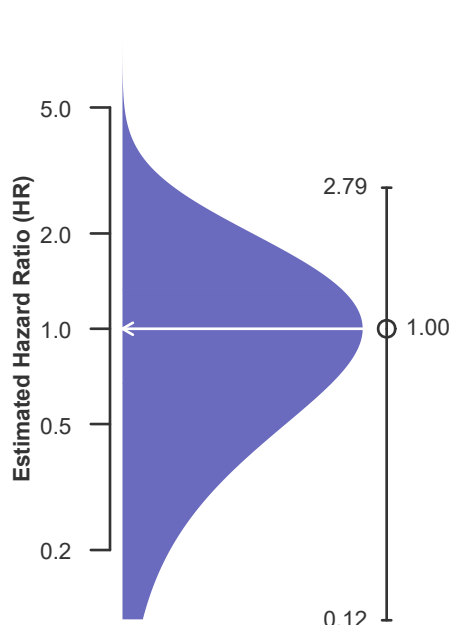
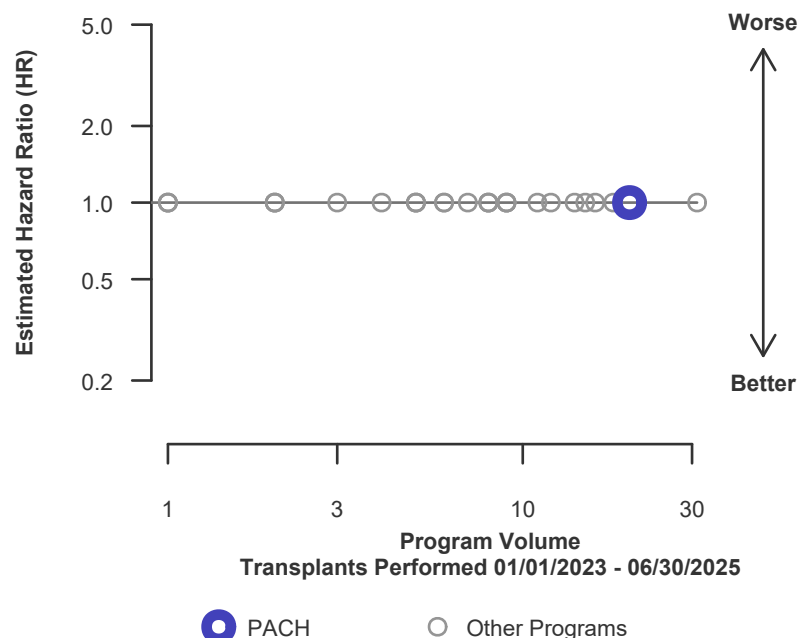


Figure C28L. Pediatric (<18) 1-month patient death HR program comparison (living donor grafts)





C. Transplant Information

Table C19. Pediatric (<18) 1-year patient survival

Single organ transplants performed between 01/01/2023 and 06/30/2025

Retransplants excluded

	PACH	U.S.
Number of transplants evaluated	51	1,234
Estimated probability of surviving at 1 year & [95% CI] (unadjusted for patient and donor characteristics)	96.08% [90.90%-100.00%]	95.91% [94.78%-97.06%]
Expected probability of surviving at 1 year (adjusted for patient and donor characteristics)	95.76%	--
Number of observed deaths during the first year after transplant	2	48
Number of expected deaths during the first year after transplant	2.12	--
Estimated hazard ratio*	0.97	--
95% credible interval for the hazard ratio**	[0.26, 2.13]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.26, 2.13], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 3% lower risk of patient death compared to an average program, but PACH's performance could plausibly range from 74% reduced risk up to 113% increased risk.

Figure C29. Pediatric (<18) 1-year patient death HR estimate

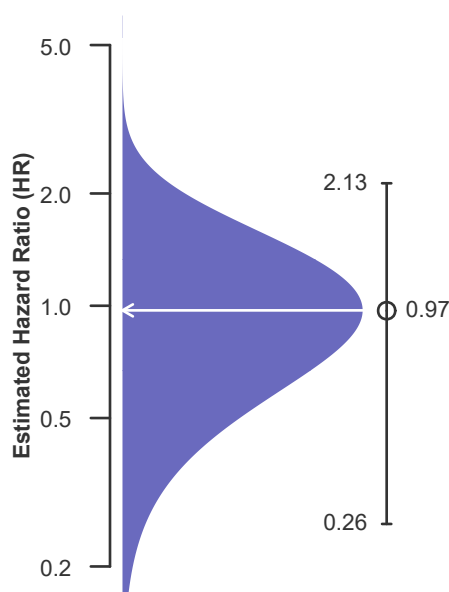
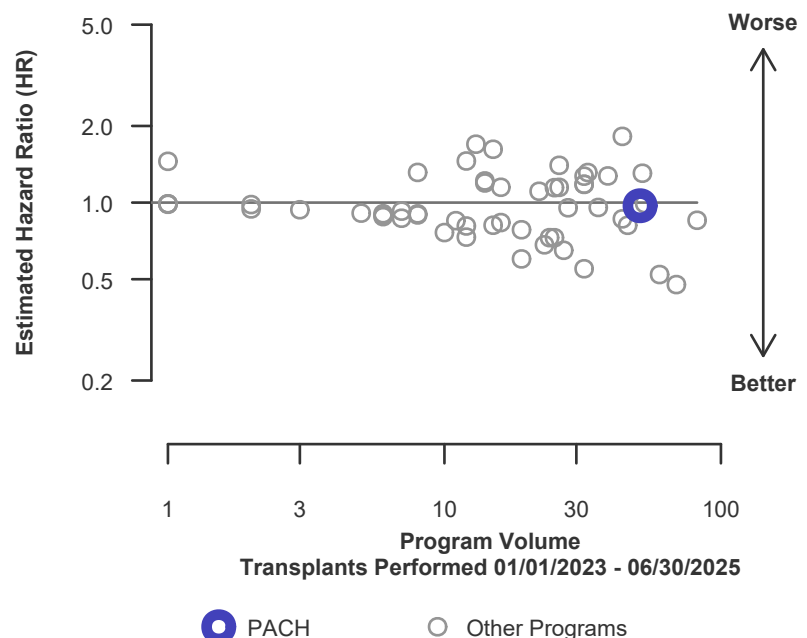


Figure C30. Pediatric (<18) 1-year patient death HR program comparison





C. Transplant Information

Table C19D. Pediatric (<18) 1-year patient survival (deceased donor graft recipients)
Single organ transplants performed between 01/01/2023 and 06/30/2025
Retransplants excluded

	PACH	U.S.
Number of transplants evaluated	31	996
Estimated probability of surviving at 1 year & [95% CI] (unadjusted for patient and donor characteristics)	96.77% [90.75%-100.00%]	95.61% [94.32%-96.93%]
Expected probability of surviving at 1 year (adjusted for patient and donor characteristics)	94.87%	--
Number of observed deaths during the first year after transplant	1	42
Number of expected deaths during the first year after transplant	1.60	--
Estimated hazard ratio*	0.83	--
95% credible interval for the hazard ratio**	[0.17, 2.01]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.17, 2.01], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 17% lower risk of patient death compared to an average program, but PACH's performance could plausibly range from 83% reduced risk up to 101% increased risk.

Figure C29D. Pediatric (<18) 1-year patient death HR estimate (deceased donor grafts)

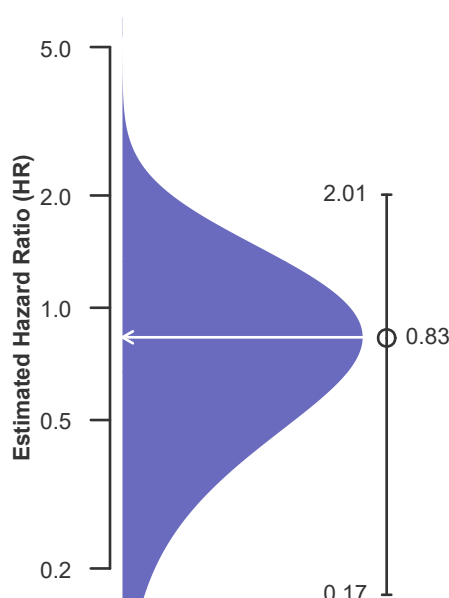
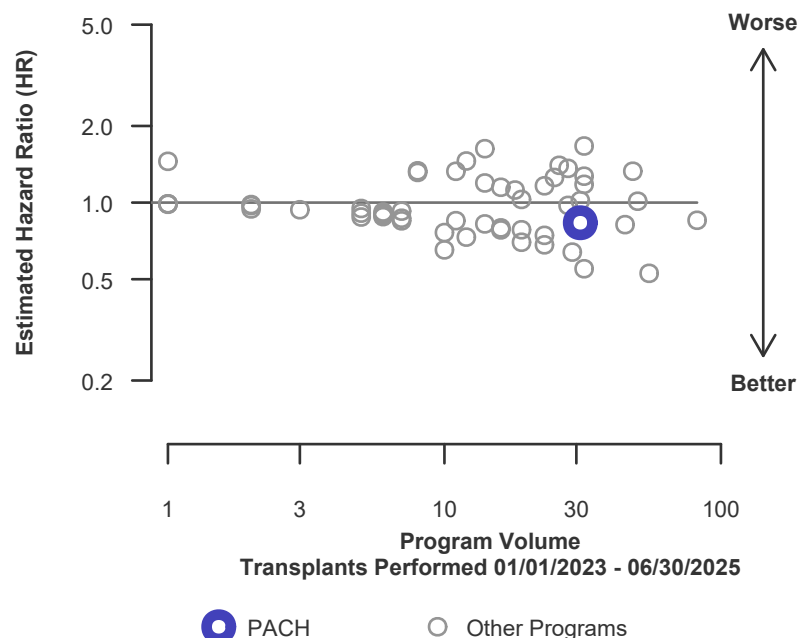


Figure C30D. Pediatric (<18) 1-year patient death HR program comparison (deceased donor grafts)





C. Transplant Information

Table C19L. Pediatric (<18) 1-year patient survival (living donor graft recipients)

Single organ transplants performed between 01/01/2023 and 06/30/2025

Retransplants excluded

	PACH	U.S.
Number of transplants evaluated	20	238
Estimated probability of surviving at 1 year & [95% CI] (unadjusted for patient and donor characteristics)	95.00% [85.91%-100.00%]	97.13% [94.87%-99.44%]
Expected probability of surviving at 1 year (adjusted for patient and donor characteristics)	97.14%	--
Number of observed deaths during the first year after transplant	1	6
Number of expected deaths during the first year after transplant	0.52	--
Estimated hazard ratio*	1.19	--
95% credible interval for the hazard ratio**	[0.25, 2.87]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.25, 2.87], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 19% higher risk of patient death compared to an average program, but PACH's performance could plausibly range from 75% reduced risk up to 187% increased risk.

Figure C29L. Pediatric (<18) 1-year patient death HR estimate (living donor grafts)

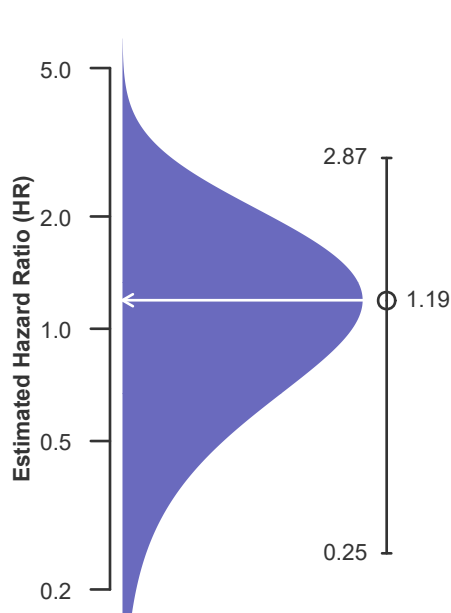
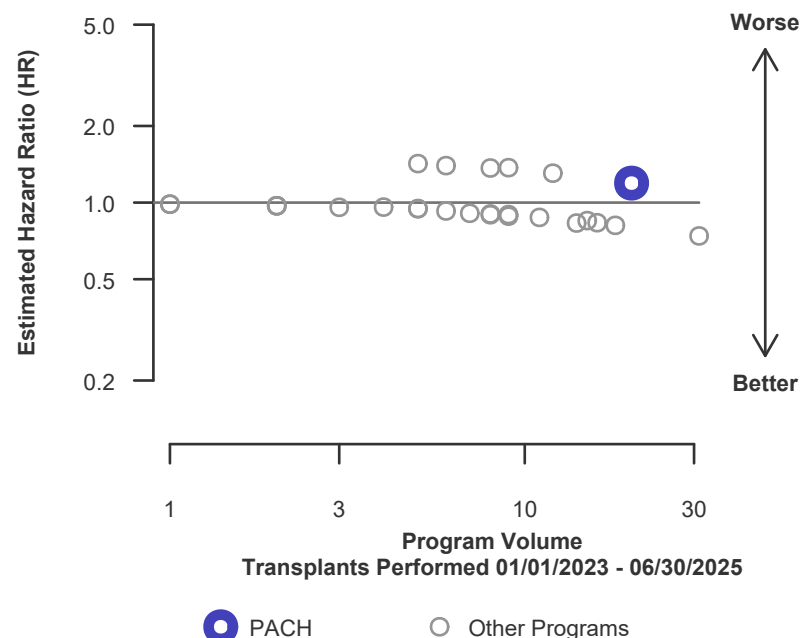


Figure C30L. Pediatric (<18) 1-year patient death HR program comparison (living donor grafts)





C. Transplant Information

Table C20. Pediatric (<18) 3-year patient survival

Single organ transplants performed between 07/01/2020 and 12/31/2022

Retransplants excluded

	PACH	U.S.
Number of transplants evaluated	54	1,161
Estimated probability of surviving at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	96.30% [91.39%-100.00%]	92.94% [91.48%-94.42%]
Expected probability of surviving at 3 years (adjusted for patient and donor characteristics)	93.61%	--
Number of observed deaths during the first 3 years after transplant	2	82
Number of expected deaths during the first 3 years after transplant	3.53	--
Estimated hazard ratio*	0.72	--
95% credible interval for the hazard ratio**	[0.20, 1.59]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.20, 1.59], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 28% lower risk of patient death compared to an average program, but PACH's performance could plausibly range from 80% reduced risk up to 59% increased risk.

Figure C31. Pediatric (<18) 3-year patient death HR estimate

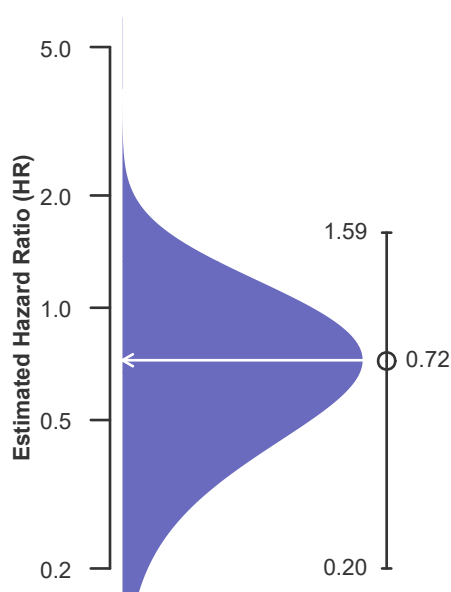
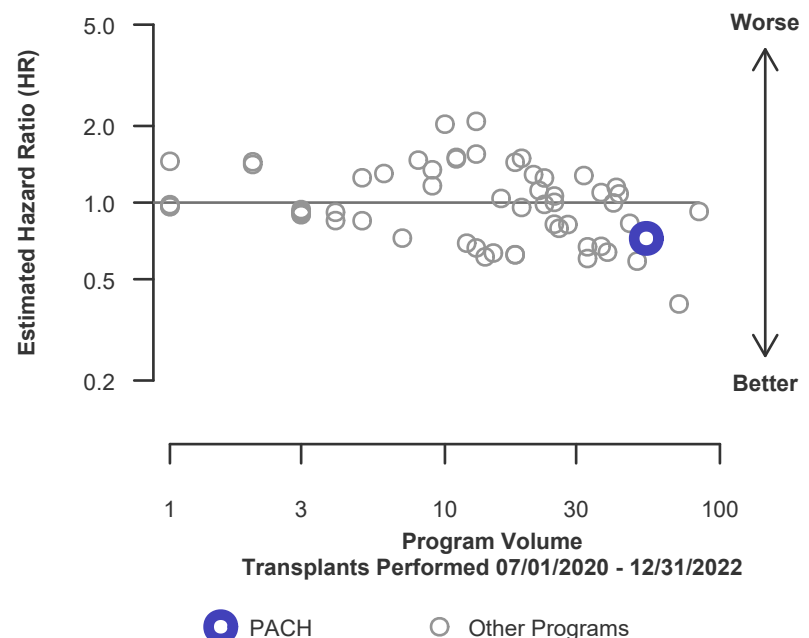


Figure C32. Pediatric (<18) 3-year patient death HR program comparison





C. Transplant Information

Table C20D. Pediatric (<18) 3-year patient survival (deceased donor graft recipients)

Single organ transplants performed between 07/01/2020 and 12/31/2022

Retransplants excluded

	PACH	U.S.
Number of transplants evaluated	38	965
Estimated probability of surviving at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	94.74% [87.90%-100.00%]	92.95% [91.35%-94.58%]
Expected probability of surviving at 3 years (adjusted for patient and donor characteristics)	93.92%	--
Number of observed deaths during the first 3 years after transplant	2	68
Number of expected deaths during the first 3 years after transplant	2.35	--
Estimated hazard ratio*	0.92	--
95% credible interval for the hazard ratio**	[0.25, 2.02]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.25, 2.02], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 8% lower risk of patient death compared to an average program, but PACH's performance could plausibly range from 75% reduced risk up to 102% increased risk.

Figure C31D. Pediatric (<18) 3-year patient death HR estimate (deceased donor grafts)

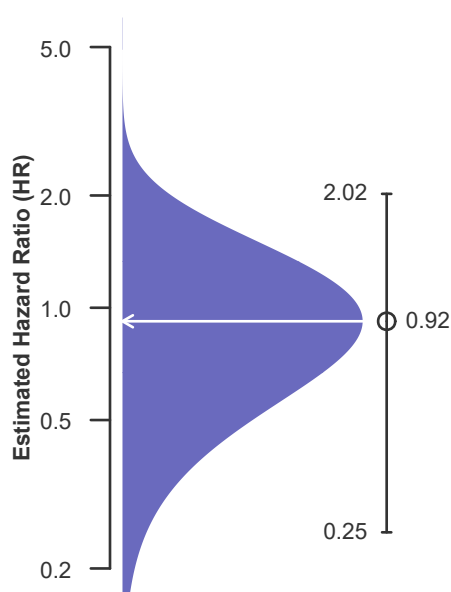
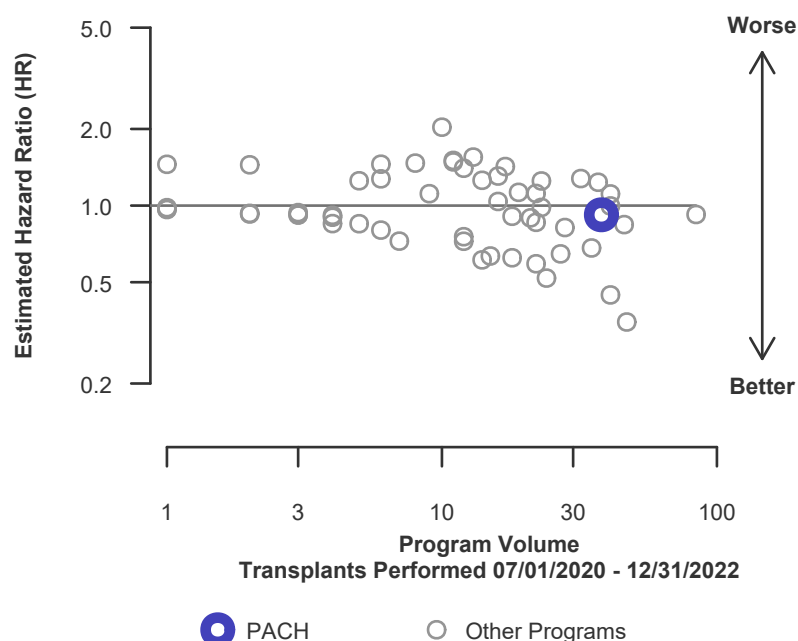


Figure C32D. Pediatric (<18) 3-year patient death HR program comparison (deceased donor grafts)





C. Transplant Information

Table C20L. Pediatric (<18) 3-year patient survival (living donor graft recipients)

Single organ transplants performed between 07/01/2020 and 12/31/2022

Retransplants excluded

	PACH	U.S.
Number of transplants evaluated	16	196
Estimated probability of surviving at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	92.86% [89.32%-96.53%]
Expected probability of surviving at 3 years (adjusted for patient and donor characteristics)	92.88%	--
Number of observed deaths during the first 3 years after transplant	0	14
Number of expected deaths during the first 3 years after transplant	1.18	--
Estimated hazard ratio*	0.63	--
95% credible interval for the hazard ratio**	[0.08, 1.75]	--

* The hazard ratio provides an estimate of how UPMC Children's Hospital of Pittsburgh's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PACH's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.08, 1.75], indicates the location of PACH's true hazard ratio with 95% probability. The best estimate is 37% lower risk of patient death compared to an average program, but PACH's performance could plausibly range from 92% reduced risk up to 75% increased risk.

Figure C31L. Pediatric (<18) 3-year patient death HR estimate (living donor grafts)

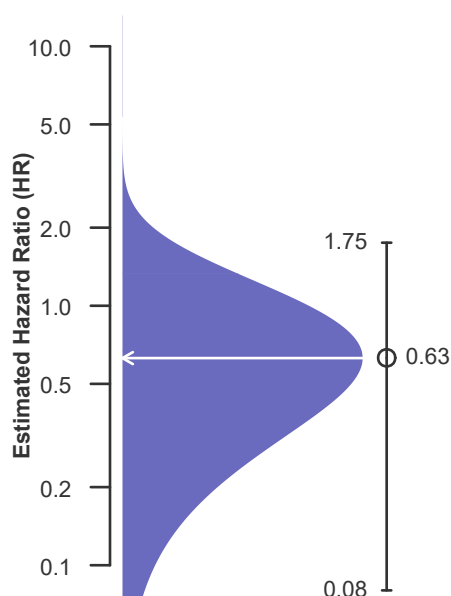
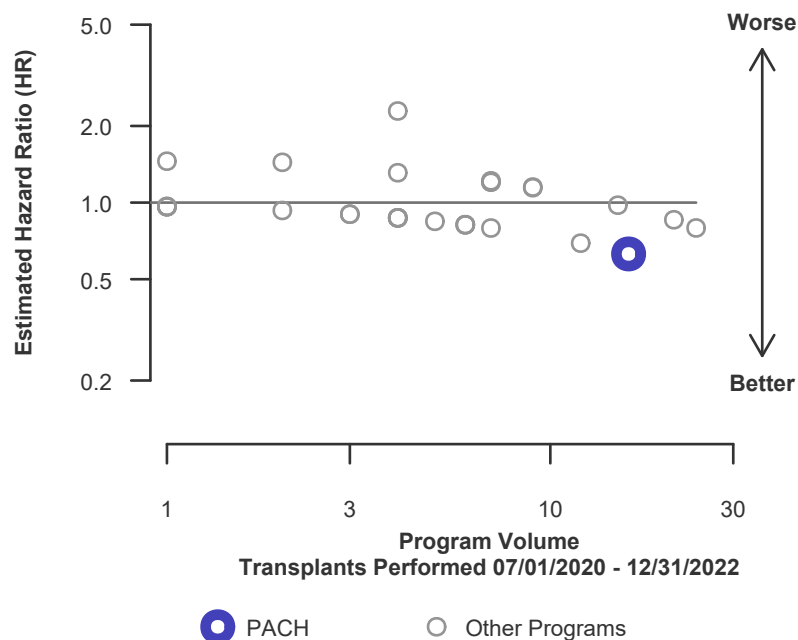


Figure C32L. Pediatric (<18) 3-year patient death HR program comparison (living donor grafts)





C. Transplant Information

Table C21. Multi-organ transplant graft survival: 01/01/2023 - 06/30/2025

Adult (18+) Transplants

Transplant Type	Transplants Performed		Liver Graft Failures		Estimated Liver Graft Survival	
	PACH-TX1	USA	PACH-TX1	USA	PACH-TX1	USA
Kidney-Liver	1	1,984	0	186	100.0%	90.6%

Pediatric (<18) Transplants

Transplant Type	Transplants Performed		Liver Graft Failures		Estimated Liver Graft Survival	
	PACH-TX1	USA	PACH-TX1	USA	PACH-TX1	USA
Kidney-Liver	2	51	0	3	100.0%	94.1%
Kidney-Pancreas-Liver-Intestine	1	9	0	3	100.0%	66.7%
Liver-Heart	1	9	0	0	100.0%	100.0%
Pancreas-Liver-Intestine	4	47	0	12	100.0%	74.5%

Table C22. Multi-organ transplant patient survival: 01/01/2023 - 06/30/2025

Adult (18+) Transplants

Transplant Type	Transplants Performed		Patient Deaths		Estimated Patient Survival	
	PACH-TX1	USA	PACH-TX1	USA	PACH-TX1	USA
Kidney-Liver	1	1,984	0	175	100.0%	91.2%

Pediatric (<18) Transplants

Transplant Type	Transplants Performed		Patient Deaths		Estimated Patient Survival	
	PACH-TX1	USA	PACH-TX1	USA	PACH-TX1	USA
Kidney-Liver	2	51	0	3	100.0%	94.1%
Kidney-Pancreas-Liver-Intestine	1	9	0	3	100.0%	66.7%
Liver-Heart	1	9	0	0	100.0%	100.0%
Pancreas-Liver-Intestine	4	47	0	9	100.0%	80.9%



D. Living Donor Information

Table D1. Living donor summary: 01/01/2023 - 12/31/2025

Living Donor Follow-Up	This Center			United States		
	01/2023- 12/2023	01/2024- 12/2024	01/2025- 06/2025	01/2023- 12/2023	01/2024- 12/2024	01/2025- 06/2025
Number of Living Donors	2	0	0	652	599	325
6-Month Follow-Up						
Donors due for follow-up	2	0	0	652	599	325
Timely clinical data	1 50.0%	0 --%	0 --%	550 84.4%	517 86.3%	274 84.3%
Timely lab data	1 50.0%	0 --%	0 --%	562 86.2%	521 87.0%	284 87.4%
12-Month Follow-Up						
Donors due for follow-up	2	0		652	599	
Timely clinical data	2 100.0%	0 --%		523 80.2%	484 80.8%	
Timely lab data	2 100.0%	0 --%		531 81.4%	496 82.8%	
24-Month Follow-Up						
Donors due for follow-up	2			652		
Timely clinical data	1 50.0%			463 71.0%		
Timely lab data	1 50.0%			473 72.5%		

Follow-up forms due during the COVID-19 amnesty period from 3/13/2020-3/31/2021 are not included in timely clinical and lab data calculations